

IMFINZI®

Durvalumab concentrate for solution for intravenous infusion 50 mg/mL

INDICATIONS AND USAGE

Non-Small Cell Lung Cancer (NSCLC)

IMFINZI is indicated for the treatment of patients with unresectable Stage III non-small cell lung cancer (NSCLC) whose disease has not progressed following concurrent platinum-based chemotherapy and radiation therapy.

IMFINZI in combination with platinum-based chemotherapy as neoadjuvant treatment, followed by IMFINZI as monotherapy after surgery, is indicated for the treatment of adults with resectable (tumours ≥ 4 cm and/or node positive) NSCLC and no known EGFR mutation or ALK rearrangements.

IMFINZI in combination with tremelimumab and platinum-based chemotherapy is indicated for the first-line treatment of adults with metastatic NSCLC with no sensitizing EGFR mutations or ALK positive mutations.

Small Cell Lung Cancer (SCLC)

IMFINZI as monotherapy is indicated for the treatment of adults with limited-stage small cell lung cancer (LS-SCLC) whose disease has not progressed following platinum-based chemoradiation therapy.

IMFINZI in combination with etoposide and either carboplatin or cisplatin is indicated for the first-line treatment of patients with extensive-stage small cell lung cancer (ES-SCLC).

Biliary Tract Cancers (BTC)

IMFINZI in combination with gemcitabine and cisplatin is indicated for the first-line treatment of adults with unresectable or metastatic biliary tract cancer (BTC).

Hepatocellular Carcinoma (HCC)

IMFINZI in combination with tremelimumab is indicated for the first line treatment of adults with advanced or unresectable hepatocellular carcinoma (HCC).

Endometrial Cancer

IMFINZI in combination with carboplatin and paclitaxel is indicated for the first-line treatment of adults with primary advanced or recurrent endometrial cancer who are candidates for systemic therapy, followed by maintenance treatment with:

- IMFINZI as monotherapy in endometrial cancer that is mismatch repair deficient (dMMR)
- IMFINZI in combination with olaparib in endometrial cancer that is mismatch repair proficient (pMMR).

Muscle Invasive Bladder Cancer (MIBC)

IMFINZI in combination with gemcitabine and cisplatin as neoadjuvant treatment, followed by IMFINZI as monotherapy adjuvant treatment after radical cystectomy, is indicated for the treatment of adults with resectable muscle invasive bladder cancer (MIBC).

DOSAGE AND ADMINISTRATION

Patient Selection

MMR testing for patients with endometrial cancer

Patients with endometrial cancer should be evaluated for treatment based on tumour MMR status confirmed by a validated test [see [Clinical Studies](#)].

Posology

The recommended dosages for IMFINZI monotherapy and IMFINZI in combination therapy is presented in Table 1. [see [Clinical Studies](#)]

IMFINZI is administered as an intravenous infusion over 60 minutes.

Table 1. Recommended dose of IMFINZI monotherapy and combination therapy

Indication	Recommended IMFINZI dose	Duration of Therapy
Monotherapy		
Stage III Unresectable NSCLC	Patients with a body weight of more than 30 kg: 10 mg/kg every 2 weeks or 1500 mg every 4 weeks Patients with a body weight of 30 kg or less: 10 mg/kg every 2 weeks until weight increases to greater than 30 kg	Until disease progression, unacceptable toxicity, or a maximum of 12 months
LS-SCLC	Patients with a body weight of more than 30 kg: 1500 mg every 4 weeks Patients with a body weight of 30 kg or less: 10 mg/kg every 2 weeks or 20 mg/kg every 4 weeks as monotherapy until weight increases to greater than 30 kg.	Until disease progression, unacceptable toxicity, or a maximum of 24 months.
Combination therapy		
Resectable NSCLC	Patients with a body weight of more than 30 kg: Neoadjuvant: IMFINZI 1500 mg in combination with chemotherapy ^a every 3 weeks for up to 4 cycles prior to surgery Adjuvant: IMFINZI 1500 mg as a single agent every 4 weeks for up to 12 cycles after surgery	Until disease progression that precludes definitive surgery, recurrence, unacceptable toxicity, or a maximum of 12 cycles after surgery

	Patients with a body weight of 30 kg or less: Neoadjuvant: IMFINZI 20 mg/kg every 3 weeks in combination with chemotherapy^a for up to 4 cycles prior to surgery Adjuvant: IMFINZI 20 mg/kg every 4 weeks for up to 12 cycles as a single agent after surgery until weight increases to greater than 30 kg	
Metastatic NSCLC	Patients with a body weight of more than 30 kg: During platinum chemotherapy: 1500 mg in combination with tremelimumab 75 mg^{b,c} and platinum-based chemotherapy^d every 3 weeks (21 days) for 4 cycles (12 weeks). Post-platinum chemotherapy: 1500 mg every 4 weeks as monotherapy and histology-based pemetrexed maintenance^{d,e} therapy every 4 weeks. A fifth dose of tremelimumab 75 mg^{f,g} should be given at week 16 alongside IMFINZI. Patients with a body weight of 30 kg or less: IMFINZI 20 mg/kg until weight is greater than 30 kg.	Until disease progression or unacceptable toxicity
ES-SCLC	Patients with a body weight of more than 30 kg: 1500 mg in combination with chemotherapy^a every 3 weeks (21 days) for 4 cycles, followed by 1500 mg every 4 weeks as monotherapy Patients with a body weight of 30 kg or less: 20 mg/kg in combination with chemotherapy^a every 3 weeks (21 days) for 4 cycles, followed by 20 mg/kg every 4 weeks as monotherapy until weight increases to greater than 30 kg	Until disease progression or unacceptable toxicity

BTC	Patients with a body weight of more than 36 kg: 1500 mg in combination with chemotherapy^a every 3 weeks (21 days) up to 8 cycles, followed by 1500 mg every 4 weeks as monotherapy Patients with a body weight of 36 kg or less: 20 mg/kg in combination with chemotherapy^a dose every 3 weeks (21 days) up to 8 cycles, followed by monotherapy at 20 mg/kg every 4 weeks until weight increases to greater than 36 kg	Until disease progression or until unacceptable toxicity
HCC	Patients with a body weight of more than 30 kg: • IMFINZI 1500 mg following a single dose of tremelimumab 300 mg^h at Day 1 of Cycle 1; • Continue IMFINZI 1500 mg as a single agent every 4 weeks. Patients with a body weight of 30 kg or less: IMFINZI 20 mg/kg until weight is greater than 30 kg.	Until disease progression or unacceptable toxicity
Endometrial Cancer	Patients with a body weight of more than 30 kg: 1120 mg in combination with carboplatin and paclitaxel^a every 3 weeks (21 days) for a minimum of 4 and up to 6 cycles, followed by IMFINZI 1 500 mg every 4 weeks as monotherapy (dMMR patients) or in combination with olaparib 300 mg twice daily (pMMR patients) Patients with a body weight of 30 kg or less: 15mg/kg in combination with carboplatin and paclitaxel^a every 3 weeks (21 days) for a minimum of 4 and up to 6 cycles, followed by IMFINZI 20mg/kg every 4 weeks as monotherapy (dMMR patients) or in combination with olaparib 300 mg	Until disease progression or unacceptable toxicity

	twice daily (pMMR patients) until weight increases to greater than 30 kg	
MIBC	Patients with body weight of more than 30 kg: Neoadjuvant: 1500 mg in combination with gemcitabine and cisplatin^a every 3 weeks for 4 cycles prior to surgery Adjuvant: IMFINZI 1500 mg as a single agent every 4 weeks for up to 8 cycles after surgery Patients with body weight of 30 kg or less: Neoadjuvant: 20 mg/kg in combination with gemcitabine and cisplatin^a every 3 weeks for 4 cycles prior to surgery Adjuvant: IMFINZI 20 mg/kg as a single agent every 4 weeks for up to 8 cycles after surgery	Until disease progression that precludes definitive surgery, recurrence, or unacceptable toxicity or a maximum of 8 cycles after surgery

^a Administer IMFINZI prior to chemotherapy when given on the same day. Refer to the Prescribing Information for the agent(s) administered in combination with IMFINZI for recommended dosage information, as appropriate.

^b Patients with a body weight of 34 kg or less must receive weight based dosing equivalent to tremelimumab 1 mg/kg until weight is greater than 34 kg.

^c When IMFINZI is administered in combination with tremelimumab and platinum-based chemotherapy, refer to the Prescribing Information for tremelimumab for dosing information.

^d When IMFINZI is administered in combination with chemotherapy, refer to the Prescribing Information for etoposide, nab-paclitaxel, gemcitabine, pemetrexed and carboplatin or cisplatin for dosing information.

^e Consider maintenance administration of pemetrexed for patients with non-squamous tumors who received treatment with pemetrexed and carboplatin/cisplatin during the platinum-based chemotherapy stage.

^f In the case of dose delay(s), a fifth dose of tremelimumab can be given after Week 16, alongside IMFINZI.

If patients receive fewer than 4 cycles of platinum-based chemotherapy, the remaining cycles of tremelimumab (up to a total of 5) alongside IMFINZI should be given during the post-platinum chemotherapy phase.

^h Patients with a body weight of 40 kg or less must receive weight-based dosing, equivalent to tremelimumab 4 mg/kg until weight is greater than 40 kg.

Dosage Modifications for Adverse Reactions

No dose reductions are recommended. In general, withhold or discontinue IMFINZI for severe (Grade 3) immune-mediated adverse reactions. Permanently discontinue IMFINZI for life-threatening (Grade 4) immune-mediated adverse reactions, recurrent severe (Grade 3) immune-mediated reactions that require systemic immunosuppressive treatment, or an inability to reduce corticosteroid dose to 10 mg or less of prednisone or equivalent per day within 12 weeks of initiating corticosteroids.

Immune-mediated adverse reactions requiring specific management are summarized in Table 2. Refer to Warnings and Precautions section for further management monitoring, monitoring and evaluation information.

Table 2. Recommended treatment modifications and management modifications for IMFINZI or IMFINZI in combination with other products.

Adverse Reaction	Severity ^a	Treatment Modification
Immune-mediated adverse reactions		
Immune-mediated pneumonitis/interstitial lung disease	Grade 2	Withhold dose ^b
	Grade 3 or 4	Permanently discontinue
Immune-mediated hepatitis	ALT or AST > 3 - ≤5 x ULN or total bilirubin > 1.5 - ≤3 x ULN	Withhold dose ^c
	ALT or AST > 5 - ≤10 x ULN	Withhold IMFINZI and permanently discontinue tremelimumab (where appropriate)
	Concurrent ALT or AST > 3 x ULN and total bilirubin > 2 x ULN ^c	Permanently discontinue
	ALT or AST > 10 x ULN or total bilirubin > 3 x ULN	

Immune-mediated hepatitis in HCC (or secondary tumor involvement of the liver with abnormal baseline values) ^d	ALT or AST > 2.5 - ≤ 5 x BLV and ≤ 20 x ULN	Withhold dose
	ALT or AST > 5 – 7 x BLV and ≤ 20 x ULN or concurrent ALT or AST 2.5 – 5 x BLV and ≤ 20 x ULN and total bilirubin > 1.5 - < 2 x ULN ^c	Withhold IMFINZI and permanently discontinue tremelimumab (where appropriate)
	ALT or AST > 7 x BLV or > 20 x ULN whichever occurs first or bilirubin > 3 x ULN	Permanently discontinue
Immune-mediated colitis or diarrhea	Grade 2	Withhold dose ^b
	Grade 3 for IMFINZI monotherapy	Withhold dose ^b
	Grade 3 for IMFINZI + tremelimumab	Permanently discontinue tremelimumab ^e
	Grade 4	Permanently discontinue
Intestinal perforation ^h	Any grade	Permanently discontinue

Immune-mediated hyperthyroidism, thyroiditis	Grade 2-4	Withhold dose until clinically stable
Immune-mediated hypothyroidism	Grade 2-4	No changes
Immune-mediated adrenal insufficiency, Hypophysitis/hypopituitarism	Grade 2-4	Withhold dose until clinically stable
Immune-mediated Type 1 diabetes mellitus	Grade 2-4	No changes
Immune-mediated nephritis	Grade 2 with serum creatinine > 1.5 - 3 x (ULN or baseline)	Withhold dose ^b
	Grade 3 with serum creatinine > 3 x baseline or > 3-6 x ULN; Grade 4 with serum creatinine > 6 x ULN	Permanently discontinue
Immune-mediated rash or dermatitis (including pemphigoid)	Grade 2 for > 1 week or Grade 3	Withhold dose ^b
	Grade 4	Permanently discontinue
Immune-mediated myocarditis	Grade 2-4	Permanently discontinue
		Withhold dose ^{b, f}

Immune-mediated myositis/polymyositis/rhabdomyolysis	Grade 2 or 3	
	Grade 4	Permanently discontinue
Infusion-related reactions	Grade 1 or 2	Interrupt or slow the rate of infusion
	Grade 3 or 4	Permanently discontinue
Immune-mediated myasthenia gravis	Grade 2 – 4	Permanently discontinue
Immune-mediated Myelitis Transverse	Any grade	Permanently discontinue
Immune-mediated meningitis	Grade 2	Withhold dose
	Grade 3 or 4	Permanently discontinue
Immune-mediated encephalitis	Grade 2 – 4	Permanently discontinue

Immune-mediated Guillain-Barre syndrome	Grade 2 – 4	Permanently discontinue
Other immune-mediated adverse reactions ^g	Grade 2 or 3	Withhold dose ^b
	Grade 4	Permanently discontinue
Non-immune-mediated adverse reactions		
Pure red cell aplasia (PRCA) ⁱ	Any Grade	Permanently discontinue
Other non-immune-mediated adverse reactions	Grade 2 or 3	Withhold dose until $\leq$ Grade 1 or return to baseline
	Grade 4	Permanently discontinue ^j

^a Common Terminology Criteria for Adverse Events, version 4.03. ALT: alanine aminotransferase; AST: aspartate aminotransferase; ULN: upper limit of normal; BLV: baseline value.

^b After withholding, IMFINZI can be resumed within 12 weeks if the adverse reactions improved to $\leq$ Grade 1 and the corticosteroid dose has been reduced to $\leq$ 10 mg prednisone or equivalent per day. IMFINZI should be permanently discontinued for recurrent Grade 3 adverse reactions, as applicable.

^c For patients with alternative cause follow the recommendations for AST or ALT increases without concurrent bilirubin elevations.

^d If AST and ALT are less than or equal to ULN at baseline in patients with liver involvement, withhold or permanently discontinue durvalumab based on recommendations for hepatitis with no liver involvement.

^e Permanently discontinue tremelimumab for Grade 3; however, treatment with durvalumab can be resumed once event has resolved.

^f Permanently discontinue IMFINZI if the adverse reaction does not resolve to $\leq$ Grade 1 within 30 days or if there are signs of respiratory insufficiency.

^g Includes immune thrombocytopenia, pancreatitis, encephalitis, immune-mediated arthritis, and uveitis.

^h Adverse reaction is only associated with IMFINZI in combination with tremelimumab.

ⁱ Adverse drug reaction is only associated when olaparib maintenance treatment is used in combination with IMFINZI, following treatment with IMFINZI in combination with platinum-based chemotherapy.

^j With the exception of Grade 4 laboratory abnormalities, about which the decision to discontinue should be based on accompanying clinical signs/symptoms and clinical judgement.

Preparation and Administration

Preparation

- Visually inspect drug product for particulate matter and discoloration prior to administration, whenever solution and container permit. Discard the vial if the solution is cloudy, discolored, or visible particles are observed.
- Do not shake the vial.
- Withdraw the required volume from the vial(s) of IMFINZI and transfer into an intravenous bag containing 0.9% Sodium Chloride Injection, USP or 5% Dextrose Injection, USP. Mix diluted solution by gentle inversion. Do not shake the solution. The final concentration of the diluted solution should be between 1 mg/mL and 15 mg/mL.
- Discard partially used or empty vials of IMFINZI.

Storage of Infusion Solution

- IMFINZI does not contain a preservative.
- Administer infusion solution immediately once prepared. If infusion solution is not administered immediately and needs to be stored, the time from preparation until the completion of the infusion should not exceed:
 - 28 days in a refrigerator at 2°C to 8°C (36°F to 46°F)
 - 12 hours at room temperature up to 25°C (77°F)
- Do not freeze.
- Do not shake.

Administration

- Administer infusion solution intravenously over 60 minutes through an intravenous line containing a sterile, low-protein binding 0.2 or 0.22 micron in-line filter.
- Use separate infusion bags and filters for each drug product.

IMFINZI in Combination with Other Products

- Administer all drug product as separate infusions.
- Do not co administer other drugs through the same infusion line.
- For platinum-based chemotherapy, refer to Prescribing Information for administration information.
- For pemetrexed therapy, refer to Prescribing Information for administration information.

Combination Regimens: Order of Infusions

IMFINZI in Combination with Tremelimumab

- Infuse tremelimumab first, followed by IMFINZI on the same day of dosing.

IMFINZI in Combination with Tremelimumab and Platinum-Based Chemotherapy

- Infuse tremelimumab first, followed by IMFINZI and then platinum-based chemotherapy on the day of dosing.

IMFINZI in Combination with Tremelimumab and Pemetrexed Therapy

- Infuse tremelimumab first, followed by IMFINZI and then pemetrexed therapy on the day of dosing.

IMFINZI in Combination with Chemotherapy

- Infuse IMFINZI first and then chemotherapy on the same day of dosing.

Combination Regimens: Infusion Instructions

IMFINZI in Combination with Tremelimumab

- Administer tremelimumab over 60 minutes followed by a 60 minutes observation period. Then administer IMFINZI as a separate intravenous infusion over 60 minutes.

IMFINZI in Combination with Tremelimumab and Platinum-Based Chemotherapy/Pemetrexed Therapy

Cycle 1

- Infuse tremelimumab over 1 hour. One to two hours after completion of tremelimumab infusion, infuse IMFINZI over 1 hour. One to two hours after completion of IMFINZI infusion, administer platinum-based chemotherapy.

Subsequent Cycles

- If there are no infusion reactions during Cycle 1, subsequent cycles of IMFINZI can be given immediately after tremelimumab. The time between the end of the IMFINZI infusion and the start of chemotherapy can be reduced to 30 minutes.

DOSAGE FORMS AND STRENGTHS

Injection: 120 mg/2.4 mL (50 mg/mL) and 500 mg/10 mL (50 mg/mL) clear to opalescent, colorless to slightly yellow solution in a single-dose vial.

CONTRAINDICATIONS

None.

WARNINGS AND PRECAUTIONS

Refer to Dosage and Administration section, Table 2 for recommended treatment modifications.

For suspected immune-mediated adverse reactions, adequate evaluation should be performed to confirm etiology or exclude external etiologies. Based on the severity of the adverse reaction, IMFINZI or IMFINZI in combination with tremelimumab should be withheld or permanently discontinued. Treatment with corticosteroids or endocrine therapy should be initiated. For events requiring corticosteroid therapy, and upon improvement to $\leq$ Grade 1, corticosteroid taper should be initiated and continued over at least 1 month. Consider increasing dose of corticosteroids and/or using additional systemic immunosuppressants if there is worsening or no improvement.

Immune-mediated pneumonitis

Immune-mediated pneumonitis or interstitial lung disease, defined as requiring use of systemic corticosteroids and with no clear alternate etiology, occurred in patients receiving IMFINZI or IMFINZI in combination with tremelimumab or IMFINZI in combination with platinum-based chemotherapy followed by IMFINZI in combination with olaparib. [see [Adverse Reactions](#)] Patients should be monitored for signs and symptoms of pneumonitis. Suspected pneumonitis should be confirmed with radiographic imaging and other infectious and disease-related etiologies excluded, and managed as recommended in Dosage and Administration section. [see

Dosage and Administration] For Grade 2 events, an initial dose of 1 – 2 mg/kg/day prednisone or equivalent should be initiated followed by a taper. For Grade 3 or 4 events, an initial dose of 2 – 4 mg/kg/day methylprednisolone or equivalent should be initiated followed by a taper.

Pneumonitis and radiation pneumonitis

Radiation pneumonitis is frequently observed in patients receiving radiation therapy to the lung and the clinical presentation of pneumonitis and radiation pneumonitis is very similar.

In the PACIFIC Study, in patients who had completed treatment with concurrent chemoradiation within 1 to 42 days prior to initiation of the trial, pneumonitis and radiation pneumonitis occurred in patients receiving IMFINZI. Pneumonitis or radiation pneumonitis occurred in 161 (33.9%) patients in the IMFINZI-treated group and 58 (24.8%) in the placebo group; including Grade 3 in 16 (3.4%) patients on IMFINZI vs. 7 (3.0%) patients on placebo and Grade 5 in 5 (1.1%) patients on IMFINZI vs. 4 (1.7%) patients on placebo. The median time to onset in the IMFINZI-treated group was 55 days (range: 1-406 days) vs. 55 days (range: 1-255 days) in the placebo group.

In patients receiving IMFINZI monotherapy, immune-mediated pneumonitis occurred in 92 (3.1%) patients, including Grade 3 in 25 (0.8%) patients, Grade 4 in 2 (<0.1%) patients, and Grade 5 in 6 (0.2%) patients. The median time to onset was 55 days (range: 2-785 days). Sixty-nine of the 92 patients received high-dose corticosteroid treatment (at least 40 mg prednisone or equivalent per day), 2 patients also received infliximab and 1 patient also received cyclosporine. IMFINZI was discontinued in 38 patients. Resolution occurred in 53 patients. Immune-mediated pneumonitis occurred more frequently in patients in the PACIFIC Study who had completed treatment with concurrent chemoradiation within 1 to 42 days prior to initiation of the study (9.9%), compared to the other patients in the combined safety database (1.8%).

In the PACIFIC Study, in patients with locally advanced, unresectable NSCLC (n=475 in the IMFINZI arm, and n=234 in the placebo arm) who had completed treatment with concurrent chemoradiation within 1 to 42 days prior to initiation of the study, immune-mediated pneumonitis occurred in 47 (9.9%) patients in the IMFINZI-treated group and 14 (6.0%) patients in the placebo group, including Grade 3 in 9 (1.9%) patients on IMFINZI vs. 6 (2.6%) patients on placebo and Grade 5 in 4 (0.8%) patients on IMFINZI vs. 3 (1.3%) patients on placebo. The median time to onset in the IMFINZI-treated group was 46 days (range: 2-342 days) vs. 57 days (range: 26-253 days) in the placebo group. In the IMFINZI-treated group 30 patients received high-dose corticosteroid treatment (at least 40 mg prednisone or equivalent per day) and 2 patients also received infliximab. In the placebo group 12 patients received high-dose corticosteroid treatment (at least 40 mg prednisone or equivalent per day) and 1 patient also received cyclophosphamide and tacrolimus.ⁱ Resolution occurred for 29 patients in the IMFINZI treated group vs 6 in placebo.

In the DUO-E Study, out of 238 patients treated with platinum-based chemotherapy in combination with IMFINZI, followed by IMFINZI in combination with olaparib (platinum-based chemotherapy + IMFINZI + olaparib arm) immune-mediated pneumonitis occurred in 5 (2.1%) patients, including Grade 3 in 3 (1.3%) patients. The median time to onset was 85 days (range: 65-321 days). Five patients received systemic corticosteroids, including 4 patients who received high-dose corticosteroid treatment (at least 40 mg prednisone or equivalent per day). Resolution occurred in all 5 patients.

In the ADRIATIC Study, in patients who had completed treatment with chemoradiation within 1 to 42 days prior to initiation of study treatment, pneumonitis or radiation pneumonitis occurred in 100 (38.2%) patients in the IMFINZI-treated group and 80 (30.2%) in the placebo group; including Grade 3 in 8 (3.1%) patients on IMFINZI vs 6 (2.3%) patients on placebo, and Grade 5 in 1 (0.4%) patient on IMFINZI vs 0 patients on placebo.

Immune-mediated hepatitis

Immune-mediated hepatitis, defined as requiring use of systemic corticosteroids and with no clear alternate etiology, occurred in patients receiving IMFINZI or IMFINZI in combination with tremelimumab. [see [Adverse Reactions](#)] Monitor alanine aminotransferase, aspartate aminotransferase, total bilirubin and alkaline phosphatase levels prior to initiation of treatment and prior to each subsequent infusion. Additional monitoring is to be considered based on clinical evaluation. Immune-mediated hepatitis should be managed as recommended in Dosage and Administration section. [see [Dosage and Administration](#)] Corticosteroids should be administered with an initial dose of 1 – 2 mg/kg/day prednisone or equivalent followed by a taper for all grades.

In patients receiving IMFINZI monotherapy, immune-mediated hepatitis occurred in 67 (2.2%) patients, including Grade 3 in 35 (1.2%) patients, Grade 4 in 6 (0.2%) and Grade 5 in 4 (0.1%) patients. The median time to onset was 36 days (range: 3-333 days). Forty-four of the 67 patients received high-dose corticosteroid treatment (at least 40 mg prednisone or equivalent per day). Three patients also received mycophenolate treatment. IMFINZI was discontinued in 9 patients. Resolution occurred in 29 patients.

Immune-mediated colitis

Immune-mediated colitis or diarrhea, defined as requiring use of systemic corticosteroids and with no clear alternate etiology, occurred in patients receiving IMFINZI or IMFINZI in combination with tremelimumab. [see [Adverse Reactions](#)] Intestinal perforation and large intestine perforation were reported in patients receiving IMFINZI in combination with tremelimumab. Patients should be monitored for signs and symptoms of colitis or diarrhea and intestinal perforation and managed as recommended in Dosage and Administration section. [see [Dosage and Administration](#)] Corticosteroids should be administered at an initial dose of 1 – 2 mg/kg/day prednisone or equivalent followed by a taper for Grades 2 – 4. Consult a surgeon immediately if intestinal perforation of ANY grade is suspected.

In patients receiving IMFINZI monotherapy, immune-mediated colitis or diarrhea occurred in 58 (1.9%) patients, including Grade 3 in 9 (0.3%) patients and Grade 4 in 2 (<0.1%) patients. The median time to onset was 70 days (range: 1-394 days). Thirty-eight of the 58 patients received high-dose corticosteroid treatment (at least 40 mg prednisone or equivalent per day). One patient also received infliximab treatment and one patient also received mycophenolate treatment. IMFINZI was discontinued in 9 patients. Resolution occurred in 43 patients.

Immune-mediated endocrinopathies

Immune-mediated hypothyroidism/hyperthyroidism/thyroiditis

Immune-mediated hypothyroidism, hyperthyroidism or thyroiditis have occurred in patients receiving IMFINZI or IMFINZI in combination with tremelimumab. [see [Adverse Reactions](#)] Patients should be monitored for abnormal thyroid function tests prior to and periodically during treatment and managed as recommended in Dosage and Administration section. [see [Dosage and Administration](#)] For immune-mediated hypothyroidism, initiate thyroid hormone

replacement as clinically indicated for Grade 2 -4. For immune-mediated hyperthyroidism/thyroiditis, symptomatic management can be implemented for Grades 2 – 4.

Immune-mediated hypothyroidism

In patients receiving IMFINZI monotherapy, immune-mediated hypothyroidism occurred in 245 (8.2%) patients, including Grade 3 in 4 (0.1%) patients. The median time to onset was 85 days (range: 1-562 days). Of the 245 patients, 240 patients received hormone replacement therapy, 6 patients received high-dose corticosteroids (at least 40 mg prednisone or equivalent per day) for immune-mediated hypothyroidism followed by hormone replacement. No patients discontinued IMFINZI due to immune-mediated hypothyroidism. Immune-mediated hypothyroidism was preceded by immune-mediated hyperthyroidism in 20 patients or immune-mediated thyroiditis in 3 patients.

IMFINZI with Carboplatin and Paclitaxel

Immune-mediated hypothyroidism occurred in 14% (34/235) of patients receiving IMFINZI in combination with carboplatin and paclitaxel. Events resolved in 8 of the 34 patients. Endocrine therapy was required in 34 of the 34 patients.

Immune-mediated hyperthyroidism

In patients receiving IMFINZI monotherapy, immune-mediated hyperthyroidism occurred in 50 (1.7%) patients, there were no Grade 3 or 4 cases. The median time to onset was 43 days (range: 1-253 days). Forty-six of the 50 patients received medical therapy (thiamazole, carbimazole, propylthiouracil, perchlorate, calcium channel blocker, or beta-blocker), 11 patients received systemic corticosteroids and 4 of the 11 patients received high-dose systemic corticosteroid treatment (at least 40 mg prednisone or equivalent per day). One patient discontinued IMFINZI due to immune-mediated hyperthyroidism. Resolution occurred in 39 patients.

Immune-mediated thyroiditis

In patients receiving IMFINZI monotherapy, immune-mediated thyroiditis occurred in 12 (0.4%) patients, including Grade 3 in 2 (<0.1%) patients. The median time to onset was 49 days (range: 14-106 days). Of the 12 patients, 10 patients received hormone replacement therapy, 1 patient received high-dose corticosteroids (at least 40 mg prednisone or equivalent per day). One patient discontinued IMFINZI due to immune-mediated thyroiditis.

Immune-mediated adrenal insufficiency

Immune-mediated adrenal insufficiency occurred in patients receiving IMFINZI or IMFINZI in combination with tremelimumab. [see [Adverse Reactions](#)] Patients should be monitored for clinical signs and symptoms of adrenal insufficiency. For symptomatic adrenal insufficiency, patients should be managed as recommended in Dosage and Administration section. [see [Dosage and Administration](#)] Corticosteroids should be administered with an initial dose of 1 – 2 mg/kg/day prednisone or equivalent followed by taper and a hormone replacement as clinical indicated for Grade 2 – 4.

In patients receiving IMFINZI monotherapy, immune-mediated adrenal insufficiency occurred in 14 (0.5%) patients, including Grade 3 in 3 (<0.1%) patients. The median time to onset was 146 days (range: 20-547 days). All 14 patients received systemic corticosteroids; 4 of the 14 patients received high-dose corticosteroid treatment (at least 40 mg prednisone or equivalent per day). No patients discontinued IMFINZI due to immune-mediated adrenal insufficiency. Resolution occurred in 3 patients.

Immune-mediated type 1 diabetes mellitus

Immune-mediated type 1 diabetes mellitus, which can present with diabetic ketoacidosis, occurred in patients receiving IMFINZI or IMFINZI in combination with tremelimumab. [see [Adverse Reactions](#)] Patients should be monitored for clinical signs and symptoms of type 1 diabetes mellitus. For symptomatic type 1 diabetes mellitus, patients should be managed as recommended in Dosage and Administration section. [see [Dosage and Administration](#)] Treatment with insulin can be initiated as clinically indicated for Grades 2 – 4.

In patients receiving IMFINZI monotherapy, Grade 3 immune-mediated type 1 diabetes mellitus occurred in 1 (<0.1%) patient. The time to onset was 43 days. This patient required long-term insulin therapy and IMFINZI was permanently discontinued due to immune-mediated type 1 diabetes mellitus.

Immune-mediated hypophysitis/hypopituitarism

Immune-mediated hypophysitis or hypopituitarism occurred in patients receiving IMFINZI or IMFINZI in combination with tremelimumab. [see [Adverse Reactions](#)] Patients should be monitored for clinical signs and symptoms of hypophysitis or hypopituitarism. For symptomatic hypophysitis or hypopituitarism, patients should be managed as recommended in Dosage and Administration section. [see [Dosage and Administration](#)] Corticosteroids should be administered with an initial dose of 1 – 2 mg/kg/day prednisone or equivalent followed by taper and a hormone replacement as clinically indicated for Grades 2 – 4.

In patients receiving IMFINZI monotherapy, immune-mediated hypophysitis/hypopituitarism occurred in 2 (<0.1%) patients both Grade 3. The time to onset for the events was 44 days and 50 days. Both patients received high-dose corticosteroid treatment (at least 40 mg prednisone or equivalent per day) and one patient discontinued IMFINZI due to immune-mediated hypophysitis/hypopituitarism.

Immune-mediated nephritis

Immune-mediated nephritis, defined as requiring use of systemic corticosteroids and with no clear alternate etiology, occurred in patients receiving IMFINZI or IMFINZI in combination with tremelimumab. [see [Adverse Reactions](#)] Patients should be monitored for abnormal renal function tests prior to and periodically during treatment with IMFINZI or IMFINZI in combination with tremelimumab and managed as recommended in Dosage and Administration section. [see [Dosage and Administration](#)] Corticosteroids should be administered with an initial dose of 1 – 2 mg/kg/day prednisone or equivalent followed by taper for Grades 2 – 4.

In patients receiving IMFINZI monotherapy, immune-mediated nephritis occurred in 14 (0.5%) patients, including Grade 3 in 2 (<0.1%) patients. The median time to onset was 71 days (range: 4-393 days). Nine patients received high-dose corticosteroid treatment (at least 40 mg prednisone or equivalent per day) and 1 patient also received mycophenolate. IMFINZI was discontinued in 5 patients. Resolution occurred in 8 patients.

Immune-mediated rash

Immune-mediated rash or dermatitis (including pemphigoid), defined as requiring use of systemic corticosteroids and with no clear alternate etiology, occurred in patients receiving IMFINZI or IMFINZI in combination with tremelimumab. [see [Adverse Reactions](#)] Patients should be monitored for signs and symptoms of rash or dermatitis and managed as recommended in Dosage and Administration section. [see [Dosage and Administration](#)]

Corticosteroids should be administered with an initial dose of 1 – 2 mg/kg/day prednisone or equivalent followed by taper for Grades 2 > 1 week or Grade 3 and 4.

In patients receiving IMFINZI monotherapy, immune-mediated rash or dermatitis (including pemphigoid) occurred in 50 (1.7%) patients, including Grade 3 in 12 (0.4%) patients. The median time to onset was 43 days (range: 4-333 days). Twenty-four of the 50 patients received high-dose corticosteroid treatment (at least 40 mg prednisone or equivalent per day). IMFINZI was discontinued in 3 patients. Resolution occurred in 31 patients.

In the DUO-E Study, out of 238 patients treated with platinum-based chemotherapy in combination with IMFINZI, followed by IMFINZI in combination with olaparib (platinum-based chemotherapy + IMFINZI + olaparib arm) immune-mediated rash occurred in 8 (3.4%) patients, including Grade 3 in 2 (0.8%) patients. The median time to onset was 155 days (range: 2-308 days). All patients received high-dose corticosteroid treatment (at least 40 mg prednisone or equivalent per day). Resolution occurred in all 8 patients.

Immune-mediated myocarditis

Immune-mediated myocarditis, which can be fatal, occurred in patients receiving IMFINZI or IMFINZI in combination with tremelimumab (see section Adverse Reactions). Patients should be monitored for signs and symptoms of immune-mediated myocarditis and managed as recommended in Dosage and Administration section. Corticosteroids should be administered with an initial dose of 2 -4 mg/kg/day prednisone or equivalent followed by taper for Grades 2 – 4. If no improvement within 2 to 3 days despite corticosteroids, promptly start additional immunosuppressive therapy. Upon resolution (Grade 0), corticosteroid taper should be initiated and continued over at least 1 month.

Other immune-mediated adverse reactions

Given the mechanism of action of IMFINZI or IMFINZI in combination with tremelimumab, other potential immune-mediated adverse reactions may occur. Patients should be monitored for signs and symptoms and managed as recommended in Dosage and Administration section. [see [Dosage and Administration](#)] Other immune-mediated adverse reactions are myasthenia gravis, myelitis transverse, myositis, polymyositis, rhabdomyolysis, Guillain-Barré syndrome, immune thrombocytopenia, pancreatitis, immune-mediated arthritis, uveitis and encephalitis (see section Adverse Reactions).

Other (hematologic/immune): Haemolytic anaemia, aplastic anaemia, hemophagocytic lymphohistiocytosis, systemic inflammatory response syndrome, histiocytic necrotizing lymphadenitis (Kikuchi lymphadenitis), sarcoidosis, immune thrombocytopenia, solid organ transplant rejection, other transplant (including corneal graft) rejection.

Infusion-related reactions

Patients should be monitored for signs and symptoms of infusion-related reactions. Severe infusion-related reactions have been reported in patients receiving IMFINZI or IMFINZI in combination with tremelimumab. [see [Adverse Reactions](#)] For Grade 1 or 2 severity, may consider pre-medications for prophylaxis of subsequent infusion reactions. For Grade 3 or 4, manage severe infusion-related reactions per institutional standard, appropriate clinical practice guidelines and/or society guidelines.

In patients receiving IMFINZI monotherapy, infusion-related reactions occurred in 49 (1.6%) patients, including Grade 3 in 5 (0.2%) patients. There were no Grade 4 or 5 events.

In the DUO-E Study, out of 238 patients treated with platinum-based chemotherapy in combination with IMFINZI, followed by IMFINZI in combination with olaparib (platinum-based chemotherapy + IMFINZI + olaparib arm), infusion-related reactions occurred in 13 (5.5%) patients, including Grade 3 in 1 (0.4%) patient. There were no Grade 4 or 5 events.

Treatment-specific precaution

IMFINZI in combination with olaparib in endometrial cancer

Haematological toxicity

Pure red cell aplasia (PRCA) [see [Adverse Reactions](#)] was reported when olaparib maintenance treatment was used in combination with IMFINZI, following treatment with IMFINZI in combination with platinum-based chemotherapy. If PRCA is confirmed, treatment with IMFINZI and olaparib should be discontinued.

Autoimmune haemolytic anemia (AIHA) was reported when olaparib maintenance treatment was used in combination with IMFINZI, following treatment with IMFINZI in combination with platinum-based chemotherapy. If AIHA is confirmed, treatment with IMFINZI and olaparib should be discontinued.

Embryo-Fetal Toxicity

Based on its mechanism of action and data from animal studies, IMFINZI can cause fetal harm when administered to a pregnant woman. In animal reproduction studies, administration of durvalumab to cynomolgus monkeys from the onset of organogenesis through delivery resulted in increased premature delivery, fetal loss and premature neonatal death. Advise pregnant women of the potential risk to a fetus. Advise females of reproductive potential to use effective contraception during treatment with IMFINZI and for at least 3 months after the last dose of IMFINZI [see [Use in Specific Populations](#)].

INTERACTION WITH OTHER MEDICINAL PRODUCTS AND OTHER FORMS OF INTERACTION

Durvalumab is an immunoglobulin, therefore no formal pharmacokinetic drug-drug interaction studies have been conducted with durvalumab.

Furthermore, in the DUO-E study, the exposure to durvalumab was similar in both treatment arms which indicates that there were no clinically meaningful PK drug-drug interactions between durvalumab and olaparib, although exposure to olaparib was not measured throughout the study.

ADVERSE REACTIONS

The following adverse reactions are discussed in greater detail in other sections of the labeling.

- Immune-mediated pneumonitis [see [Warnings and Precautions](#)].
- Immune-mediated hepatitis [see [Warnings and Precautions](#)].
- Immune-mediated colitis [see [Warnings and Precautions](#)].
- Immune-mediated endocrinopathies [see [Warnings and Precautions](#)].
- Immune-mediated nephritis [see [Warnings and Precautions](#)].
- Immune-mediated rash [see [Warnings and Precautions](#)].
- Immune-mediated myocarditis [see [Warnings and Precautions](#)].
- Other immune-mediated adverse reactions [see [Warnings and Precautions](#)].
- Infusion-related reactions [see [Warnings and Precautions](#)].

Clinical Trials Experience

Because clinical trials are conducted under widely varying conditions, adverse reaction rates observed in the clinical trials of a drug cannot be directly compared to rates in the clinical trials of another drug and may not reflect the rates observed in practice.

The data described in the Warnings and Precautions section reflect exposure to IMFINZI in 1889 patients from the PACIFIC study (a randomized, placebo-controlled study that enrolled 475 patients with Stage III NSCLC), and Study 1108 (an open-label, single-arm, multicohort study that enrolled 970 patients with advanced solid tumours), and additional open-label, single-arm study that enrolled 444 patients with metastatic lung cancer, indications for which durvalumab is not approved. In these studies, IMFINZI was administered at a dose of 10 mg/kg every 2 weeks. Among the 1889 patients, 38% were exposed for 6 months or more and 18% were exposed for 12 months or more. The data also reflects exposure to IMFINZI 1500 mg every 4 weeks as a single agent in 262 patients from the ADRIATIC study (a randomized, double-blind study in patients with LS-SCLC) and to IMFINZI in combination with chemotherapy in 265 patients from the CASPIAN study (a randomized, open-label study in patients with ES-SCLC), in 338 patients from the TOPAZ-1 study (a randomized, double-blind study in patients with BTC). In the CASPIAN and TOPAZ-1 studies, IMFINZI was administered at a dose of 1500 mg every 3 or 4 weeks.

The data described in this section reflect exposure to IMFINZI in patients with Stage III NSCLC enrolled in the PACIFIC study, in patients with LS-SCLC enrolled in the ADRIATIC study, in patients with ES-SCLC enrolled in the CASPIAN study, in patients with BTC enrolled in the TOPAZ-1 study, in patients with endometrial cancer enrolled in the DUO-E study and in patients with MIBC enrolled in the NIAGARA study.

Summary of the safety profile

IMFINZI in combination with chemotherapy

The safety of IMFINZI in combination with chemotherapy is based on pooled data in 1 769 patients from 5 studies (TOPAZ-1, CASPIAN, DUO-E, AEGEAN and NIAGARA). The most common (> 10%) adverse reactions were neutropenia (41.7%), anaemia (40.8%), nausea (40.1%), fatigue (39.6%), constipation (29.7%), decreased appetite (22.2%), thrombocytopenia (21.5%), alopecia (19.7%), rash (19.7%), diarrhoea (18.2%), vomiting (16.8%), abdominal pain (16.7%), neuropathy peripheral (16.3%), leukopenia (14.8%), pyrexia (14.0%), pruritus (13.0%), hypothyroidism (11.9%), arthralgia (11.5%), cough/productive cough (11.0%), and aspartate aminotransferase increased/alanine aminotransferase increased (10.7%). The most common (> 2%) NCI CTCAE Grade $\geq$ 3 adverse reactions were neutropenia (25.2%), anaemia (13.7%), thrombocytopenia (6.9%), leukopenia (4.5%), fatigue (2.8%), pneumonia (2.4%) and febrile neutropenia (2.1%).

IMFINZI was discontinued due to adverse reactions in 6.2% of patients. The most common adverse reactions leading to treatment discontinuation were rash (0.7%), pneumonitis (0.7%) and fatigue (0.6%).

IMFINZI was delayed or interrupted due to adverse reactions in 29.2% of patients. The most common adverse reactions leading to dose delay or interruption were neutropenia (12.6%), thrombocytopenia (4.5%), anaemia (3.9%) and leukopenia (2.1%).

IMFINZI in combination with platinum-based chemotherapy followed by IMFINZI in combination with olaparib 300 mg twice daily

The safety of IMFINZI given in combination with platinum-based chemotherapy followed by IMFINZI in combination with olaparib 300 mg twice daily is based on data in 238 patients with endometrial cancer. The most common (> 20%) adverse reactions were anaemia (61.8%), nausea (54.6%), fatigue (54.2%), neuropathy peripheral (51.7%), alopecia (50.8%), neutropenia (39.5%), constipation (32.8%), thrombocytopenia (29.8%), diarrhoea (28.2%), vomiting (25.6%), arthralgia (24.4%), rash (23.5%), abdominal pain (23.5%), decreased appetite (23.1%) and leukopenia (20.2%).

The most common (> 2%) NCI CTCAE Grade ≥ 3 adverse reactions were neutropenia (25.2%), anaemia (23.5%), leukopenia (6.7%), thrombocytopenia (5.9%), fatigue (5.5%), febrile neutropenia (3.4%), nausea (2.9%), aspartate aminotransferase increased / alanine aminotransferase increased (2.9%) and neuropathy peripheral (2.5%).

IMFINZI was discontinued in 4.6% of patients. The most common adverse reaction leading to treatment discontinuation was pneumonitis (1.7%).

IMFINZI was interrupted in 38.2% of patients. The most common adverse reactions leading to dose interruption were anaemia (13.4%), thrombocytopenia (11.8%), neutropenia (10.1%), leukopenia (2.9%), hypothyroidism (2.1%) and upper respiratory tract infection (2.1%).

Non-Small Cell Lung Cancer

Neoadjuvant and Adjuvant Treatment of Resectable NSCLC – AEGEAN

The safety of IMFINZI in combination with neoadjuvant platinum-containing chemotherapy followed by surgery, and continued adjuvant treatment with IMFINZI as a single agent after surgery, was investigated in AEGEAN, a randomized, double-blind, placebo-controlled, multicenter study for patients with resectable NSCLC (Stage IIA to select Stage IIIB [AJC, 8th edition]); squamous or non-squamous) [see [Clinical Studies](#)].

Safety data are available for the 799 patients who received IMFINZI in combination with chemotherapy (n=401) or placebo in combination with chemotherapy (n=398).

The median duration of exposure to IMFINZI 1500 mg every 3 weeks in the neoadjuvant phase was 12 weeks (range: 0 to 19 weeks). The median duration of exposure to IMFINZI 1500 mg every 4 weeks in the adjuvant phase was 37 weeks (range: 4 to 67 weeks). The median age of patients who received IMFINZI was 65 years (range: 30 to 88), 52% age 65 or older, 12% age 75 or older; 65% male; 54% White, 41% Asian, 1% Black, 3% Other races; and 17% Hispanic or Latino.

The most common adverse reactions (occurring in $\geq 20\%$ of patients) were anemia, nausea, constipation, fatigue, musculoskeletal pain, and rash.

Table 3 summarizes the adverse reactions that occurred in ($\geq 10\%$) patients treated with IMFINZI in combination with chemotherapy.

Table 3. Adverse Reactions Occurring in $\geq 10\%$ of Patients in the AEGEAN Study

Adverse Reaction	IMFINZI with Chemotherapy N=401		Placebo with Chemotherapy N=398	
	All Grades (%)	Grade 3 or 4 (%)	All Grades (%)	Grade 3 or 4 (%)
Gastrointestinal disorders				
Nausea	25	0.2	29	0.3
Constipation	25	0.2	21	0
Diarrhea [*]	14	1.0	13	1.3
Vomiting	11	0.7	11	1.0
General disorders and administration site conditions				
Fatigue [†]	25	0	25	1.5
Skin and subcutaneous tissue disorders				
Rash [‡]	22	0.5	14	0.3
Pruritus	12	0.2	6	0
Musculoskeletal and connective tissue disorders				
Musculoskeletal pain [§]	24	1.0	29	0.5
Metabolism and nutrition disorders				
Decreased appetite	18	0.2	18	0.3
Nervous system disorders				
Peripheral neuropathy	16	0.5	22	0.8
Endocrine disorders				
Hypothyroidism [¶]	11	0	3.8	0
Respiratory, thoracic and mediastinal disorders				
Cough / Productive cough	11	0	13	0
Pneumonia ^{#,‡}	11	3.5	10	3.0
COVID-19 [§]	11	0.2	9	0.8
Psychiatric Disorders				
Insomnia	10	0	12	0

^{*} includes colitis, diarrhea, enteritis, and proctitis.

[†] includes fatigue and asthenia.

[‡] includes dermatitis, dermatitis acneiform, drug eruption, eczema, eczema asteatotic, erythema, palmar-erythrodysesthesia syndrome, pemphigoid, rash, rash erythematous, rash macular, rash maculo-papular, rash papular, rash pruritic, and rash pustular, skin exfoliation, and urticarial dermatitis.

[§] includes arthralgia, arthritis, back pain, bone pain, chest pain, musculoskeletal chest pain, musculoskeletal pain, musculoskeletal discomfort, musculoskeletal stiffness, myalgia, neck pain, non-cardiac chest pain, pain in extremity, and spinal pain.

^{||} includes dysaesthesia, hypoaesthesia, neuralgia, neuropathy peripheral, paraesthesia, peripheral sensory neuropathy, and polyneuropathy.

[¶] includes blood thyroid stimulating hormone increased and hypothyroidism.

[#] includes lower respiratory tract infection, lung abscess, paraneoplastic pneumonia, pneumonia, pneumonia aspiration, pneumonia bacterial, pneumonia chlamydial, pneumonia cryptococcal, pneumonia fungal, pneumonia pseudomonal, pneumonia streptococcal, pneumonia viral, and post-procedural pneumonia.

[‡] Five Grade 5 events in the IMFINZI arm and four Grade 5 events in the Placebo arm.

[§] Includes COVID-19 and COVID-19 Pneumonia. Five Grade 5 events in the IMFINZI arm and One Grade 5 event in the placebo arm.

Table 4 summarizes the laboratory abnormalities in patients treated with IMFINZI in combination with chemotherapy.

Table 4. Select Laboratory Abnormalities ($\geq 20\%$) That Worsened from Baseline in Patients with Disease Who Received IMFINZI with Chemotherapy in AEGEAN

Laboratory Abnormality [*]	IMFINZI with Chemotherapy [†]		Placebo with Chemotherapy [‡]	
	All Grades (%)	Grade 3 or 4 (%)	All Grades (%)	Grade 3 or 4 (%)
Hematology				
Hemoglobin decreased	78	10	75	9
Leukocytes decreased	63	12	64	11
Neutrophils decreased	52	24	56	27
Platelets decreased	46	7	44	8
Lymphocytes decreased	41	11	37	9
Chemistry				
Calcium corrected, decreased	51	3.3	52	4.5
Alanine aminotransferase increased	49	6	42	2
Aspartate aminotransferase increased	47	3.5	37	1.8
Potassium increased	33	1.5	29	2
Sodium decreased	35	5	33	6
Gamma glutamyl transferase increased	36	4.7	35	2.1
Creatinine increased	32	2.3	27	3.3
Amylase increased	25	4.7	24	3.6
Magnesium decreased	22	2.8	20	3.6
Lipase increased	23	4.9	24	7

* Graded per NCI CTCAE V5.

† The denominator used to calculate the rate varied from 349 to 399 based on the number of patients with a baseline value and at least one post-treatment value.

‡ The denominator used to calculate the rate varied from 333 to 398 based on the number of patients with a baseline value and at least one post-treatment value.

Neoadjuvant Phase of AEGEAN

A total of 401 patients received at least 1 dose of IMFINZI in combination with platinum-containing chemotherapy as neoadjuvant treatment and 398 patients received at least 1 dose of placebo in combination with platinum-containing chemotherapy as neoadjuvant treatment.

Serious adverse reactions occurred in 21% of patients who received IMFINZI in combination with platinum-containing chemotherapy as neoadjuvant treatment; the most frequent ($\geq 1\%$) serious adverse reactions were pneumonia (2.7%), anemia (1.5%), myelosuppression (1.5%), vomiting (1.2%), neutropenia (1%), and acute kidney injury (1%). Fatal adverse reactions occurred in 2% of patients, including death due to COVID-19 pneumonia (0.5%), sepsis (0.5%), myocarditis (0.2%), decreased appetite (0.2%), hemoptysis (0.2%), and death not otherwise specified (0.2%).

Permanent discontinuation of any study drug due to an adverse reaction occurred in 14% of patients who received IMFINZI in combination with platinum-containing chemotherapy as neoadjuvant treatment; the most frequent ($>0.5\%$) adverse reactions that led to permanent discontinuation of any study drug were anemia (1.5%), neutropenia (0.7%), myelosuppression (0.7%), and periphery sensory neuropathy (0.7%). Permanent discontinuation of IMFINZI due to an adverse reaction occurred in 6.7% of patients who received IMFINZI in combination with platinum-containing chemotherapy as neoadjuvant treatment; the most frequent ($\geq 0.5\%$) adverse reactions that led to permanent discontinuation of IMFINZI were peripheral sensory neuropathy (0.7%) and pneumonitis (0.5%).

Of the 401 IMFINZI-treated patients and 398 placebo-treated patients who received neoadjuvant treatment, 1.7% (n=7) and 1% (n=4), respectively, did not receive surgery due to adverse reactions. Adverse reactions that led to cancellation of surgery in the IMFINZI arm were COVID-19 pneumonia, HIV infection, pneumonitis, prostate cancer, colon cancer, pruritus, and colitis.

Of the 325 IMFINZI-treated patients who received surgery, 4% (n=15) experienced delay of surgery (a surgical delay is defined as on-study surgery occurring more than 40 days after the last dose of study treatment in the neoadjuvant period) due to adverse reactions. Of the 326 placebo-treated patients who received surgery, 4% (n=16) experienced delay of surgery due to adverse reactions.

Of the 325 IMFINZI-treated patients who received surgery, 6.5% (n=21) did not receive adjuvant treatment due to adverse reactions. Of the 326 placebo-treated patients who received surgery, 5.8% (n=19) did not receive adjuvant treatment due to adverse reactions.

Adjuvant Phase of AEGEAN

A total of 265 patients in the IMFINZI arm and 254 patients in the placebo arm received at least 1 dose of adjuvant treatment.

Of the patients who received single agent IMFINZI as adjuvant treatment, 13% experienced serious adverse reactions. The most frequent serious adverse reactions reported in $>1\%$ of patients were pneumonia (1.9%), pneumonitis (1.1%), and COVID-19 (1.1%). Four fatal adverse reactions occurred during the adjuvant phase of the study, including COVID-19 pneumonia, pneumonia aspiration, interstitial lung disease and aortic aneurysm. Permanent discontinuation of adjuvant IMFINZI due to an adverse reaction occurred in 8% of patients. The most frequent ($\geq 0.5\%$) adverse reaction that led to permanent discontinuation of adjuvant IMFINZI was pneumonitis (1.1%) and rash (0.8%).

Unresectable Stage III NSCLC - PACIFIC

The safety of IMFINZI in patients with Stage III NSCLC who completed concurrent platinum-based chemoradiotherapy within 42 days prior to initiation of study drug was evaluated in the PACIFIC study, a multicenter, randomized, double-blind, placebo-controlled study. A total of 475 patients received IMFINZI 10 mg/kg intravenously every 2 weeks. The study excluded patients who had disease progression following chemoradiation, with active or prior autoimmune disease within 2 years of initiation of the study or with medical conditions that required systemic immunosuppression [see [Clinical Studies](#)].

The study population characteristics were: median age of 64 years (range: 23 to 90), 45% age 65 years or older, 70% male, 69% White, 27% Asian, 75% former smoker, 16% current

smoker, and 51% had WHO performance status (PS) of 1. All patients received definitive radiotherapy as per protocol, of which 92% received a total radiation dose of 54 Gy to 66 Gy. The median duration of exposure to IMFINZI was 10 months (range: 0.2 to 12.6).

IMFINZI was discontinued due to adverse reactions in 15% of patients. The most common adverse reactions leading to IMFINZI discontinuation were pneumonitis or radiation pneumonitis in 6% of patients. Serious adverse reactions occurred in 29% of patients receiving IMFINZI. The most frequent serious adverse reactions reported in at least 2% of patients were pneumonitis or radiation pneumonitis (7%) and pneumonia (6%). Fatal pneumonitis or radiation pneumonitis and fatal pneumonia occurred in < 2% of patients and were similar across arms. The most common adverse reactions (occurring in $\geq 20\%$ of patients) were cough, fatigue, pneumonitis or radiation pneumonitis, upper respiratory tract infections, dyspnea and rash.

Table 5 summarizes the adverse reactions that occurred in at least 10% of patients treated with IMFINZI.

Table 5. Adverse Reactions Occurring in $\geq 10\%$ Patients in the PACIFIC Study

Adverse Reaction	IMFINZI N=475		Placebo ¹ N=234	
	All Grades (%)	Grades 3-4 (%)	All Grades (%)	Grades 3-4 (%)
Respiratory, Thoracic and Mediastinal Disorders				
Cough/Productive Cough	40	0.6	30	0.4
Pneumonitis ² /Radiation Pneumonitis	34	3.4	25	3
Dyspnea ³	25	1.5	25	2.6
Gastrointestinal Disorders				
Diarrhea	18	0.6	19	1.3
Abdominal pain ⁴	10	0.4	6	0.4
Endocrine Disorders				
Hypothyroidism ⁵	12	0.2	1.7	0
Skin and Subcutaneous Tissue Disorders				
Rash ⁶	23	0.6	12	0
Pruritus ⁷	12	0	6	0
General Disorders				
Fatigue ⁸	34	0.8	32	1.3
Pyrexia	15	0.2	9	0
Infections				
Upper respiratory tract infections ⁹	26	0.4	19	0
Pneumonia ¹⁰	17	7	12	6

¹ The PACIFIC study was not designed to demonstrate statistically significant difference in adverse reaction rates for IMFINZI, as compared to placebo, for any specific adverse reaction listed in Table 3

² Includes acute interstitial pneumonitis, interstitial lung disease, pneumonitis, pulmonary fibrosis

³ Includes dyspnea and exertional dyspnea

⁴ Includes abdominal pain, abdominal pain lower, abdominal pain upper, and flank pain

⁵ Includes autoimmune hypothyroidism and hypothyroidism

⁶ Includes rash erythematous, rash generalized, rash macular, rash maculopapular, rash papular, rash pruritic, rash pustular, erythema, eczema, rash and dermatitis

⁷ Includes pruritus generalized and pruritus

⁸ Includes asthenia and fatigue

⁹ Includes laryngitis, nasopharyngitis, peritonsillar abscess, pharyngitis, rhinitis, sinusitis, tonsillitis, tracheobronchitis, and upper respiratory tract infection

¹⁰ Includes lung infection, pneumocystis jirovecii pneumonia, pneumonia, pneumonia adenoviral, pneumonia bacterial, pneumonia cytomegaloviral, pneumonia haemophilus, pneumonia klebsiella, pneumonia necrotising, pneumonia pneumococcal, and pneumonia streptococcal

Other adverse reactions occurring in less than 10% of patients treated with IMFINZI were dysphonia, dysuria, night sweats, peripheral edema, and increased susceptibility to infections. Table 6 summarizes the laboratory abnormalities that occurred in at least 20% of patients treated with IMFINZI.

Table 6. Laboratory Abnormalities Worsening From Baseline Occurring in $\geq 20\%$ of Patients in the PACIFIC Study

Laboratory Abnormality	IMFINZI		Placebo	
	All Grades ¹ (%) ²	Grade 3 or 4 (%)	All Grades ¹ (%) ²	Grade 3 or 4 (%)
Chemistry				
Hyperglycemia	52	8	51	8
Hypocalcemia	46	0.2	41	0
Increased ALT	39	2.3	22	0.4
Increased AST	36	2.8	21	0.4
Hyponatremia	33	3.6	30	3.1
Hyperkalemia	32	1.1	29	1.8
Increased GGT	24	3.4	22	1.7
Hematology				
Lymphopenia	43	17	39	18

¹ Graded according to NCI CTCAE version 4.0

² Each test incidence is based on the number of patients who had both baseline and at least one on-study laboratory measurement available: IMFINZI (range: 464 to 470) and placebo (range: 224 to 228)

Small Cell Lung Cancer

Limited Stage Small Cell Lung Cancer – ADRIATIC

The safety of IMFINZI as a single agent in patients with LS-SCLC without disease progression following completion of concurrent platinum-based chemoradiotherapy (60-66 Gy once daily over 6 weeks or 45 Gy twice daily over 3 weeks) within 42 days prior to initiation of study drug, was evaluated in the ADRIATIC study, a multicenter, randomized, double-blind, placebo-controlled study [[see Clinical Studies \(14.2\)](#)]. A total of 262 patients received IMFINZI 1,500 mg every 4 weeks until disease progression or unacceptable toxicity or a maximum of 24 months. The study excluded patients with Stage I or II LS-SCLC who were considered medically operable and patients with active or prior autoimmune disease or with medical conditions that required systemic corticosteroids or immunosuppressants.

The study population characteristics were: median age of 62 years (range: 28 to 84); 39% age 65 years or older, 6% age 75 years or older; 69% male; 50% white, 48% Asian, 1.3% other races; 4.2% Hispanic or Latino; 68% former smoker, 22% current smoker; and 51% had WHO performance status of 1. Sixty-seven percent of patients received a total radiation dose of 60 Gy to 66 Gy once daily and 27% of patients received a total radiation dose of 45 Gy twice daily.

The median duration of exposure to IMFINZI was 9.2 months (range: 0.92 to 25) in the IMFINZI arm.

Serious adverse reactions occurred in 30% of patients receiving IMFINZI. The most frequent serious adverse reactions reported in $\geq 1\%$ of patients receiving IMFINZI were pneumonitis or radiation pneumonitis (12%), and pneumonia (5%). Fatal adverse reactions occurred in 2.7% of patients who received IMFINZI including pneumonia (1.5%), cardiac failure, encephalopathy and pneumonitis (0.4% each). Permanent discontinuation of IMFINZI due to adverse reactions occurred in 16% of the patients. Adverse reactions which resulted in permanent discontinuation of IMFINZI in $\geq 1\%$ of patients included pneumonitis or radiation pneumonitis (9%) and pneumonia (1.5%). Dosage interruptions of IMFINZI due to an adverse reaction occurred in 35% of patients. Adverse reactions which required dosage interruption in $\geq 5\%$ of patients included pneumonitis or radiation pneumonitis (17%). The most common adverse reactions occurring in $\geq 20\%$ of patients receiving IMFINZI were pneumonitis or radiation pneumonitis (38%), and fatigue (21%).

Table 7 summarizes the adverse reactions that occurred in patients treated with IMFINZI in the ADRIATIC study.

Table 7. Adverse Reactions ($\geq 10\%$) in Patients with LS-SCLC Who Received IMFINZI in the ADRIATIC Study

	IMFINZI (N=262)		Placebo (N=265)	
Adverse Reaction	All Grades (%)	Grade 3 or 4 (%)	All Grades (%)	Grade 3 or 4 (%)
Respiratory, thoracic and mediastinal disorders				
Pneumonitis or Radiation pneumonitis*	38	3.1	30	2.6
Cough/Productive cough	17	0	14	0
Dyspnea†	11	0.4	7	0
General disorders				
Fatigue‡	21	0.4	20	2.3
Skin and subcutaneous tissue disorders				
Rash‡	18	0.4	11	0
Pruritus	13	0	7	0
Endocrine disorders				
Hypothyroidism¶	17	0	4.9	0
Hyperthyroidism#	12	0	1.9	0
Metabolism and nutrition disorders				

	IMFINZI (N=262)		Placebo (N=265)	
Adverse Reaction	All Grades (%)	Grade 3 or 4 (%)	All Grades (%)	Grade 3 or 4 (%)
Decreased appetite	17	0	13	0
Nervous system disorders				
Dizziness [§]	14	0	9	0
Infections and infestations				
Pneumonia [§]	13	3.1	9	4.2
Gastrointestinal disorders				
Nausea	13	0	11	0
Diarrhea	11	1.9	8	0
Constipation	10	0	10	0

* Includes pneumonitis, immune-mediated lung disease, interstitial lung disease, radiation pneumonitis, and lung radiation fibrosis.

† Includes dyspnea and exertional dyspnea.

‡ Includes fatigue and asthenia.

‡ Includes dermatitis, acneiform dermatitis, eczema, rash, maculo-papular rash, papular rash, pruritic rash, and skin exfoliation.

¶ Includes hypothyroidism, increased blood thyroid stimulating hormone, and decreased thyroxine free.

Includes hyperthyroidism, decreased blood thyroid stimulating hormone, increased thyroxine free, increased thyroxine, increased tri-iodothyronine free, and increased tri-iodothyronine.

§ Includes pneumonia, atypical pneumonia, lower respiratory tract infection, bacterial pneumonia, pneumocystis jirovecii pneumonia, legionella pneumonia, and viral pneumonia.

§ Includes dizziness, postural dizziness, vertigo, and positional vertigo.

Table 8 summarizes the laboratory abnormalities that occurred in at least 20% of patients treated with IMFINZI.

Table 8. Select Laboratory Abnormalities (≥ 20%) That Worsened from Baseline in Patients with LS-SCLC Who Received IMFINZI in the ADRIATIC Study

Laboratory Abnormality*	IMFINZI†		Placebo‡	
	All Grades (%)	Grade 3 or 4 (%)	All Grades (%)	Grade 3 or 4 (%)
Chemistry				
Hypocalcemia	43	0	43	0.8
Hyperglycemia	38	3.2	45	1.5
ALT increased	36	2.3	29	2.3
AST increased	33	2.3	28	1.5
Gamma Glutamyl Transferase increased	32	7	27	2.9

Laboratory Abnormality*	IMFINZI†		Placebo‡	
	All Grades (%)	Grade 3 or 4 (%)	All Grades (%)	Grade 3 or 4 (%)
Hyponatremia	32	5	29	6.2
Hyperkalemia	23	1.2	17	0.8
Creatinine increased*	21	0	17	0.8
Hematology				
Lymphocytes decreased	34	10	33	10
Leukocytes decreased	26	0.4	33	1.1

* Graded according to NCI CTCAE version 4.03, except creatinine increased which is graded according to NCI CTCAE version 5.0.

† The denominator used to calculate the rate varied from 63 to 259 based on the number of patients with a baseline value and at least one post-treatment value.

‡ The denominator used to calculate the rate varied from 65 to 262 based on the number of patients with a baseline value and at least one post-treatment value.

Extensive Stage Small Cell Lung Cancer - CASPIAN

The safety of IMFINZI in combination with etoposide and either carboplatin or cisplatin in previously untreated ES-SCLC was evaluated in CASPIAN, a randomized, open-label, multicenter, active-controlled study. A total of 265 patients received IMFINZI 1500 mg in combination with chemotherapy every 3 weeks for 4 cycles followed by IMFINZI 1500 mg every 4 weeks until disease progression or unacceptable toxicity. The study excluded patients with active or prior autoimmune disease or with medical conditions that required systemic corticosteroids or immunosuppressants [see [Clinical Studies](#)]. Among 265 patients receiving IMFINZI, 49% were exposed for 6 months or longer and 19% were exposed for 12 months or longer.

Among 266 patients receiving chemotherapy alone, 57% of the patients received 6 cycles of chemotherapy and 8% of the patients received PCI after chemotherapy.

IMFINZI was discontinued due to adverse reactions in 7% of the patients receiving IMFINZI plus chemotherapy. These include pneumonitis, hepatotoxicity, neurotoxicity, sepsis, diabetic ketoacidosis and pancytopenia (1 patient each). Serious adverse reactions occurred in 31% of patients receiving IMFINZI plus chemotherapy. The most frequent serious adverse reactions reported in at least 1% of patients were febrile neutropenia (4.5%), pneumonia (2.3%), anemia (1.9%), pancytopenia (1.5%), pneumonitis (1.1%) and COPD (1.1%). Fatal adverse reactions occurred in 4.9% of patients receiving IMFINZI plus chemotherapy. These include pancytopenia, sepsis, septic shock, pulmonary artery thrombosis, pulmonary embolism, and hepatitis (1 patient each) and sudden death (2 patients). The most common adverse reactions (occurring in $\geq 20\%$ of patients) were nausea, fatigue/asthenia and alopecia.

Table 9 summarizes the adverse reactions that occurred in patients treated with IMFINZI plus chemotherapy.

Table 9. Adverse Reactions Occurring in $\geq 10\%$ Patients in the CASPIAN study

	IMFINZI with etoposide and either carboplatin or cisplatin N = 265		Etoposide and either carboplatin or cisplatin N = 266	
Adverse Reaction	All Grades (%)	Grade 3-4 (%)	All Grades (%)	Grade 3-4 (%)
Respiratory, thoracic and mediastinal disorders				
Cough/Productive Cough	15	0.8	9	0
Gastrointestinal disorders				
Nausea	34	0.4	34	1.9
Constipation	17	0.8	19	0
Vomiting	15	0	17	1.1
Diarrhea	10	1.1	11	1.1
Endocrine disorders				
Hyperthyroidism ^a	10	0	0.4	0
Skin and subcutaneous tissue disorders				
Alopecia	31	1.1	34	0.8
Rash ^b	11	0	6	0
General disorders and administration site conditions				
Fatigue/Asthenia	32	3.4	32	2.3
Metabolism and nutrition disorders				
Decreased appetite	18	0.8	17	0.8

^a Includes hyperthyroidism and Basedow's disease

^b Includes rash erythematous, rash generalized, rash macular, rash maculopapular, rash papular, rash pruritic, rash pustular, erythema, eczema, rash and dermatitis

Table 10 summarizes the laboratory abnormalities that occurred in at least 20% of patients treated with IMFINZI plus chemotherapy.

Table 10. Laboratory Abnormalities Worsening from Baseline Occurring in $\geq 20\%$ ¹ of Patients in the CASPIAN study

	IMFINZI with Etoposide and either Carboplatin or Cisplatin	Etoposide and either Carboplatin or Cisplatin
Laboratory Abnormality	Grade² 3 or 4 (%)³	Grade² 3 or 4 (%)³
Chemistry		
Hyponatremia	11	13
Hypomagnesemia	11	6
Hyperglycemia	5	5
Increased Alkaline Phosphatase	4.9	3.5
Increased ALT	4.9	2.7
Increased AST	4.6	1.2
Hypocalcemia	3.5	2.4
Blood creatinine increased	3.4	1.1
Hyperkalemia	1.5	3.1

TSH decreased < LLN ⁴ and ≥ LLN at baseline	NA	NA
Hematology		
Neutropenia	41	48
Lymphopenia	14	13
Anemia	13	22
Thrombocytopenia	12	15

¹ The frequency cut off is based on any grade change from baseline

² Graded according to NCI CTCAE version 4.03

³ Each test incidence is based on the number of patients who had both baseline and at least one on-study laboratory measurement available: IMFINZI (range: 258 to 263) and chemotherapy (range: 253 to 262) except magnesium IMFINZI + chemotherapy (18) and chemotherapy (16)

⁴ LLN = lower limit of normal

Biliary Tract Cancer-TOPAZ-1

The safety of IMFINZI in combination with gemcitabine and cisplatin in locally advanced or metastatic BTC was evaluated in TOPAZ-1, a randomized, double-blind, placebo-controlled, multicenter trial. A total of 338 patients received IMFINZI 1,500 mg in combination with gemcitabine and cisplatin every 3 weeks up to 8 cycles followed by IMFINZI 1,500 mg every 4 weeks until disease progression or unacceptable toxicity. Patients with active or prior documented autoimmune or inflammatory disorders, HIV infection or other active infections, including tuberculosis or hepatitis C were ineligible [see [Clinical Studies](#)].

IMFINZI was discontinued due to adverse reactions in 6% of the patients receiving IMFINZI plus chemotherapy. The most frequently reported events resulting in discontinuation were sepsis (3 patients) and ischemic stroke (2 patients). The remaining events were dispersed across system organ classes and reported in 1 patient each. Serious adverse reactions occurred in 47% of patients receiving IMFINZI plus chemotherapy. The most frequent serious adverse reactions reported in at least 2% of patients were cholangitis (7%), pyrexia (3.8%), anemia (3.6%), sepsis (3.3%) and acute kidney injury (2.4%). Fatal adverse reactions occurred in 3.6% of patients receiving IMFINZI plus chemotherapy. These include ischemic or hemorrhagic stroke (4 patients), sepsis (2 patients), upper gastrointestinal hemorrhage (2 patients). The most common adverse reactions (occurring in ≥ 20% of patients) were fatigue, nausea, constipation, decreased appetite, abdominal pain, rash and pyrexia. Table 11 summarizes the adverse reactions that occurred in patients treated with IMFINZI plus chemotherapy.

Table 11. Adverse Reactions Occurring in $\geq 10\%$ of Patients in the TOPAZ-1 Study

	IMFINZI with Gemcitabine and Cisplatin N = 338		Placebo with Gemcitabine and Cisplatin N = 342	
Adverse Reaction	All Grades* (%)	Grade* 3-4 (%)	All Grades* (%)	Grade* 3-4 (%)
General disorders and administration site conditions				
Fatigue [†]	42	6	43	6
Pyrexia	20	1.5	16	0.6
Gastrointestinal disorders				
Nausea	40	1.5	34	1.8
Constipation	32	0.6	29	0.3
Abdominal pain [‡]	24	0.6	23	2.9
Vomiting	18	1.5	18	2.0
Diarrhea	17	1.2	15	1.8
Metabolism and nutrition disorders				
Decreased appetite	26	2.1	23	0.9
Skin and subcutaneous tissue disorders				
Rash [§]	23	0.9	14	0
Pruritus	11	0	8	0
Psychiatric disorders				
Insomnia	10	0	11	0

* Graded according to NCI CTCAE version 5.0

[†] Includes fatigue, malaise, cancer fatigue and asthenia.

[‡] Includes abdominal pain, abdominal pain lower, abdominal pain upper and flank pain.

[§] Includes rash macular, rash maculopapular, rash morbilliform, rash papular, rash pruritic, rash pustular, rash erythematous, dermatitis acneiform, dermatitis bullous, drug eruption, eczema, erythema, dermatitis and rash.

Table 12 summarizes the laboratory abnormalities in patients treated with IMFINZI plus chemotherapy.

Table 12. Laboratory Abnormalities Worsening from Baseline Occurring in $\geq 20\%$ * of Patients in the TOPAZ-1 study

	IMFINZI with Gemcitabine and Cisplatin	Placebo with Gemcitabine and Cisplatin
Laboratory Abnormality	Grade[†] 3 or 4 (%)	Grade[†] 3 or 4 (%)
Chemistry		
Hyponatremia	18	13
Gamma-glutamyltransferase increased	12	13
Increased bilirubin	10	14
Hypokalemia	8	4.4
Increased AST	8	8
Increased ALT	7	6
Blood creatinine increased	5	2.1
Hypomagnesemia	4.5	2.2
Hypoalbuminemia	3.6	2.9
Hyperkalemia	2.1	2.1
Increased Alkaline Phosphatase	1.8	3.8
Hypocalcemia	1.8	2.4
Hematology		

	IMFINZI with Gemcitabine and Cisplatin	Placebo with Gemcitabine and Cisplatin
Laboratory Abnormality	Grade [†] 3 or 4 (%)	Grade [†] 3 or 4 (%)
Neutropenia	48	49
Anemia	31	28
Leukopenia	28	28
Lymphopenia	23	15
Thrombocytopenia	18	18

* The frequency cut off is based on any grade change from baseline

† Graded according to NCI CTCAE version 5.0. Each test incidence is based on the number of patients who had both baseline and at least one on-study laboratory measurement available: IMFINZI + Gem/Cis (range: 312 to 335) and Placebo + Gem/Cis (range: 319 to 341).

Endometrial Cancer

Advanced or Recurrent Endometrial Cancer – DUO-E

Adverse reactions known to occur with durvalumab, chemotherapy or olaparib given alone, may occur during treatment with these medicinal products in combination, even if these reactions were not reported in clinical trials with combination therapy. Please refer to the local prescribing information for chemotherapy and olaparib.

Table 13. Adverse drug reactions in patients treated with IMFINZI in combination with platinum-based chemotherapy followed by IMFINZI as monotherapy (n=235) or in combination with olaparib (n=238)

Adverse Reactions	Platinum-based chemotherapy + IMFINZI + olaparib* (n=238)		Platinum-based chemotherapy + IMFINZI** (n=235)		Platinum-based chemotherapy (n=236)	
	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)
Blood and lymphatic system disorders						
Anaemia ^a	147 (61.8) Very common	56 (23.5) Very common	111 (47.2) Very common	38 (16.2) Very common	128 (54.2) Very common	35 (14.8) Very common
Neutropenia ^a	94 (39.5) Very common	60 (25.2) Very common	76 (32.3) Very common	45 (19.1) Very common	91 (38.6) Very common	49 (20.8) Very common
Thrombocytopenia ^a	71 (29.8) Very common	14 (5.9) Common	66 (28.1) Very common	16 (6.8) Common	52 (22.0) Very common	11 (4.7) Common
Leukopenia ^a	48 (20.2) Very common	16 (6.7) Common	40 (17.0) Very common	11 (4.7) Common	45 (19.1) Very common	13 (5.5) Common
Lymphopenia ^b	17 (7) Common	5 (2) Common	N/A 17 (7) Common	N/A 7 (3) Common	N/A 17 (7) Common	N/A 6 (3) Common
Febrile neutropenia ^a	8 (3.4) Common	8 (3.4) Common	7 (3.0) Common	6 (2.6) Common	10 (4.2) Common	9 (3.8) Common

Adverse Reactions	Platinum-based chemotherapy + IMFINZI + olaparib* (n=238)		Platinum-based chemotherapy + IMFINZI** (n=235)		Platinum-based chemotherapy (n=236)	
	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)
Aplasia pure red cell	3 (1.3) Common	3 (1.3) Common	0	0	0	0
Pancytopenia ^a	2 (0.8) Uncommon	1 (0.4) Uncommon	0	0	1 (0.4) Uncommon	1 (0.4) Uncommon
Immune thrombocytopenia	0	0	1 (0.4) Uncommon	1 (0.4) Uncommon	0	0
Immune system disorders						
Hypersensitivity ^{b,c}	11 (5) Common	2 (0.8) Uncommon	N/A 6 (3) Common	N/A 1 (0.4) Uncommon	N/A 6 (3) Common	N/A 1 (0.4) Uncommon
Vascular disorders						
Venous thromboembolic events ^{b,d}	20 (8) Common	8 (3) Common	19 (8) Common	6 (3) Common	18 (8) Common	7 (3) Common
Endocrine disorders						
Hypothyroidism ^e	37 (15.5) Very common	0	39 (16.6) Very common	0	11 (4.7) Common	0
Hyperthyroidism ^f	16 (6.7) Common	0	18 (7.7) Common	0	4 (1.7) Common	0
Thyroiditis ^g	6 (2.5) Common	0	4 (1.7) Common	0	0	0
Adrenal insufficiency	0	0	1 (0.4) Uncommon	0	0	0
Type 1 diabetes mellitus	0	0	1 (0.4) Uncommon	1 (0.4) Uncommon	0	0
Eye disorders						
Uveitis	1 (0.4) Uncommon	0	2 (0.9) Uncommon	0	0	0
Gastrointestinal disorders						
Nausea ^a	130 (54.6) Very common	7 (2.9) Common	96 (40.9) Very common	1 (0.4) Uncommon	105 (44.5) Very common	3 (1.3) Common
Constipation ^a	78 (32.8) Very common	0	64 (27.2) Very common	2 (0.9) Uncommon	81 (34.3) Very common	5 (2.1) Common
Diarrhoea	67 (28.2) Very common	3 (1.3) Common	74 (31.5) Very common	4 (1.7) Common	66 (28.0) Very common	6 (2.5) Common
Vomiting ^a	61 (25.6)	1 (0.4)	49 (20.9)	4 (1.7) Common	43 (18.2)	3 (1.3) Common

Adverse Reactions	Platinum-based chemotherapy + IMFINZI + olaparib* (n=238)		Platinum-based chemotherapy + IMFINZI** (n=235)		Platinum-based chemotherapy (n=236)	
	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)
	Very common	Uncommon	Very common		Very common	
Abdominal pain ^h	56 (23.5) Very common	3 (1.3) Common	59 (25.1) Very common	1 (0.4) Uncommon	59 (25.0) Very common	3 (1.3) Common
Stomatitis ^a	27(11.3) Very common	0	21 (8.9) Common	0	23 (9.7) Common	0
Dyspepsia ^b	22 (9) Common	0	N/A 12 (5) Common	N/A 0	N/A 18 (8) Common	N/A 0
Colitis ⁱ	4 (1.7) Common	2 (0.8) Uncommon	5 (2.1) Common	2 (0.9) Uncommon	1 (0.4) Uncommon	0
General disorders and administration site conditions						
Fatigue ^a	129 (54.2) Very common	13 (5.5) Common	101 (43.0) Very common	8 (3.4) Common	105 (44.5) Very common	7 (3.0) Common
Peripheral oedema ^j	39 (16.4) Very common	1 (0.4) Uncommon	36 (15.3) Very common	2 (0.9) Uncommon	27 (11.4) Very common	1 (0.4) Uncommon
Pyrexia	24 (10.1) Very common	0	21 (8.9) Common	1 (0.4) Uncommon	18 (7.6) Common	1 (0.4) Uncommon
Hepatobiliary disorders						
AST/ALT increased ^k	35 (14.7) Very common	7 (2.9) Common	31 (13.2) Very common	4 (1.7) Common	26 (11.0) Very common	3 (1.3) Common
Hepatitis ^l	2 (0.8) Uncommon	1 (0.4) Uncommon	2 (0.9) Uncommon	1 (0.4) Uncommon	1 (0.4) Uncommon	0
Infections and infestations						
Upper respiratory tract infection ^m	24 (10.1) Very common	0	21 (8.9) Common	0	24 (10.2) Very common	0
Pneumonia ⁿ	7 (2.9) Common	3 (1.3) Common	2 (0.9) Uncommon	2 (0.9) Uncommon	1 (0.4) Uncommon	1 (0.4) Uncommon
Oral candidiasis	4 (1.7) Common	0	1 (0.4) Uncommon	0	0	0
Dental and oral soft tissue infections ^o	6 (2.5) Common	1 (0.4) Uncommon	9 (3.8) Common	1 (0.4) Uncommon	4 (1.7) Common	0
Influenza	1 (0.4) Uncommon	0	3 (1.3) Common	0	1 (0.4) Uncommon	0

Adverse Reactions	Platinum-based chemotherapy + IMFINZI + olaparib* (n=238)		Platinum-based chemotherapy + IMFINZI** (n=235)		Platinum-based chemotherapy (n=236)	
	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)
Injury, poisoning and procedural complications						
Infusion related reaction ^p	14 (5.9) Common	2 (0.8) Uncommon	15 (6.4) Common	0	24 (10.2) Very common	3 (1.3) Common
Metabolism and nutrition disorders						
Decreased appetite ^a	55 (23.1) Very common	3 (1.3) Common	42 (17.9) Very common	0	46 (19.5) Very common	0
Musculoskeletal and connective tissue disorders						
Myalgia	30 (12.6) Very common	0	32 (13.6) Very common	0	44 (18.6) Very common	1 (0.4) Uncommon
Myositis	1 (0.4) Uncommon	0	6 (2.6) Common	1 (0.4) Uncommon	0	0
Immune-mediated arthritis ^q	0	0	1 (0.4) Uncommon	0	0	0
Nervous system disorders						
Neuropathy peripheral ^a	123 (51.7) Very common	6 (2.5) Common	126 (53.6) Very common	3 (1.3) Common	138 (58.5) Very common	7 (3.0) Common
Dizziness ^b	40 (17) Very common	1 (0.4) Uncommon	N/A 32 (14) Very common	N/A 0	N/A 31 (13) Very common	N/A 0
Headache ^b	39 (16) Very common	0	N/A 30 (13) Very common	N/A 0	N/A 35 (15) Common	N/A 0
Dysgeusia ^{b,r}	30 (13) Very common	0	N/A 26 (11) Very common	N/A 0	N/A 31 (13) Very common	N/A 0
Myasthenia gravis	0	0	2 (0.9) Uncommon	0	0	0
Renal and urinary disorders						
Blood creatinine increased	24 (10.1) Very common	0	10 (4.3) Common	0	13 (5.5) Common	1 (0.4) Uncommon
Dysuria	10 (4.2) Common	0	10 (4.3) Common	0	12 (5.1) Common	0
Nephritis ^s	0	0	0	0	1 (0.4) Uncommon	0
Respiratory, thoracic and mediastinal disorders						

Adverse Reactions	Platinum-based chemotherapy + IMFINZI + olaparib* (n=238)		Platinum-based chemotherapy + IMFINZI** (n=235)		Platinum-based chemotherapy (n=236)	
	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)	Frequency of any Grade (%)	Frequency of Grade 3-4 (%)
Cough / productive cough	33 (13.9) Very common	0	37 (15.7) Very common	0	29 (12.3) Very common	0
Dyspnoea ^{b,t}	34 (14) Very common	2 (0.8) Uncommon	N/A 33 (14) Very common	N/A 2 (0.9) Uncommon	N/A 32 (14) Very common	N/A 1 (0.4) Uncommon
Pneumonitis	10 (4.2) Common	3 (1.3) Common	3 (1.3) Common	1 (0.4) Uncommon	1 (0.4) Uncommon	9
Dysphonia	4 (1.7) Common	0	2 (0.9) Uncommon	0	3 (1.3) Common	0
Interstitial lung disease	2 (0.8) Uncommon	1 (0.4) Uncommon	1 (0.4) Uncommon	1 (0.4) Uncommon	0	0
Skin and subcutaneous tissue disorders						
Alopecia ^a	121 (50.8) Very common	0	118 (50.2) Very common	0	118 (50.0) Very common	0
Rash ^{a,u}	56 (23.5) Very common	2 (0.8) Uncommon	62 (26.4) Very common	4 (1.7) Common	48 (20.3) Very common	3 (1.3) Common
Pruritus	37 (15.5) Very common	2 (0.8) Uncommon	36 (15.3) Very common	0	29 (12.3) Very common	0
Dermatitis ^{a,v}	4 (1.7) Common	0	8 (3.4) Common	2 (0.9) Uncommon	1 (0.4) Common	0
Night sweats	2 (0.8) Uncommon	0	1 (0.4) Uncommon	0	2 (0.8) Uncommon	0
Pemphigoid ^w	0	0	1 (0.4) Uncommon	0	1 (0.4) Uncommon	0

* Overall study of induction treatment with up to six 21-day cycles with platinum-based chemotherapy in combination with IMFINZI, followed by maintenance treatment with IMFINZI in combination with Olaparib.

** Overall study of induction treatment with up to six 21-day cycles with platinum-based chemotherapy in combination with IMFINZI, followed by maintenance treatment with IMFINZI as monotherapy.

^a Adverse reaction only applies to chemotherapy ADRs in the DUO-E study

^b Adverse reaction only applies to olaparib ADRs in the DUO-E study

^c Includes drug hypersensitivity and hypersensitivity.

^d Includes deep vein thrombosis, embolism, embolism venous, pelvic venous thrombosis, superficial venous thrombosis and thrombosis.

^e Includes blood thyroid stimulating hormone increased and hypothyroidism.

^f Includes blood thyroid stimulating hormone decreased and hyperthyroidism.

^g Includes autoimmune thyroiditis, immune-mediated thyroiditis and thyroiditis.

^h Includes abdominal pain, abdominal pain lower, flank pain and abdominal pain upper.

ⁱ Includes colitis, enterocolitis, and proctitis.

^j Includes peripheral edema and peripheral swelling.

^k Includes ALT increased, AST increased, and hepatic enzymes increased.

^l Includes hepatitis, immune-mediated hepatitis, and hepatotoxicity.

^m Includes nasopharyngitis, pharyngitis, rhinitis, sinusitis, tracheobronchitis, and upper respiratory tract infection.

ⁿ Includes pneumonia and pneumonia bacterial.

^o Includes gingivitis, pulpitis dental, tooth abscess, and tooth infection.

^p Includes infusion related reaction and urticaria.

^q Reported as rheumatoid arthritis.

^r Includes dysgeusia and taste disorder.

^s Reported as immune-mediated nephritis.

^t Includes dyspnoea and dyspnoea exertional.

^u Includes eczema, erythema, rash, rash erythematous, rash macular, rash maculo-papular, rash papular, rash pruritic and rash pustular.

^v Includes dermatitis and immune-mediated dermatitis.

^w Reported as dermatitis bullous.

Table 14. Laboratory Abnormalities Worsening from Baseline Occurring in $\geq 20\%$ of Patients in the DUO-E study

	Platinum-based chemotherapy + IMFINZI + Olaparib	Platinum-based Chemotherapy + IMFINZI	Platinum-based Chemotherapy
Laboratory Abnormality	Grade[†]3 or 4 (%)[‡]	Grade[†]3 or 4 (%)[‡]	Grade[†]3 or 4 (%)[‡]
Laboratory Abnormality			
Hyponatremia	3.8	6	4.8
Hypomagnesemia	1.8	3.3	1.9
Hypermagnesemia	0.9	2.3	0.9
Hypokalemia	5	3.9	1.7
Creatinine increased	1.7	0.4	1.7
Increased ALT	3.8	3.5	1.3
Increased AST	3.4	3.0	0.4
Increased GGT	4.2	4.7	2.9
Increased Alkaline phosphatase	0.4	0.4	0.4
Haematology			
Neutropenia	28	22	23
Leukopenia	21	15	16
Lymphocytopenia	24	11	16
Anaemia	25	17	14
Thrombocytopenia	7	9	6

* The frequency cut off is based on any grade change from baseline.

[†] Graded according to NCI CTCAE version 5.0.

[‡] Each test incidence is based on the number of patients who had both baseline and at least one on-study laboratory measurement available: chemotherapy + IMFINZI + olaparib (range: 215 to 238), chemotherapy + IMFINZI (range: 202 to 231), and chemotherapy (range: 205 to 231).

Muscle Invasive Bladder Cancer

Neoadjuvant and adjuvant treatment of MIBC – NIAGARA

The safety of IMFINZI in combination with neoadjuvant gemcitabine and cisplatin followed by surgery and continued IMFINZI treatment as adjuvant, single-agent therapy was evaluated

in NIAGARA, a randomized, open-label, multicenter trial. Patients received IMFINZI in combination with chemotherapy (n=530) or received chemotherapy alone (n=526) [see [Clinical Studies](#)].

The median duration of exposure to IMFINZI 1,500 mg every 3 weeks in the neoadjuvant phase was 12 weeks (range: 1.1 to 84 weeks). The median duration of exposure to IMFINZI 1,500 mg every 4 weeks in the adjuvant phase was 32 weeks (range: 2.4 to 50 weeks).

The most common adverse reactions, including laboratory abnormalities, in the overall study (occurring in $\geq 20\%$ of patients) were decreased hemoglobin, decreased neutrophils, increased blood creatinine, decreased sodium, nausea, increased ALT, decreased calcium, decreased platelets, fatigue, increased potassium, decreased lymphocytes, increased AST, constipation, decreased magnesium, decreased appetite, increased alkaline phosphate, rash, pyrexia, diarrhea, vomiting, and abdominal pain.

Table 15 summarizes the adverse reactions that occurred in patients treated with IMFINZI plus chemotherapy

Table 15. Adverse Reactions Occurring in $\geq 10\%$ of Patients in the NIAGARA Study

	IMFINZI with gemcitabine and cisplatin N = 530			Gemcitabine and cisplatin N = 526		
Adverse Reaction	All (%)	Grades* 3-4 (%)	Grade* 3-4 (%)	All (%)	Grades* 3-4 (%)	Grade* 3-4 (%)
Gastrointestinal disorders						
Nausea	54	1.5	48	1		
Constipation	39	0.8	39	0.8		
Diarrhea	21	1.5	14	0.4		
Vomiting [†]	20	0.9	19	0.2		
Abdominal pain [†]	20	0.9	13	1		
General disorders and administration site conditions						
Fatigue [†]	52	2.3	49	3		
Pyrexia [†]	22	0.4	17	0		
Edema [†]	13	0.4	13	0		
Metabolism and nutrition disorders						
Decreased appetite	27	0.6	25	0.6		
Skin and subcutaneous tissue disorders						
Rash [†]	23	1.3	12	0.6		
Pruritus	15	0	7	0		
Nervous system disorders						
Peripheral neuropathy [†]	16	0.2	14	0		
Headache [†]	11	0	11	0		
Dizziness [†]	11	0	10	0.2		
Endocrine disorders						
Hypothyroidism [†]	13	0.4	2.3	0		
Vascular disorders						
Hypertension [†]	12	4.5	9	2.9		
Hemorrhage [†]	11	0.9	10	2.1		

* Graded according to NCI CTCAE version 5.0.

† Includes multiple similar terms.

Table 16 summarizes the laboratory abnormalities in patients treated with IMFINZI plus chemotherapy.

Table 16. Select Laboratory Abnormalities That Worsened from Baseline in $\geq 20\%$ of Patients Who Received IMFINZI with Chemotherapy in the NIAGARA study

Laboratory Abnormality	IMFINZI with gemcitabine and cisplatin [†]		Gemcitabine and cisplatin [‡]	
	All Grades [†] (%)	Grade [†] 3 or 4 (%)	All Grades [†] (%)	Grade [†] 3 or 4 (%)
Chemistry				
Increased blood creatinine	63	9	58	7
Decreased sodium	54	9	55	10
Increased ALT	53	2.3	54	4.0
Decreased calcium	52	1.3	43	1.2
Increased potassium	51	4.2	49	4.4
Increased AST	42	1.5	39	2.1
Decreased magnesium	38	1.9	37	2.3
Increased alkaline phosphate	26	0.8	25	0.4
Hematology				
Decreased hemoglobin	88	13	87	13
Decreased neutrophils	76	31	74	34
Decreased platelets	52	6	50	7
Decreased lymphocytes	44	10	40	8

[‡] Each test incidence is based on the number of patients who had both baseline and at least one on-study laboratory measurement available: IMFINZI plus chemotherapy (range: 482 to 528), and chemotherapy (range: 467 to 521).

[†] Graded per NCI CTCAE V5.

Neoadjuvant Phase of NIAGARA

A total of 530 patients received at least 1 dose of IMFINZI in combination with chemotherapy as neoadjuvant treatment in the IMFINZI treatment arm, and 526 patients received at least 1 dose of chemotherapy as neoadjuvant treatment in the chemotherapy treatment arm. In the neoadjuvant phase, serious adverse reactions occurred in 24% of patients who received IMFINZI in combination with chemotherapy; the most frequent ($\geq 1\%$) serious adverse reactions were pulmonary embolism (1.9%), febrile neutropenia (1.5%), acute kidney injury (1.3%), thrombocytopenia (1.3%), urinary tract infection (1.3%) and pneumonia (1.3%). Fatal adverse reactions occurred in 1.1% of patients including sepsis (0.2%), myocardial infarction (0.2%), and pulmonary embolism (0.2%). One fatal adverse reaction of pneumonia was reported in 1 (0.2%) patient in the post-surgery phase before adjuvant treatment started.

Permanent discontinuation of IMFINZI due to an adverse reaction in the neoadjuvant phase occurred in 9% of patients while receiving IMFINZI in combination with chemotherapy. The most frequent ($\geq 0.5\%$) adverse reactions that led to permanent discontinuation of IMFINZI

were blood creatinine increased (0.9%), neutropenia (0.6%), acute kidney injury (0.6%), asthenia (0.6%) and fatigue (0.6%).

Of the 530 patients in the IMFINZI treatment arm and 526 patients in the chemotherapy treatment arm who received neoadjuvant treatment, 1 (0.2%) patient in each treatment arm did not receive surgery due to adverse reactions. The adverse reaction that led to cancellation of surgery in the IMFINZI treatment arm was interstitial lung disease.

Of the 469 patients in the IMFINZI treatment arm who underwent radical cystectomy, 4 (0.8%) patients experienced delay of surgery (defined as occurring more than 56 days after the last dose of neoadjuvant treatment) due to adverse reactions.

Adjuvant Phase of NIAGARA

A total of 383 patients (72%) in the IMFINZI treatment arm received at least 1 dose of adjuvant treatment.

Serious adverse reactions occurred in 26% of patients receiving IMFINZI as adjuvant treatment. The most frequent serious adverse reactions (occurring in $\geq 1\%$ of patients) were urinary tract infection (7%), acute kidney injury (3.7%), hydronephrosis (2.1%), pyelonephritis (2.1%), urosepsis (1.8%), and sepsis (1.6%). Fatal adverse reactions occurred in 1.8% of patients, including COVID-19 (0.3%), severe acute respiratory syndrome (0.3%), cardiopulmonary failure (0.3%), gastrointestinal haemorrhage (0.3%), and chronic hepatic failure (0.3%).

Permanent discontinuation of adjuvant IMFINZI due to an adverse reaction occurred in 5% of patients. The most frequent ($\geq 0.5\%$) adverse reactions that led to permanent discontinuation of adjuvant IMFINZI were nephritis (0.8%), fatigue (0.5%), diarrhea (0.5%), decreased appetite (0.5%) and pneumonitis (0.5%).

USE IN SPECIFIC POPULATIONS

Pregnancy

Risk summary

Based on its mechanism of action and data from animal studies, IMFINZI can cause fetal harm when administered to a pregnant woman [see [Clinical Pharmacology](#)]. There are no data on the use of IMFINZI in pregnant women.

In animal reproduction studies, administration of durvalumab to pregnant cynomolgus monkeys from the confirmation of pregnancy through delivery at exposure levels approximately 6 to 20 times higher than those observed at the clinical dose of 10 mg/kg (based on AUC) resulted in an increase in premature delivery, fetal loss and premature neonatal death (*see Data*). Human immunoglobulin G1 (IgG1) is known to cross the placental barrier; therefore, durvalumab has the potential to be transmitted from the mother to the developing fetus. Apprise pregnant women of the potential risk to a fetus.

In the U.S. general population, the estimated background risk of major birth defects and miscarriage in clinically recognized pregnancies is 2-4% and 15-20%, respectively.

Data

Animal Data

As reported in the literature, the PD-1/PD-L1 pathway plays a central role in preserving pregnancy by maintaining maternal immune tolerance to the fetus. In mouse allogeneic pregnancy models, disruption of PD-L1 signaling was shown to result in an increase in fetal loss. The effects of durvalumab on prenatal and postnatal development were evaluated in reproduction studies in cynomolgus monkeys. Durvalumab was administered from the confirmation of pregnancy through delivery at exposure levels approximately 6 to 20 times higher than those observed at the recommended clinical dose of 10 mg/kg (based on AUC). Administration of durvalumab resulted in premature delivery, fetal loss (abortion and stillbirth) and increase in neonatal deaths. Durvalumab was detected in infant serum on postpartum Day 1, indicating the presence of placental transfer of durvalumab. Based on its mechanism of action, fetal exposure to durvalumab may increase the risk of developing immune-mediated disorders or altering the normal immune response and immune-mediated disorders have been reported in PD-1 knockout mice.

Lactation

Risk Summary

There is no information regarding the presence of durvalumab in human milk, the effects on the breastfed infant, or the effects on milk production. Human IgG1 is excreted in human milk. Durvalumab was present in the milk of lactating cynomolgus monkeys and was associated with premature neonatal death (*see Data*).

Because of the potential for adverse reactions in breastfed infants, advise women not to breastfeed during treatment with IMFINZI and for at least 3 months after the last dose.

Data

In lactating cynomolgus monkeys, durvalumab was present in breast milk at about 0.15% of maternal serum concentrations after administration of durvalumab from the confirmation of pregnancy through delivery at exposure levels approximately 6 to 20 times higher than those observed at the recommended clinical dose of 10 mg/kg (based on AUC). Administration of durvalumab resulted in premature neonatal death.

Females and Males of Reproductive Potential

Pregnancy testing

Verify pregnancy status of females of reproductive potential prior to initiating treatment with IMFINZI.

Contraception

Females

IMFINZI can cause fetal harm when administered to a pregnant woman [*see Use in Specific Populations*]. Advise females of reproductive potential to use effective contraception during treatment with IMFINZI and for at least 3 months following the last dose of IMFINZI.

Pediatric Use

The safety and effectiveness of IMFINZI have not been established in pediatric patients. The safety, preliminary efficacy and pharmacokinetics of IMFINZI in combination with tremelimumab followed by IMFINZI as a single agent were assessed but not established in a multi-centre, open-label, dose finding and dose expansion study (NCT03837899) in pediatric

patients with advanced malignant solid tumours (except primary central nervous system tumours) who had disease progression and for whom no standard of care treatment exists. The study enrolled 50 pediatric patients with an age range from 1 to 17 years with primary tumour categories: neuroblastoma, solid tumour and sarcoma. Patients received either IMFINZI 20 mg/kg in combination with tremelimumab 1 mg/kg or IMFINZI 30 mg/kg in combination with tremelimumab 1 mg/kg intravenously every 4 weeks for 4 cycles, followed by IMFINZI as a single agent every 4 weeks. In the dose finding phase, IMFINZI and tremelimumab combination therapy was preceded by a single dose of IMFINZI monotherapy; 8 patients in this phase however discontinued treatment prior to receiving tremelimumab. No new safety signals were observed in pediatric patients in this study, relative to the known safety profiles of IMFINZI and tremelimumab in adults. Based on population PK analysis, durvalumab systemic exposure in pediatric patients > 35 kg receiving IMFINZI 20 mg/kg every 4 weeks was similar to exposure in adults receiving IMFINZI 20 mg/kg every 4 weeks, whereas in pediatric patients ($\geq$ 35 kg) receiving IMFINZI 30 mg/kg every 4 weeks, exposure was approximately 1.5 fold higher compared to exposure in adults receiving IMFINZI 20 mg/kg every 4 weeks. In pediatric patients < 35 kg receiving IMFINZI 30 mg/kg every 4 weeks, the systemic exposure was similar to exposure in adults receiving IMFINZI 20 mg/kg every 4 weeks.

Geriatric Use

Of the 401 patients with resectable NSCLC treated with IMFINZI in combination with chemotherapy in the AEGEAN study, 209 (52%) patients were 65 years or older and 49 (12%) patients were 75 years or older. There were no overall clinically meaningful differences in safety or efficacy between patients $\geq$ 65 years of age and younger patients.

Of the 476 patients treated with IMFINZI in the PACIFIC study, 45% were 65 years or older, while 7.6% were 75 years or older. No overall differences in safety or effectiveness were observed between patients 65 years or older and younger patients. The PACIFIC study did not include sufficient numbers of patients aged 75 years and over to determine whether they respond differently from younger patients.

Of the 262 patients with LS-SCLC treated with IMFINZI, 103 (39%) patients were 65 years or older and 15 (6%) patients were 75 years or older. There were no clinically meaningful differences in safety and efficacy between patients 65 years or older and younger patients.

Of the 265 patients with ES-SCLC treated with IMFINZI in combination with chemotherapy, 101 (38%) patients were 65 years or older and 19 (7.2%) patients were 75 years or older. There were no clinically meaningful differences in safety or efficacy between patients 65 years or older and younger patients.

Of the 338 patients with BTC treated with IMFINZI in combination with chemotherapy in the TOPAZ-1 study, 158 (47%) patients were 65 years or older and 38 (11%) patients were 75 years or older. No overall differences in safety or effectiveness of IMFINZI have been observed between patients 65 years of age and older and younger adult patients.

Of the 235 patients with endometrial cancer treated with IMFINZI with carboplatin and paclitaxel, 49% of patients were 65 years of age or older and 12% of patients were 75 years of age or older. No overall differences in safety or effectiveness of IMFINZI have been observed between patients 65 years of age and older and younger adult patients.

Of the 530 patients with MIBC treated with IMFINZI in combination with gemcitabine and cisplatin in the NIAGARA study, 272 (51%) patients were 65 years or older, and 57 (11%) patients were 75 years or older. No overall differences in safety or effectiveness of IMFINZI have been observed between patients 65 years of age and older and younger adult patients.

OVERDOSAGE

There is no information on overdose with IMFINZI.

DESCRIPTION

Durvalumab is a programmed cell death ligand 1 (PD-L1) blocking antibody. Durvalumab is a human immunoglobulin G1 kappa (IgG1 κ) monoclonal antibody that is produced by recombinant DNA technology in Chinese Hamster Ovary (CHO) cell suspension culture.

IMFINZI (durvalumab) Injection for intravenous use is a sterile, preservative-free, clear to opalescent, colorless to slightly yellow solution, free from visible particles.

Each 500 mg vial of IMFINZI contains 500 mg of durvalumab in 10 mL solution. Each mL contains durvalumab, 50 mg, L-histidine (2 mg), L-histidine hydrochloride monohydrate (2.7 mg), α,α -trehalose dihydrate (104 mg), Polysorbate 80 (0.2 mg), and Water for Injection, USP.

Each 120 mg vial of IMFINZI contains 120 mg of durvalumab in 2.4 mL solution. Each mL contains durvalumab, 50 mg, L-histidine (2 mg), L-histidine hydrochloride monohydrate (2.7 mg), α,α -trehalose dihydrate (104 mg), Polysorbate 80 (0.2 mg), and Water for Injection, USP.

CLINICAL PHARMACOLOGY

Mechanism of Action

Expression of programmed cell death ligand-1 (PD-L1) can be induced by inflammatory signals (e.g., IFN-gamma) and can be expressed on both tumor cells and tumor-associated immune cells in the tumor microenvironment. PD-L1 blocks T-cell function and activation through interaction with PD-1 and CD80 (B7.1). By binding to its receptors, PD-L1 reduces cytotoxic T-cell activity, proliferation, and cytokine production.

Durvalumab is a human immunoglobulin G1 kappa (IgG1 κ) monoclonal antibody that binds to PD-L1 and blocks the interaction of PD-L1 with PD-1 and CD80 (B7.1). Blockade of PD-L1/PD-1 and PD-L1/CD80 interactions releases the inhibition of immune responses, without inducing antibody dependent cell-mediated cytotoxicity (ADCC).

PD-L1 blockade with durvalumab led to increased T-cell activation in vitro and decreased tumor size in co-engrafted human tumor and immune cell xenograft mouse models.

Pharmacodynamics

The steady state AUC, C_{trough} , and C_{max} in patients administered with 1500 mg every 4 weeks are 6% higher, 19% lower, and 55% higher than those administered with 10 mg/kg every 2 weeks, respectively. Based on the modeling of pharmacokinetic data and exposure relationships for safety, there are no anticipated clinically meaningful differences in efficacy and safety for the doses of 1500 mg every 4 weeks compared to 10 mg/kg every 2 weeks in patients weighing > 30 kg with NSCLC.

Pharmacokinetics

The pharmacokinetics of durvalumab as a single agent was studied in patients with doses ranging from 0.1 mg/kg (0.01 times the approved recommended dosage) to 20 mg/kg (2 times the approved recommended dosage) administered once every two, three or four weeks.

PK exposure increased more than dose-proportionally at doses < 3 mg/kg (0.3 times the approved recommended dosage) and dose proportionally at doses $\geq$ to 3 mg/kg every 2 weeks. Steady state was achieved at approximately 16 weeks.

The pharmacokinetics of durvalumab is similar when assessed as a single agent and when in combination with chemotherapy and in combination with platinum-based chemotherapy followed by IMFINZI in combination with olaparib.

Distribution

The geometric mean (% coefficient of variation [CV%]) steady state volume of distribution (V_{ss}) was 5.6 (18%) L.

Elimination

Durvalumab clearance decreases over time, with a mean maximal reduction (CV%) from baseline values of approximately 23% (57%) resulting in a geometric mean (CV%) steady state clearance (CL_{ss}) of 8.2 mL/h (39%) at day 365; the decrease in CL_{ss} is not considered clinically relevant. The geometric mean (CV%) terminal half-life, based on baseline CL was approximately 18 (24%) days.

Specific Populations

There were no clinically significant differences in pharmacokinetics of durvalumab based on body weight (31 – 175 kg), age (18–96 years), sex, race (White, Black, Asian, Native Hawaiian, Pacific Islander, or Native American), albumin levels (4 – 57 g/L), lactate dehydrogenase (LDH) levels (18 – 15,800 U/L), creatinine levels, soluble PD-L1 (67 – 3,470 pg/mL), tumor type (NSCLC, SCLC and BTC), mild or moderate renal impairment (CL_{cr} 30 to 89 mL/min), mild or moderate hepatic impairment (bilirubin $\leq$ 3x ULN and any AST). The effect of severe renal impairment (CL_{cr} 15 to 29 mL/min) or severe hepatic impairment (bilirubin > 3x ULN and any AST) on the pharmacokinetics of durvalumab is unknown.

Immunogenicity

The observed incidence of anti-drug antibodies (ADA) is highly dependent on the sensitivity and specificity of the assay. Differences in assay methods preclude meaningful comparison of the incidence of ADAs in the studies described below with the incidence of ADAs in other studies including those of IMFINZI or of other durvalumab products.

In clinical studies of patients who received IMFINZI for 10 to 48 weeks at dosages of 1500 mg every 4 hours, 10 mg/kg every 2 weeks, 20 mg/kg every 4 weeks as a single agent or 1120 mg every 3 weeks, or 1500 mg every 3 weeks in the combination therapies, 3.1 % (159/5121) of evaluable patients tested positive for treatment-emergent anti-durvalumab antibodies, and 10% (16/159) of treatment-emergent-ADA positive patients had neutralizing antibodies against durvalumab. There were no identified clinically significant effects of ADAs on durvalumab pharmacokinetics or safety; however, the effect of these ADAs on the effectiveness of IMFINZI is unknown.

In the DUO-E study, the number of the patients who were treated with platinum-based chemotherapy + IMFINZI (n=198) or platinum-based chemotherapy + IMFINZI + Olaparib (n=207) and evaluable for the presence of ADAs, 2 (1.0%) patients tested positive for treatment-emergent ADAs in the platinum-based chemotherapy + IMFINZI arm and no patients tested positive for treatment-emergent ADAs in the platinum-based chemotherapy + IMFINZI + olaparib arm. Neutralizing antibodies against durvalumab were detected in 1 (0.5%) patient in the platinum-based chemotherapy + IMFINZI arm and 0 patients in the platinum-based chemotherapy + IMFINZI + olaparib arm. There were insufficient number of patients with treatment-emergent ADAs or neutralizing antibodies to determine whether ADAs have an impact on pharmacokinetics or safety of durvalumab.

NONCLINICAL TOXICOLOGY

Carcinogenesis, Mutagenesis, Impairment of Fertility

The carcinogenic and genotoxic potential of durvalumab have not been evaluated.

Animal fertility studies have not been conducted with durvalumab. In repeat-dose toxicology studies with durvalumab in sexually mature cynomolgus monkeys of up to 3 months duration, there were no notable effects on the male and female reproductive organs.

Animal Toxicology and/or Pharmacology

In animal models, inhibition of PD-L1/PD-1 signaling increased the severity of some infections and enhanced inflammatory responses. M. tuberculosis-infected PD-1 knockout mice exhibit markedly decreased survival compared with wild-type controls, which correlated with increased bacterial proliferation and inflammatory responses in these animals. PD-L1 and PD-1 knockout mice have also shown decreased survival following infection with lymphocytic choriomeningitis virus.

CLINICAL STUDIES

Non-Small Cell Lung Cancer (NSCLC)

Neoadjuvant and Adjuvant Treatment of Resectable NSCLC – AEGEAN Study

The efficacy of IMFINZI in combination with neoadjuvant chemotherapy, followed by surgery and continued adjuvant treatment with IMFINZI as a single agent was investigated in AEGEAN (NCT03800134), a randomized, double-blind, placebo-controlled, multicenter trial conducted in 802 patients with previously untreated and resectable squamous or non-squamous NSCLC (Stage IIA to select Stage IIIB [AJCC, 8th edition]). Patients were enrolled regardless of tumor PD-L1 expression. Eligible patients had no prior exposure to immune-mediated therapy, a WHO/ECOG Performance status of 0 or 1, and at least one RECIST 1.1 target lesion.

Patients with active or prior documented autoimmune disease, or use of any immunosuppressive medication within 14 days of the first dose of IMFINZI were ineligible. The population for efficacy analyses was a modified intent-to-treat [mITT] which excluded patients with known EGFR mutations or ALK rearrangements.

Crossover between the study arms was not permitted. Randomization was stratified by disease stage (Stage II vs. Stage III) and by PD-L1 expression (TC < 1% vs. TC ≥ 1%) status. Patients were randomized 1:1 to one of the following treatment arms:

- Arm 1: Neoadjuvant IMFINZI 1500 mg once every 3 weeks for up to 4 cycles in combination with:
Squamous tumor histology: carboplatin AUC 6 and paclitaxel 200 mg/m² on Day 1 of each 3-week cycle, OR cisplatin 75 mg/m² on Day 1 and gemcitabine 1250 mg/m² on Day 1 and Day 8 of each 3-week cycle, for 4 cycles

Non-squamous tumor histology: pemetrexed 500 mg/m² and cisplatin 75 mg/m² on Day 1 of each 3-week cycle, for 4 cycles OR pemetrexed 500 mg/m² and carboplatin AUC 5 on Day 1 of each 3-week cycle, for 4 cycles.

Followed by adjuvant IMFINZI 1500 mg as a single agent for up to 12 cycles post-surgery.

- Arm 2: Neoadjuvant placebo in combination with 4 cycles of chemotherapy (*see above*) prior to surgery.

Followed by placebo for up to 12 cycles post-surgery.

All study medications were administered via intravenous infusion. In the event of unfavorable tolerability, patients who met the eligibility criteria were switched from cisplatin to carboplatin therapy at any point during the study. In patients with comorbidities or unable to tolerate cisplatin as per Investigators judgment, carboplatin AUC 5 could be administered from cycle 1. Treatment with IMFINZI or placebo continued until completion of the treatment, disease progression that precluded definitive surgery, inability to complete definitive surgery, disease recurrence in the adjuvant phase, or unacceptable toxicity. A RECIST 1.1 tumor assessment was performed at baseline, and upon completion of the neoadjuvant period (prior to surgery). Tumor assessments were conducted at 5 weeks postoperatively, prior to the start of adjuvant therapy and every 12 weeks until week 48, every 24 weeks for approximately 4 years, and then every 48 weeks thereafter until disease progression, consent withdrawal, or death.

The trial was not designed to isolate the effect of IMFINZI in each phase (neoadjuvant or adjuvant) of treatment.

The major efficacy outcome measures of the study were pathological complete response (pCR) by blinded central pathology review and event-free survival (EFS) by blinded independent central review (BICR) assessment. Additional efficacy outcome measures were major pathological response (MPR) by blinded central pathology review, DFS by BICR, and OS.

The demographics and baseline disease characteristics were as follows: male (72%); median age 65 years (range: 30 to 88); age ≥ 65 years (52%); WHO/ECOG PS 0 (68%), WHO/ECOG PS 1 (32); White (54%), Asian (41%), Black or African American (0.9%), American Indian or Alaska Native (1.4%), Other Race (2.6%); Not Hispanic or Latino (84%); current or past smokers (86%); squamous histology (49%) and non-squamous histology (51%); Stage II (28%), Stage III (71%); PD-L1 expression status TC ≥ 1% (67%), PD-L1 expression status TC < 1% (33%).

In the mITT population, 78% of patients in Arm 1 completed definitive surgery compared to 77% of patients in Arm 2.

The trial demonstrated statistically significant improvements in EFS and pCR rate (see Table 17 and Figure 1) in the IMFINZI in combination with chemotherapy arm compared to the placebo in combination with chemotherapy arm.

Table 17. Efficacy Results for the AEGEAN Study (mITT)

	IMFINZI 1,500 mg every 3 weeks with chemotherapy/ IMFINZI (N = 366)	Placebo with Chemotherapy/Placebo (N = 374)
EFS*		
Number of events, n (%)	98 (27)	138 (37)
Median EFS (95% CI) (months)	NR (31.9, NR)	25.9 (18.9, NR)
Hazard ratio (95% CI)	0.68 (0.53, 0.88)	
2-sided p-value ^{†,§}	0.0039	
pCR ^{*,‡,§}		
Number of patients with response	63	16
pCR rate, % (95% CI)	17.2 (13.5, 21.5)	4.3 (2.5, 6.8)
p-value	< 0.0001	
Difference in proportions, % (95% CI) [¥]	13.0 (8.7, 17.6)	

* Results are based on planned EFS interim analysis and pCR final analysis (DCO: 10 November 2022) which occurred 46.3 months after study initiation.

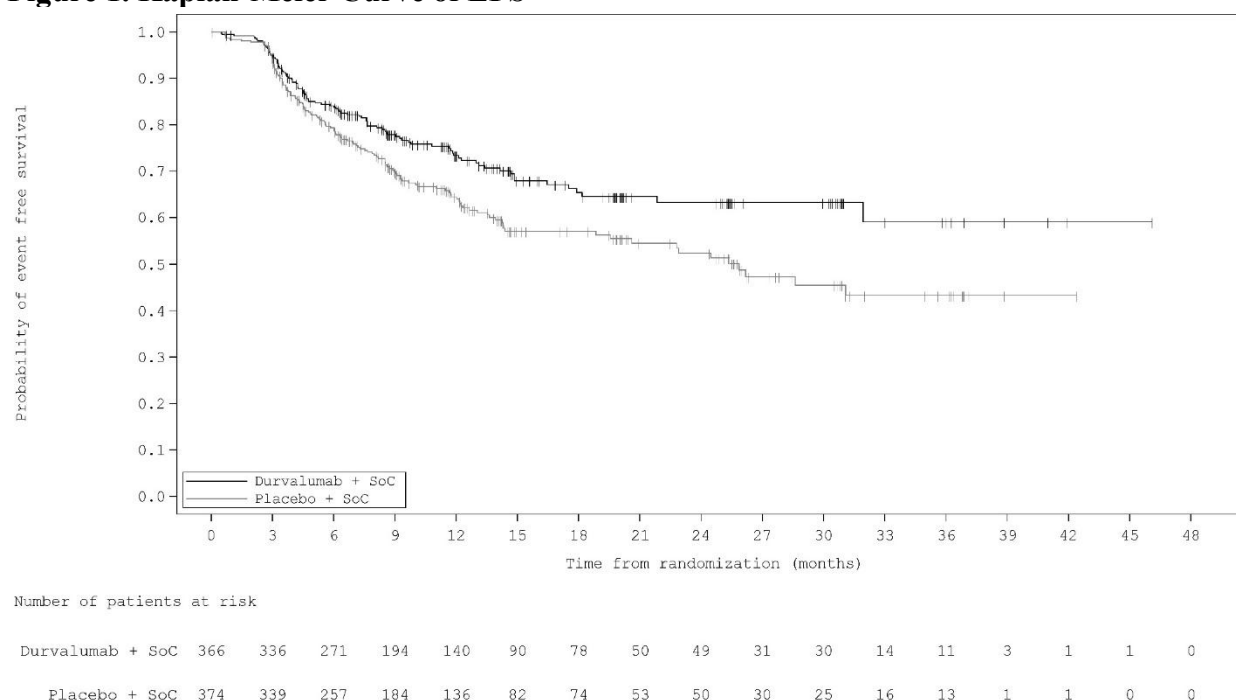
† Compared to a two-sided p-value boundary of 0.00989.

‡ Based on a pre-specified pCR interim analysis (DCO: 14 January 2022) in n = 402, the pCR rate was statistically significant (p = 0.000036) compared to significance level of 0.0082%.

§ The 2-sided p-value for pCR was calculated based on a stratified CMH test. The 2-sided p-value for EFS was calculated based on a stratified log-rank test. Stratification factors include PD-L1 and disease stage.

¥ Confidence interval for the difference in proportions was calculated based on stratified Miettinen and Nurminen method.

Figure 1. Kaplan-Meier Curve of EFS



At the interim analysis, the trial demonstrated a statistically significant difference in MPR rate (34% vs. 14%; $p < 0.0001$). At the time of the prespecified interim analyses, overall survival (OS) was not formally tested for statistical significance.

Unresectable Stage III NSCLC- PACIFIC Study

The efficacy of IMFINZI was evaluated in the PACIFIC Study, a randomised, double-blind, placebo-controlled, multicentre study in 713 patients with locally advanced, unresectable NSCLC. Patients had completed at least 2 cycles of definitive platinum-based chemotherapy with radiation therapy within 1 to 42 days prior to initiation of the study and had a ECOG performance status of 0 or 1. Ninety-two percent of patients had received a total dose of 54 to 66 Gy of radiation. The study excluded patients who had progressed following chemoradiation therapy, patients with prior exposure to any anti-PD-1 or anti-PD-L1 antibody, patients with active or prior documented autoimmune disease within 2 years of initiation of the study; a history of immunodeficiency; a history of severe immune-mediated adverse reactions; medical conditions that required systemic immunosuppression, except physiological dose of systemic corticosteroids; active tuberculosis or hepatitis B or C or HIV infection or patients receiving live attenuated vaccine within 30 days before or after the start of IMFINZI. Patients were randomised 2:1 to receive 10 mg/kg IMFINZI ($n = 476$) or 10 mg/kg placebo ($n = 237$) via intravenous infusion every 2 weeks for up to 12 months or until unacceptable toxicity or confirmed disease progression. Randomisation was stratified by gender, age (< 65 years vs. ≥ 65 years) and smoking status (smoker vs. non-smoker). Patients with disease control at 12 months were given the option to be re-treated upon disease progression. Tumour assessments were conducted every 8 weeks for the first 12 months and then every 12 weeks thereafter.

Patients were enrolled regardless of their tumour PD-L1 expression level. Where available, archival tumour tissue specimens taken prior to chemoradiation therapy were retrospectively tested for PD-L1 expression on tumour cells (TC) using the VENTANA PD-L1 (SP263) IHC assay. Of the 713 patients randomised, 63% of patients provided a tissue sample of sufficient quality and quantity to determine PD-L1 expression and 37% were unknown.

The demographics and baseline disease characteristics were well balanced between study arms. Baseline demographics of the overall study population were as follows: male (70%), age $\geq$ 65 years (45%), age $\geq$ 75 years (8%), White (69%), Asian (27%), other (4%), current smoker (16%), past-smoker (75%), never smoker (9%), ECOG Performance Status 0 (49%), ECOG Performance Status 1 (51%). Disease characteristics were as follows: Stage IIIA (53%), Stage IIIB (45%), histological sub-groups of squamous (46%), non-squamous (54%). Of 451 patients with PD L1 expression available, 67% were TC $\geq$ 1% [PD-L1 TC 1-24% (32%), PD L1 TC $\geq$ 25% (35%)] and 33% were TC < 1%.

The two primary endpoints of the study were progression-free survival (PFS) and overall survival (OS) of IMFINZI vs. placebo. Secondary efficacy endpoints included PFS at 12 months (PFS 12) and 18 months (PFS 18) from randomisation and Time from Randomisation to Second Progression (PFS2). PFS was assessed by Blinded Independent Central Review (BICR) according to RECIST v1.1.

The study demonstrated a statistically significant improvement in PFS in the IMFINZI-treated group compared with the placebo group [hazard ratio (HR) = 0.52 (95% CI: 0.42, 0.65), $p < 0.0001$]. The study demonstrated a statistically significant improvement in OS in the IMFINZI-treated group compared with the placebo group [HR = 0.68 (95% CI: 0.53, 0.87), $p = 0.00251$].

In the 5 year follow-up analysis, with a median follow-up of 34.2 months, IMFINZI continued to demonstrate improved OS and PFS compared to placebo. The OS and PFS results from the primary analysis and the follow-up analysis are summarized in Table 18.

Table 18. Efficacy results for the PACIFIC Study

	Primary Analysis^a		5 Year Follow-up Analysis^b	
	IMFINZI (n = 476)	Placebo (n = 237)	IMFINZI (n = 476)	Placebo (n = 237)
OS				
Number of deaths (%)	183 (38.4%)	116 (48.9%)	264 (55.5%)	155 (65.4%)
Median (months) (95% CI)	NR (34.7, NR)	28.7 (22.9, NR)	47.5 (38.1, 52.9)	29.1 (22.1, 35.1)
HR (95% CI)	0.68 (0.53, 0.87)		0.72 (0.59, 0.89)	
2- sided p-value	0.00251			
OS at 24 months (%) (95% CI)	66.3% (61.7%, 70.4%)	55.6% (48.9%, 61.3%)	66.3% (61.8%, 70.4%)	55.3% (48.6%, 61.4%)
p-value	0.005			
OS at 48 months (%) (95% CI)			49.7% (45.0%, 54.2%)	36.3% (30.1%, 42.6%)
OS at 60 months (%) (95% CI)			42.9% (38.2%, 47.4%)	33.4% (27.3%, 39.6%)
PFS				

	Primary Analysis^a		5 Year Follow-up Analysis^b	
	IMFINZI (n = 476)	Placebo (n = 237)	IMFINZI (n = 476)	Placebo (n = 237)
Number of events (%)	214 (45.0%)	157 (66.2%)	268 (56.3%)	175 (73.8%)
Median PFS (months) (95% CI)	16.8 (13.0, 18.1)	5.6 (4.6, 7.8)	16.9 (13.0, 23.9)	5.6 (4.8, 7.7)
HR (95% CI)	0.52 (0.42, 0.65)		0.55 (0.45, 0.68)	
p-value	p < 0.0001			
PFS at 12 months (%) (95% CI)	55.9% (51.0%, 60.4%)	35.3% (29.0%, 41.7%)	55.7% (51.0%, 60.2%)	34.5% (28.3%, 40.8%)
PFS at 18 months (%) (95% CI)	44.2% (37.7%, 50.5%)	27.0% (19.9%, 34.5%)	49.1% (44.2%, 53.8%)	27.5% (21.6%, 33.6%)
PFS at 48 months (%) (95% CI)			35.0% (29.9%, 40.1%)	19.9% (14.4%, 26.1%)
PFS at 60 months (%) (95% CI)			33.1% (28.0%, 38.2%)	19.0% (13.6%, 25.2%)
PFS2^c				
Median PFS2 (months) (95% CI)	28.3 (25.1, 34.7)	17.1 (14.5, 20.7)		
HR (95% CI)	0.58 (0.46, 0.73)			
p-value	p < 0.0001			

^a Primary analysis of PFS at data cut-off 13 February 2017. Primary analysis of OS and PFS2 at data cut-off 22 March 2018.

^b Follow-up OS and PFS analysis at data cut-off 11 January 2021.

^c PFS2 is defined as the time from the date of randomisation until the date of second progression (defined by local standard clinical practice) or death.

NR: Not Reached

Kaplan-Meier curves for OS and PFS from the 5 year follow-up analysis are presented in Figures 2 and 3.

Figure 2. Kaplan-Meier curve of OS

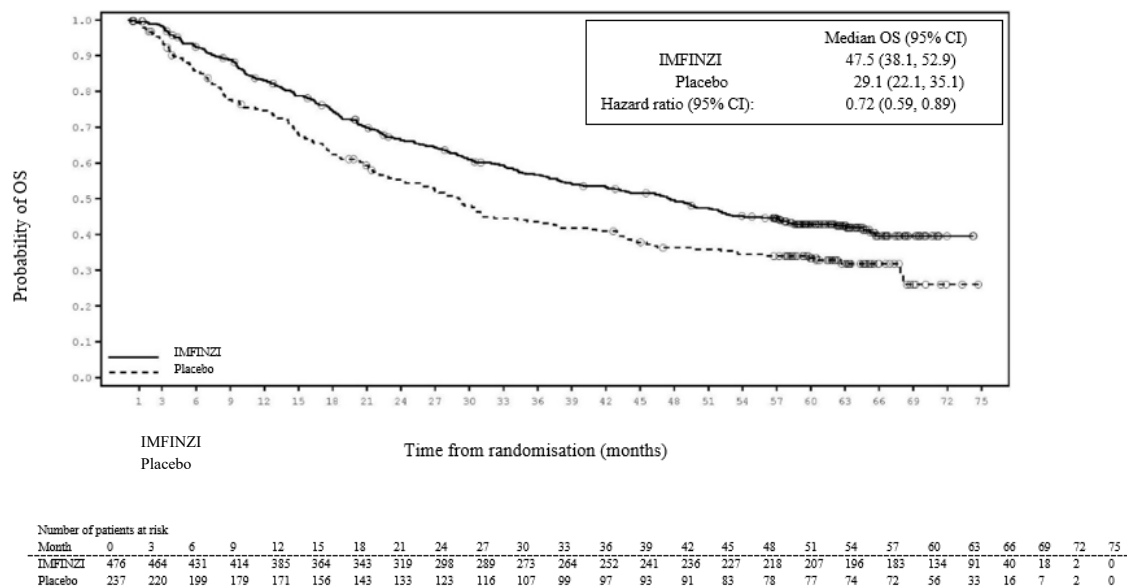
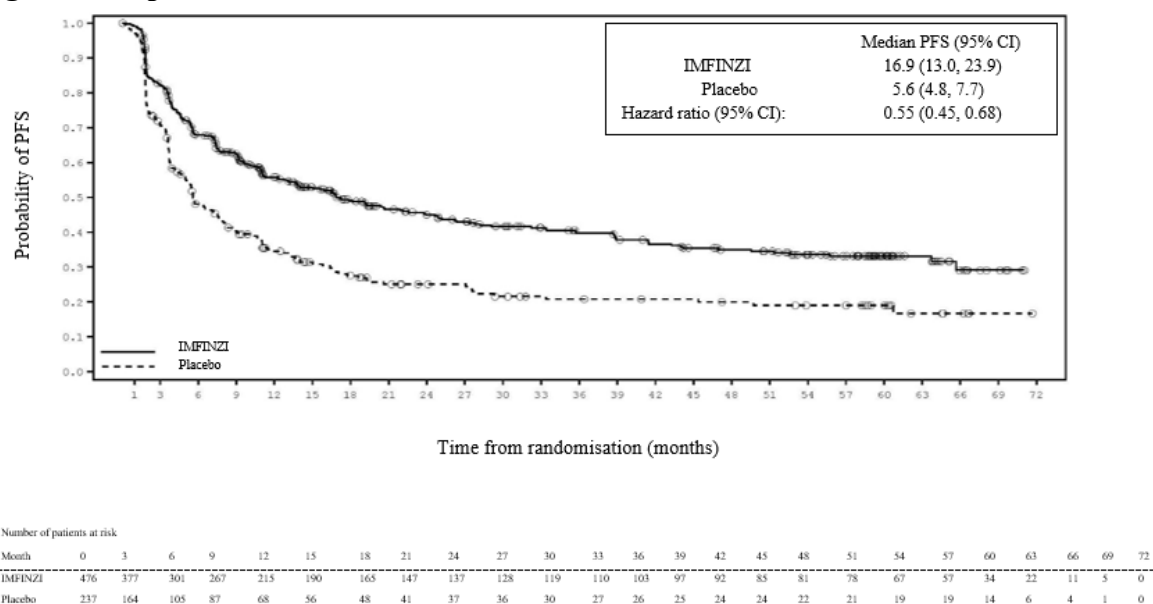


Figure 3. Kaplan-Meier curve of PFS



The improvements in PFS and OS in favour of patients receiving IMFINZI compared to those receiving placebo were consistently observed in all predefined subgroups analysed, including ethnicity, age, gender, smoking history, EGFR mutation status and histology.

Post-hoc subgroup analysis by PD-L1 expression

Additional subgroup analyses were conducted to evaluate the efficacy by tumour PD-L1 expression ($\geq 25\%$, $1-24\%$, $\geq 1\%$, $< 1\%$) and for patients whose PD-L1 status cannot be

established (PD-L1 unknown). PFS and OS results from the 5 year follow-up analysis are summarised in Figures 4, 5, 6 and 7.

Figure 4. Kaplan-Meier curve of OS for PD-L1 TC ≥ 1%

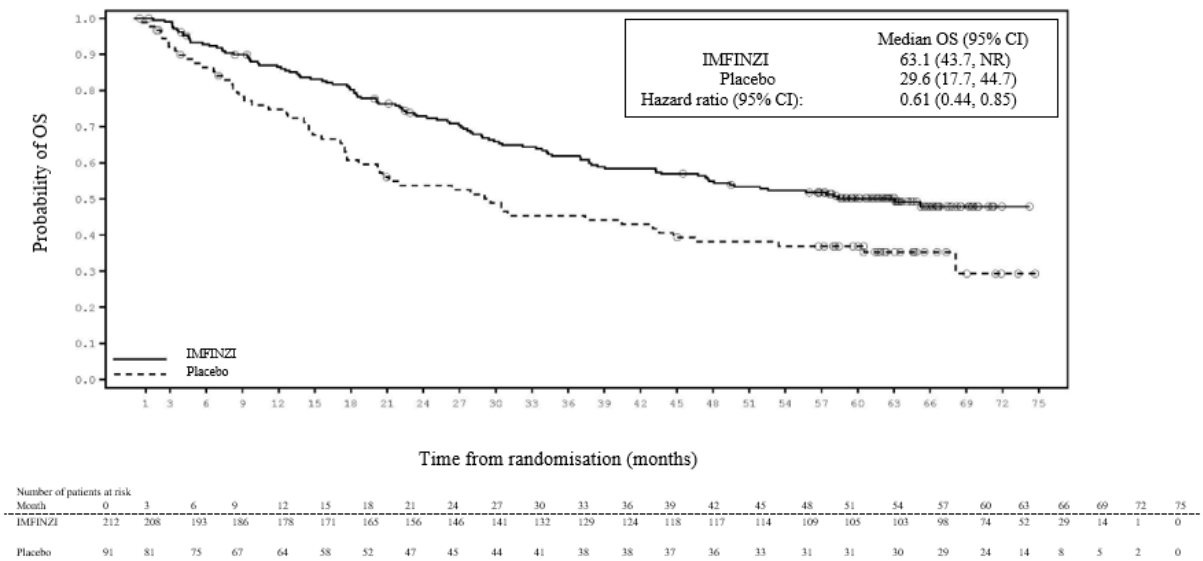


Figure 5. Kaplan-Meier curve of PFS for PD-L1 TC ≥ 1%

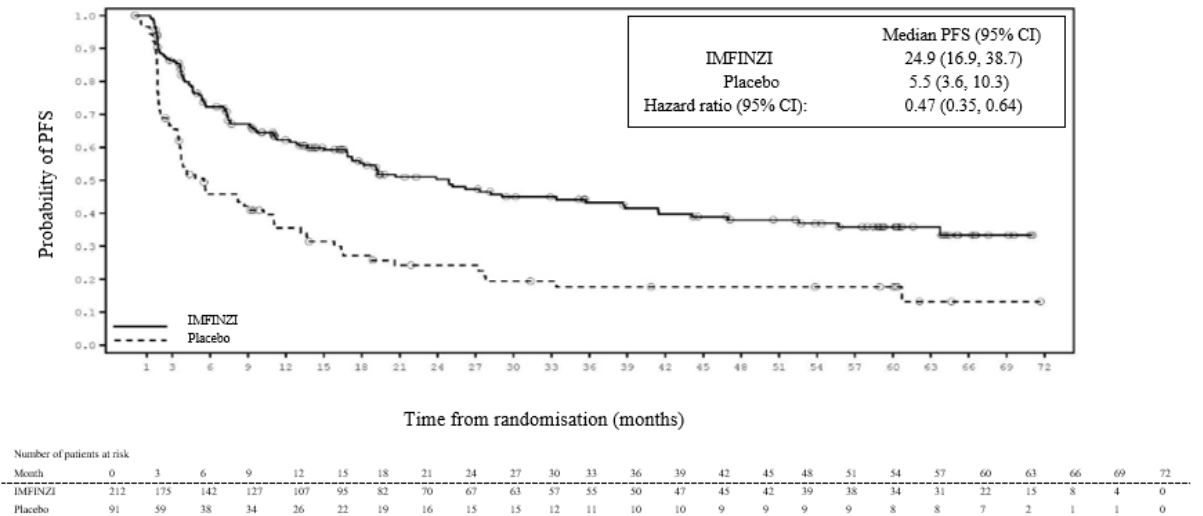


Figure 6. Forest plot of OS by PD-L1 expression

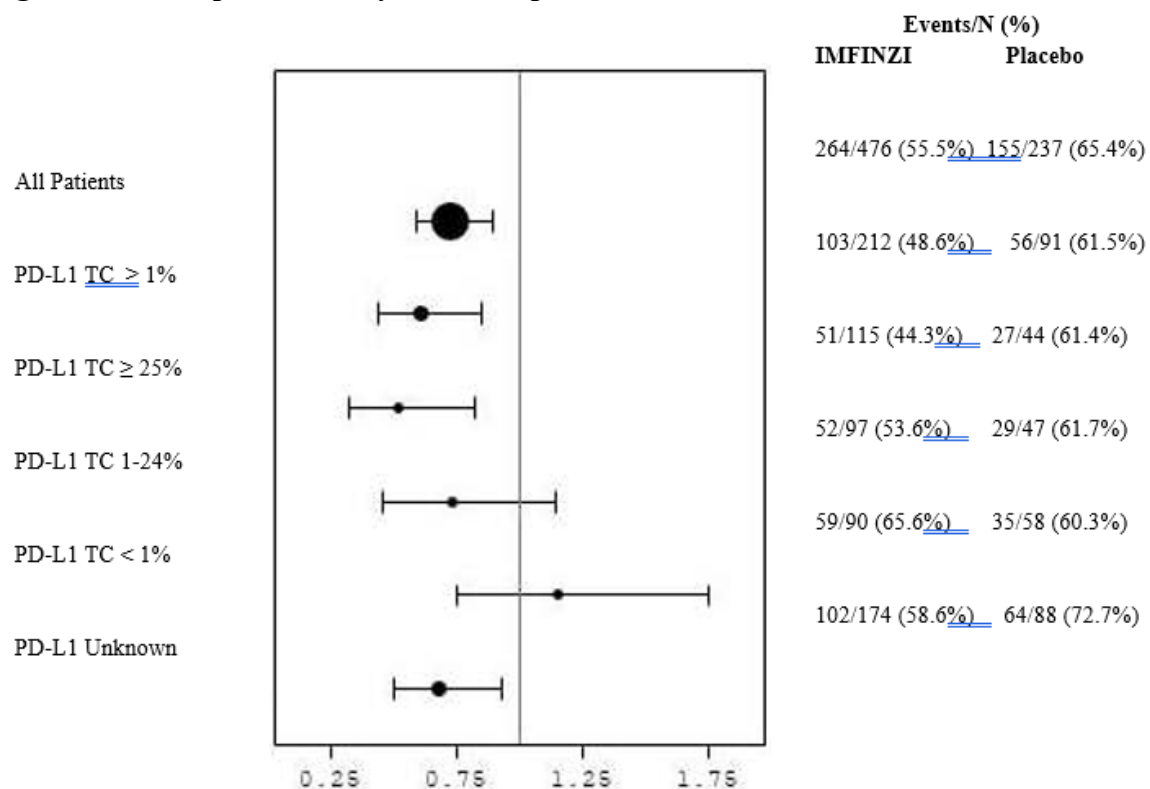
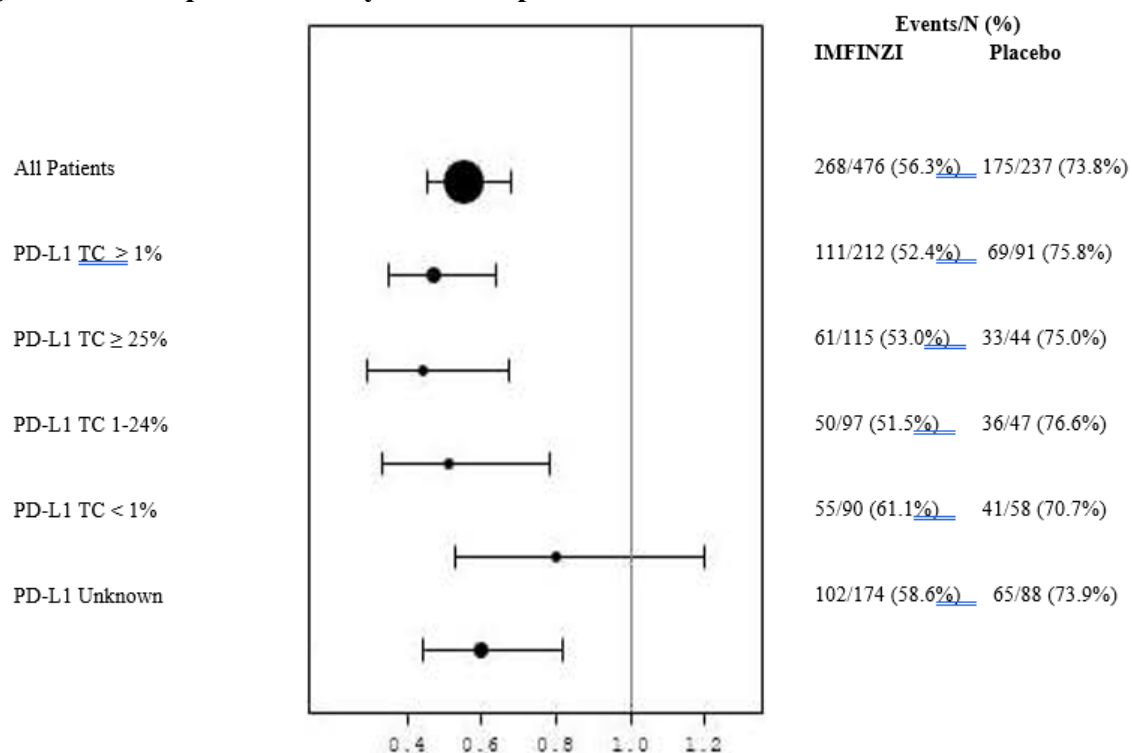


Figure 7. Forest plot of PFS by PD-L1 expression



Overall the safety profile of durvalumab in PD-L1 TC $\geq$ 1% subgroup was consistent with the intent to treat population, as was the PD-L1 TC < 1% subgroup.

Patient-reported outcomes

Patient-reported symptoms, function and health-related quality of life (HRQoL) were collected using the EORTC QLQ-C30 and its lung cancer module (EORTC QLQ-LC13). The LC13 and C30 were assessed at baseline, every 4 weeks for the first 8 weeks, followed by every 8 weeks until completion of the treatment period or discontinuation of IMFINZI due to toxicity or disease progression. Compliance was similar between the IMFINZI and placebo treatment groups (83% vs. 85.1% overall of evaluable forms completed).

At baseline, no differences in patient-reported symptoms, function and HRQoL were observed between IMFINZI and placebo groups. Throughout the duration of the study to Week 48, there was no clinically meaningful difference between IMFINZI and placebo groups in symptoms, functioning and HRQoL (as assessed by a difference of greater than or equal to 10 points).

Small Cell Lung Cancer (SCLC)

Limited-stage SCLC – ADRIATIC Study

The efficacy of IMFINZI was evaluated in the ADRIATIC Study (NCT03703297), a randomized, double-blind, placebo-controlled, multicenter study in 730 patients with histologically or cytologically confirmed LS-SCLC (Stage I to III according to AJCC, 8th edition) whose disease had not progressed following concurrent chemoradiation therapy (cCRT). Patients who had Stage I or II disease had to be medically inoperable as determined by the investigator. Eligible patients completed cCRT consisting of 4 cycles of platinum-based chemotherapy and either 60-66 Gy once daily over 6 weeks or 45 Gy twice daily over 3 weeks radiation therapy within 42 days prior to the first dose of IMFINZI or placebo. Prophylactic cranial irradiation (PCI) could be delivered at the discretion of the investigator after cCRT and had to be completed within 42 days prior to the first dose of IMFINZI or placebo. Patients with active or prior documented autoimmune disease within 5 years of initiation into the study; a history of active primary immunodeficiency; a history of Grade $\geq$ 2 pneumonitis or active tuberculosis or hepatitis B or C or HIV infection; active interstitial lung disease were ineligible. Patients with mixed SCLC and NSCLC histology were also ineligible. Randomization was stratified by stage (I/II versus III) and receipt of PCI (yes versus no).

Patients were randomized 1:1:1 to receive IMFINZI as a single agent, IMFINZI in combination with another agent, or placebo. All study medications were administered intravenously. The evaluation of efficacy for LS-SCLC relied on comparison between:

- Arm 1: IMFINZI 1,500 mg in combination with placebo every 4 weeks for 4 cycles, followed by IMFINZI 1,500 mg every 4 weeks.
- Arm 2: Placebo in combination with a second placebo every 4 weeks for 4 cycles, followed by a single placebo every 4 weeks.

A total of 530 patients were randomized between Arms 1 and 2, 264 patients to the IMFINZI arm and 266 patients to the placebo arm. Treatment continued until disease progression, until

unacceptable toxicity, or for a maximum of 24 months. Tumor assessments were conducted every 8 weeks for the first 72 weeks, then every 12 weeks up to 96 weeks and then every 24 weeks thereafter.

The major efficacy outcome measures were OS and PFS assessed by BICR according to RECIST v1.1.

The baseline demographics and disease characteristics for patients in the IMFINZI and placebo arms were as follows: male (69%); age ≥ 65 years (39%); White (50%), Black or African-American (0.8%), Asian (48%), other race (1.3%); Hispanic or Latino (4.2%); current smoker (22%), past-smoker (68%), never smoker (9%); WHO/ECOG PS 0 (49%), WHO/ECOG PS 1 (51%); and Stage I (3.6%), Stage II (9%), Stage III (87%).

Prior to randomization, all patients received platinum-based chemotherapy (66% cisplatin-etoposide, 34% carboplatin-etoposide) with 88% of patients receiving 4 cycles; 72% of patients received once daily radiation (of which 92% received $\geq 60 - \leq 66$ Gy QD); 28% received twice daily radiation (of which 97% received 45 Gy twice daily) and 54% of patients received PCI.

Efficacy results are presented in Table 19 and Figure 8.

Table 19. Efficacy Results for the ADRIATIC Study

	IMFINZI (n=264)	Placebo (n=266)
OS		
Number of deaths (%)	115 (44)	146 (55)
Median OS (months) (95% CI) [†]	55.9 (37.3, NR)	33.4 (25.5, 39.9)
HR (95% CI) [‡]	0.73 (0.57, 0.93)	
p-value [§]	0.0104	
PFS [¶]		
Number of events (%)	139 (53)	169 (64)
Median PFS (months) (95% CI) [†]	16.6 (10.2, 28.2)	9.2 (7.4, 12.9)
HR (95% CI) [#]	0.76 (0.61, 0.95)	
p-value [§]	0.0161	

[†] Calculated using the Kaplan Meier technique. CI for median derived based on Brookmeyer-Crowley method.

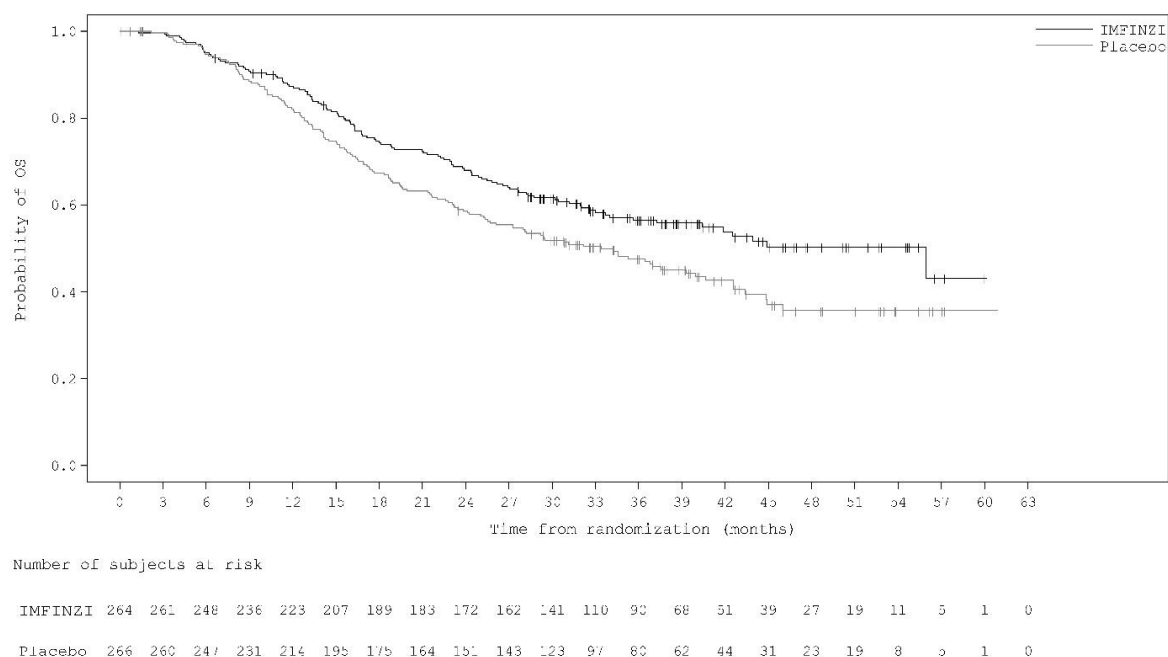
[‡] Based on Cox proportional hazards model stratified by receipt of PCI.

[§] Compared with allocated alpha of 0.0168 for OS and 0.0280 for PFS (Lan-DeMets spending function approximating O'Brien Fleming boundary) for interim analysis.

[¶] Assessed by BICR according to RECIST v1.1.

[#] Based on Cox proportional hazards model stratified by TNM stage and receipt of PCI.

Figure 8. Kaplan-Meier Curves of OS in the ADRIATIC Study



Extensive-stage SCLC – CASPIAN Study

CASPIAN was a study designed to evaluate the efficacy of IMFINZI in combination with etoposide and either carboplatin or cisplatin. CASPIAN was a randomized, open-label, multicentre study in 805 treatment naïve ES-SCLC patients with WHO/ECOG Performance status of 0 or 1, body weight >30 kg, suitable to receive a platinum-based chemotherapy regimen as first-line treatment for SCLC, with life expectancy ≥ 12 weeks, at least one target lesion by RECIST 1.1 and adequate organ and bone marrow function. Patients with asymptomatic or treated brain metastases were eligible. The study excluded patients with a history of chest radiation therapy; a history of active primary immunodeficiency; autoimmune disorders including paraneoplastic syndrome (PNS); active or prior documented autoimmune or inflammatory disorders; use of systemic immunosuppressants within 14 days before the first dose of the treatment except physiological dose of systemic corticosteroids; active tuberculosis or hepatitis B or C or HIV infection; or patients receiving live attenuated vaccine within 30 days before or after the start of IMFINZI.

Randomisation was stratified by the planned platinum-based (carboplatin or cisplatin) therapy in cycle 1.

Patients were randomised 1:1:1 to receive:

- Arm 1: IMFINZI 1500 mg + tremelimumab 75 mg + etoposide and either carboplatin or cisplatin
- Arm 2: IMFINZI 1500 mg + etoposide and either carboplatin or cisplatin
- Arm 3: Either carboplatin (AUC 5 or 6 mg/mL/min) or cisplatin (75-80 mg/m²) on Day 1 and etoposide (80-100 mg/m²) intravenously on Days 1, 2, and 3 of each 21-day cycle for between 4 – 6 cycles.

For patients randomised to Arm 1 and 2, etoposide and either carboplatin or cisplatin was limited to 4 cycles on an every 3 week schedule subsequent to randomisation. IMFINZI monotherapy continued every 4 weeks until disease progression or unacceptable toxicity. Administration of IMFINZI monotherapy was permitted beyond disease progression if the patient was clinically stable and deriving clinical benefit as determined by the investigator.

Patients randomised to Arm 3 were permitted to receive a total of up to 6 cycles of etoposide and either carboplatin or cisplatin. After completion of etoposide + platinum, PCI was permitted only in Arm 3 per investigator discretion.

Tumour assessments were conducted at Week 6 and Week 12 from the date of randomization, and then every 8 weeks until confirmed objective disease progression. Survival assessments were conducted every 2 months following treatment discontinuation.

The primary endpoints of the study were Overall Survival (OS) of IMFINZI + etoposide + platinum (Arm 2) vs. etoposide + platinum alone (Arm 3) and IMFINZI + tremelimumab + etoposide + platinum (Arm 1) vs. etoposide + platinum alone (Arm 3). The key secondary endpoint was progression-free survival (PFS). Other secondary endpoints were Objective Response Rate (ORR), OS and PFS landmarks and Patient-Reported Outcomes (PRO). PFS and ORR were assessed using Investigator assessments according to RECIST v1.1.

The demographics and baseline disease characteristics were well balanced between the two study arms (268 patients in Arm 2 and 269 patients in Arm 3). Baseline demographics of the overall study population were as follows: male (69.6%), age ≥ 65 years (39.6%), median age 63 years (range: 28 to 82 years), white (83.8%), Asian (14.5%), black or African American (0.9%), other (0.6 %), non-Hispanic or Latino (96.1%), current or past-smoker (93.1%), never smoker (6.9%), WHO/ECOG PS 0 (35.2%), WHO/ECOG PS 1 (64.8%), Stage IV 90.3%, 24.6% of the patients received cisplatin and 74.1% of the patients received carboplatin. In Arm 3, 56.8% of the patients received 6 cycles of etoposide + platinum and 7.8% of the patients received PCI.

At a planned interim (primary) analysis the study demonstrated a statistically significant improvement in OS with IMFINZI + etoposide + platinum (Arm 2) vs. etoposide + platinum alone (Arm 3) [HR=0.73 (95% CI: 0.591, 0.909), $p=0.0047$]. Although not formally tested for significance, IMFINZI + etoposide + platinum demonstrated an improvement in PFS vs. etoposide + platinum alone [HR=0.78 (95% CI: 0.645, 0.936)].

The PFS, ORR and DoR results from the planned final analysis (DCO: 27 Jan 2020) are summarized in Table 21. Kaplan-Meier curve for PFS is presented in Figure 10.

The OS results with the planned long-term OS follow-up analysis (DCO: 22 March 2021) (median follow-up: 39.3 months) are presented in Table 20. IMFINZI + etoposide + platinum (Arm 2) vs. etoposide + platinum (Arm 3) continued to demonstrate sustained improvement in OS. Kaplan-Meier curve for OS is presented in Figure 9.

Table 20. Efficacy Results for the CASPIAN Study

	Final analysis^a		Long-term follow-up analysis^b	
	Arm 2: IMFINZI + etoposide and either carboplatin or cisplatin (n=268)	Arm 3: etoposide + and either carboplatin or cisplatin (n=269)	Arm 2: IMFINZI + etoposide and either carboplatin or cisplatin (n=268)	Arm 3: etoposide + and either carboplatin or cisplatin (n=269)
OS				
Number of deaths (%)	210 (78.4)	231 (85.9)	221 (82.5)	248 (92.2)
Median OS (months) (95% CI)	12.9 (11.3, 14.7)	10.5 (9.3, 11.2)	12.9 (11.3, 14.7)	10.5 (9.3, 11.2)
HR (95% CI) ^{bc}	0.75 (0.625, 0.910)		0.71 (0.595, 0.858)	
p-value ^d	0.0032		0.0003	
OS at 18 months (%) (95% CI)	32.0 (26.5, 37.7)	24.8 (19.7, 30.1)	32.0 (26.5, 37.7)	24.8 (19.7, 30.1)
OS at 36 months (%) (95% CI)			17.6 (13.3, 22.4)	5.8 (3.4, 9.1)
PFS				
Number of events (%)	234 (87.3)	236 (87.7)		
Median PFS (months) (95% CI)	5.1 (4.7, 6.2)	5.4 (4.8, 6.2)		
HR (95% CI) ^c	0.80 (0.665, 0.959)			
PFS at 6 months (%) (95% CI)	45.4 (39.3, 51.3)	45.8 (39.5, 51.9)		
PFS at 12 months (%) (95% CI)	17.9 (13.5, 22.8)	5.3 (2.9, 8.8)		
ORR n (%) (95% CI)^e	182 (67.9) (62.0, 73.5)	156 (58.0) (51.8, 64.0)		
Complete Response n (%)	7 (2.6)	2 (0.7)		
Partial Response n (%)	175 (65.3)	154 (57.2)		
Median DoR (months) (95% CI)^{e,f}	5.1 (4.9, 5.3)	5.1 (4.8, 5.3)		

^a Final PFS, ORR and DoR analysis at data cut-off 27 January 2020.

^b Long-term follow-up OS analysis at data cut-off 22 March 2021.

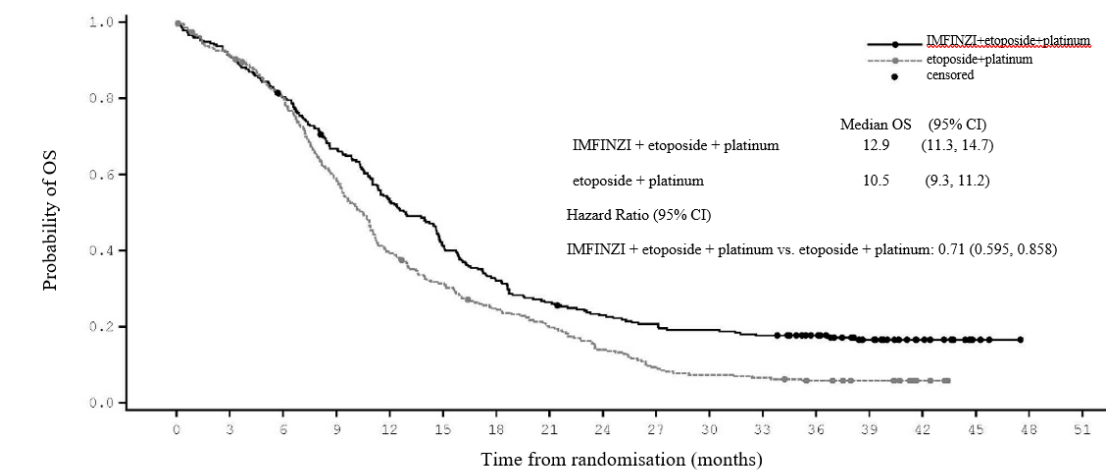
^c The analysis was performed using the stratified log-rank test, adjusting for planned platinum therapy in Cycle 1 (carboplatin or cisplatin), and using the rank tests of association approach.

^d At the interim analysis (data cut-off 11 March 2019) the OS p-value was 0.0047, which met the boundary for declaring statistical significance of 0.0178 for a 4% overall 2-sided alpha, based on a Lan-DeMets alpha spending function with O'Brien Fleming type boundary with the actual number of events observed.

^e Confirmed Objective Response.

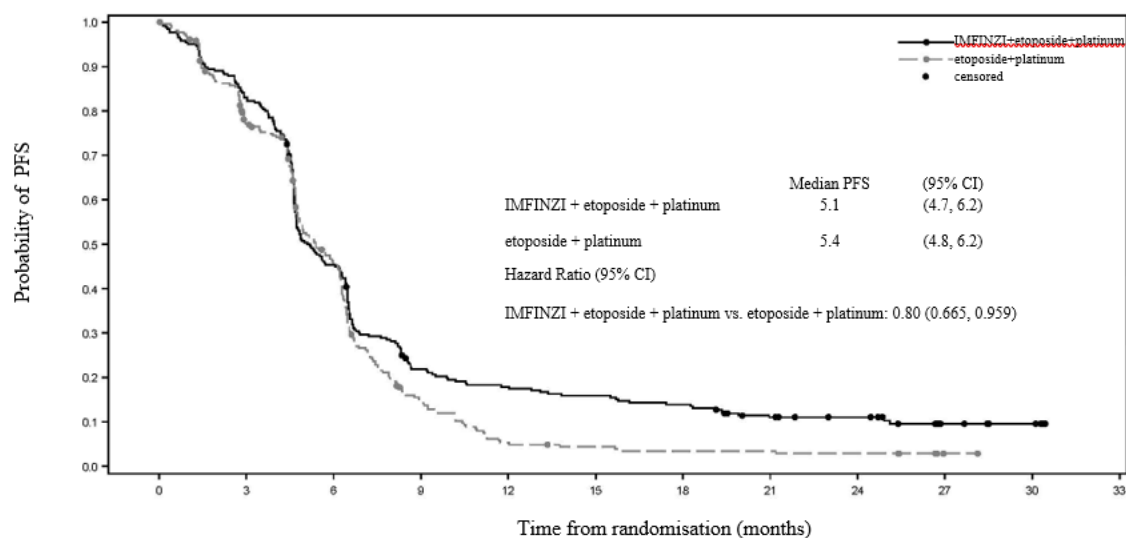
^f Post-hoc analysis.

Figure 9. Kaplan-Meier curve of OS



Number of patients at risk	0	3	6	9	12	15	18	21	24	27	30	33	36	39	42	45	48	51
IMFINZI + etoposide + platinum	268	244	214	177	140	109	85	70	60	54	50	46	39	25	13	3	0	0
etoposide + platinum	269	243	212	156	104	82	64	51	36	24	19	17	13	10	3	0	0	0

Figure 10. Kaplan-Meier curve of PFS



Number of patients at risk	0	3	6	9	12	15	18	21	24	27	30	33
IMFINZI + etoposide + platinum	268	220	119	55	45	40	35	24	18	8	5	0
etoposide + platinum	269	195	110	33	12	9	7	7	6	1	0	0

Subgroup analysis

The improvements in OS in favour of patients receiving IMFINZI + etoposide + platinum compared to those receiving etoposide + platinum alone, were consistently observed across the prespecified subgroups based on demographics, geographical region, carboplatin or cisplatin use and disease characteristics.

Metastatic NSCLC – POSEIDON Study

POSEIDON was a study designed to evaluate the efficacy of IMFINZI with or without tremelimumab in combination with platinum-based chemotherapy. POSEIDON was a randomized, open-label, multicenter study in 1013 metastatic NSCLC patients with no sensitizing epidermal growth factor receptor (EGFR) mutation or anaplastic lymphoma kinase (ALK) genomic tumour aberrations. Patients with histologically or cytologically documented metastatic NSCLC were eligible for enrolment. Patients had no prior chemotherapy or any other systemic therapy for metastatic NSCLC. Prior to randomization, patients had tumour PD-L1 status confirmed by using the Ventana PD-L1 (SP263) assay. Patients had a World Health Organization (WHO)/Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1 at enrolment.

The study excluded patients with active or prior documented autoimmune disease; active and/or untreated brain metastases; a history of immunodeficiency; administration of systemic immunosuppression within 14 days before the start of IMFINZI or tremelimumab, except physiological dose of systemic corticosteroids; active tuberculosis or hepatitis B or C or HIV infection; or patients receiving live attenuated vaccine within 30 days before or after the start of IMFINZI and/or tremelimumab.

Randomisation was stratified by tumour cells (TC) PD-L1 expression (TC $\geq$ 50% vs. TS < 50%), disease stage (Stage IVA vs. Stage IVB, per the 8th edition of American Joint Committee on Cancer), and histology (non-squamous and squamous).

Patients were randomized 1:1:1 to receive:

- Arm 1: IMFINZI 1500 mg with tremelimumab 75 mg and platinum-based chemotherapy every 3 weeks for 4 cycles followed by, IMFINZI 1500 mg every 4 weeks as monotherapy. A fifth dose of tremelimumab 75 mg was given at Week 16 alongside IMFINZI dose 6.
- Arm 2: IMFINZI 1500 mg and platinum-based chemotherapy every 3 weeks for 4 cycles, followed by IMFINZI 1500 mg every 4 weeks as monotherapy.
- Arm 3: Platinum-based chemotherapy every 3 weeks for 4 cycles. Patient could receive 2 additional cycles (a total of 6 cycles post-randomisation), as clinically indicated, at the Investigator's discretion.

In the 3 treatment arms, patients received one of the following histology-based chemotherapy regimens:

- Non-squamous NSCLC
 - Pemetrexed 500 mg/m² with carboplatin AUC 5-6 or cisplatin 75 mg/m² every 3 weeks. Unless contraindicated by the investigator, pemetrexed maintenance could be given.
- Squamous NSCLC
 - Gemcitabine 1000 or 1250 mg/m² on Days 1 and 8 with cisplatin 75 mg/m² or carboplatin AUC 5-6 on Day 1 every 3 weeks.
- Non-squamous or squamous NSCLC
 - Nab-paclitaxel 100 mg/m² on Days 1, 8, and 15 with carboplatin AUC 5-6 on Day 1 every 3 weeks.

Tremelimumab was given up to a maximum of 5 doses unless there was disease progression or unacceptable toxicity, IMFINZI and histology-based pemetrexed maintenance therapy (when applicable) was continued until disease progression or unacceptable toxicity.

Tumour assessments were conducted at Week 6 and Week 12 from the date of randomization, and then every 8 weeks until confirmed objective disease progression. Survival assessments were conducted every 2 months following treatment discontinuation.

The dual primary endpoints of the study were progression-free survival (PFS) and overall survival (OS) for IMFINZI + platinum-based chemotherapy vs. platinum-based chemotherapy alone. The key secondary endpoints of the study were PFS and OS for IMFINZI + tremelimumab + platinum-based chemotherapy and platinum-based chemotherapy alone. The secondary endpoints included objection response rate (ORR) and Duration of Response (DoR). PFS, ORR, and DoR, were assessed using Blinded Independent Central Review (BICR) according to RECIST v1.1.

The demographic and baseline disease characteristics were well-balanced between study arms. Baseline demographic of the overall study population were as follows: male (76.0%), age $\geq$ 65 years (47.1%), age $\geq$ 75 years (11.3%) median age 64 years (range: 27 to 87 years), White (55.9%), Asian (34.6%), Black or African American (2.0%), Other (7.6%), non-Hispanic or Latino (84.2%), current smoker or past-smoker (78.0%), WHO/ECOG PS 0 (33.4%), WHO/ECOG PS 1 (66.5%). Disease characteristics as follows: Stage IVA (50.0%), Stage IVB (49.6%), histological sub-groups of squamous (36.9%), non-squamous (62.9%), brain metastases (10.5%), PD-L1 expression TC $\geq$ 50% (28.8), PD-L1 expression TC < 50% (71.1%).

The study showed a statistically significant improved in OS with IMFINZI + tremelimumab + platinum-based chemotherapy vs. platinum-based chemotherapy. IMFINZI + tremelimumab + platinum-based chemotherapy showed a statistically significant improvement in PFS vs. platinum-based chemotherapy alone. The results are summarized below:

Table 21. Efficacy results for the POSEIDON study

	Arm 1: IMFINZI+tremelimumab +platinum-based chemotherapy (N=338)	Arm 3: Platinum-based chemotherapy (N=337)
OS ^a		
Number of deaths (%)	251 (74.3)	285 (84.6)
Median OS (months) (95% CI)	14.0 (11.7, 16.1)	11.7 (10.5, 13.1)
HR (95% CI) ^b	0.77 (0.650, 0.916)	
p-value ^c	0.00304	
PFS ^a		
Number of events (%)	238 (70.4)	258 (76.6)
Median PFS (months) (95% CI)	6.2 (5.0, 6.5)	4.8 (4.6, 5.8)

	Arm 1: IMFINZI+tremelimumab +platinum-based chemotherapy (N=338)	Arm 3: Platinum-based chemotherapy (N=337)
HR (95% CI) ^b	0.72 (0.600, 0.860)	
p-value ^c	0.00031	
ORR n (%) ^{d,e}	130 (38.8)	81 (24.4)
Complete Response n (%)	2 (0.6)	0
Partial Response n (%)	128 (38.2)	81 (24.4)
Median DoR (months) 95% CI ^{d,e}	9.5 (7.2, NR)	5.1 (4.4, 6.0)

^a Analysis of PFS at data cut off 24 July 2019 (median follow up 10.15 months). Analysis of OS at data cut off 12 March 2021 (median follow up 34.86 months). The boundaries for declaring efficacy (Arm 1 vs. Arm 3: PFS 0.00735, OS 0.00797; 2-sided) were determined by a Lan-DeMets alpha spending function that approximates an O'Brien Fleming approach. PFS was assessed by BICR according to RECIST v1.1.

^b HR are derived using a Cox pH model stratified by PD-L1, histology and disease stage.

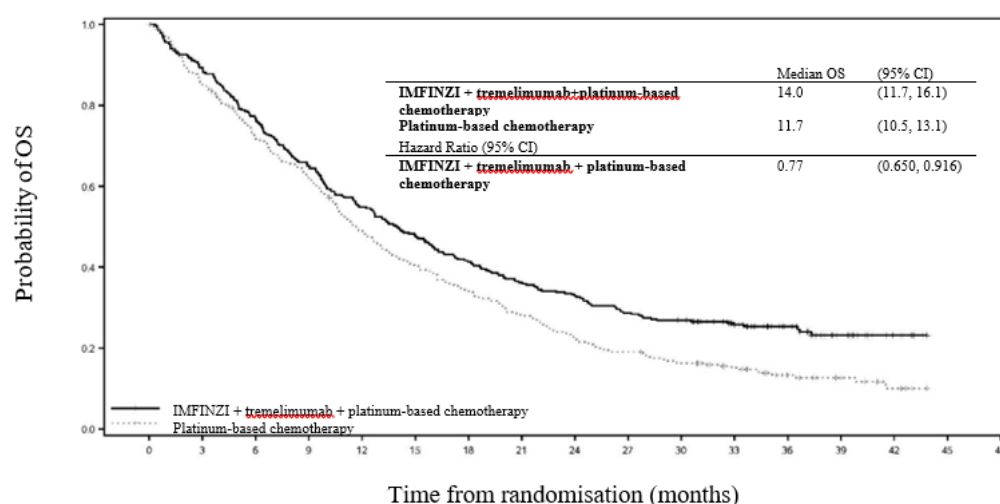
^c 2-sided p-value based on a log-rank test stratified by PD-L1, histology and disease stage.

^d Confirmed Objective Response.

^e Post-hoc analysis.

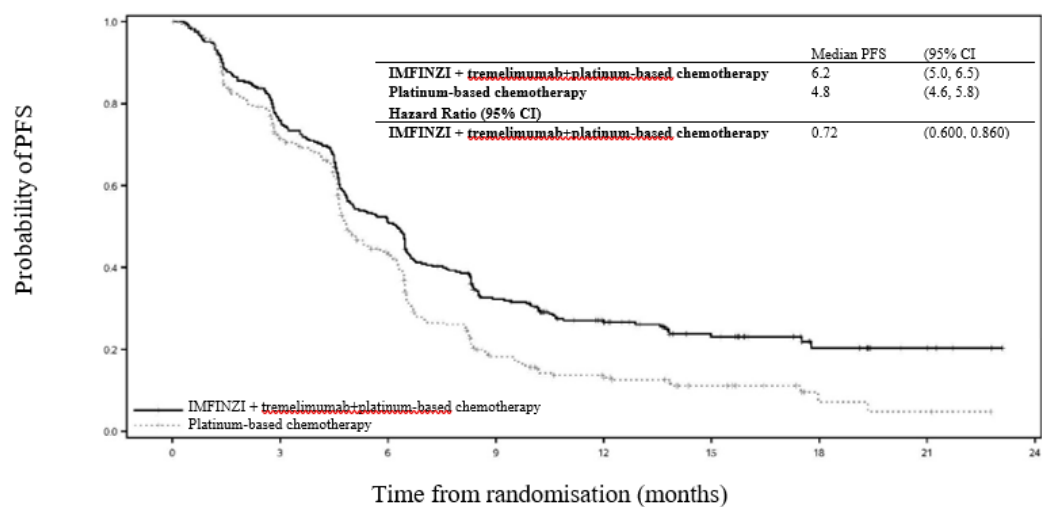
NR = Not Reached, CI = Confidence Interval

Figure 11. Kaplan-Meier curve of OS



Number of patients at risk															
Month	0	3	6	9	12	15	18	21	24	27	30	33	36	39	42
IMFINZI + tremelimumab + platinum-based chemotherapy	338	298	256	217	183	159	137	120	109	95	88	64	41	20	9
Platinum-based chemotherapy	337	284	236	204	160	132	111	91	72	62	52	38	21	13	6

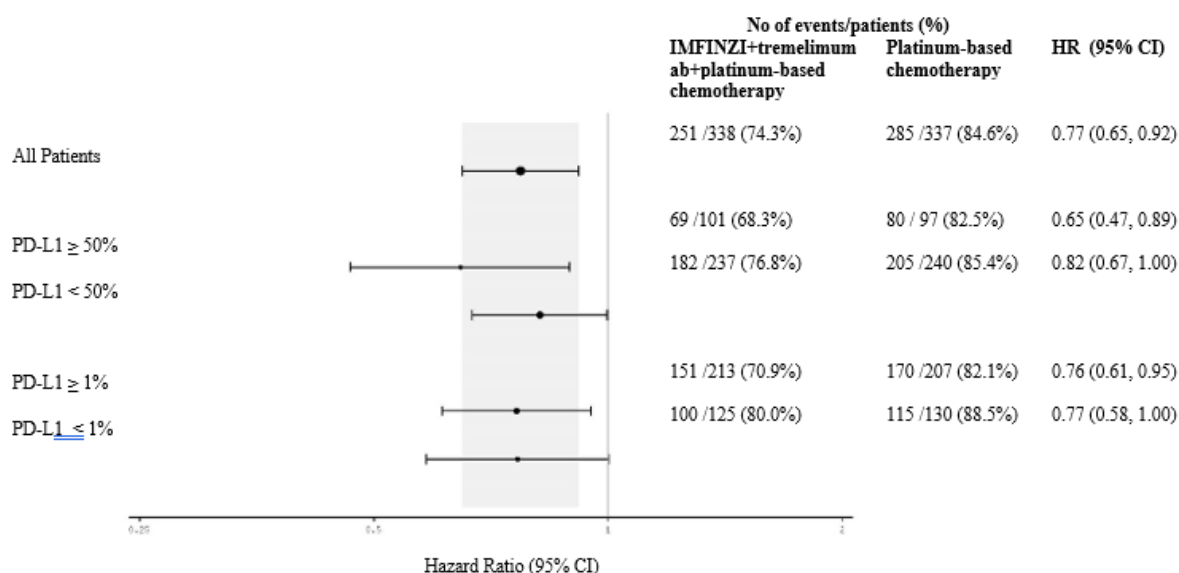
Figure 12. Kaplan-Meier curve of PFS



Number of patients at risk									
Month	0	3	6	9	12	15	18	21	24
IMFINZI + tremelimumab + platinum-based chemotherapy	338	243	161	94	56	32	13	5	0
Platinum-based chemotherapy	337	219	121	43	23	12	3	2	0

Figure 13 summarises efficacy results of OS by tumour PD-L1 expression in prespecified subgroup analyses.

Figure 13. Forest plot of OS by PD-L1 expression for IMFINZI+tremelimumab+platinum-based chemotherapy vs. platinum-based chemotherapy



Elderly population

A total of 75 patients aged ≥ 75 years were enrolled in the IMFINZI in combination with tremelimumab and chemotherapy (n=35) and platinum-based chemotherapy only (n=40) arms of the POSEIDON study. An exploratory HR of 1.05 (95% CI: 0.64, 1.71) for OS was observed for the IMFINZI in combination with tremelimumab and platinum-based chemotherapy vs. platinum-based chemotherapy within this study subgroup. Due to the exploratory nature of this subgroup analysis no definitive conclusions can be drawn, but caution is suggested when considering this regimen for elderly patients.

Biliary Tract Cancer (BTC)

Locally Advanced or Metastatic BTC – TOPAZ-1 Study

TOPAZ-1 was a study designed to evaluate the efficacy of IMFINZI in combination with gemcitabine and cisplatin. TOPAZ-1 was a randomised, double-blind, placebo-controlled, multicentre study in 685 patients with unresectable or metastatic BTC (including intrahepatic and extrahepatic cholangiocarcinoma and gallbladder carcinoma) and ECOG Performance status of 0 or 1. Patients had not received previous therapy in the advanced/unresectable setting. Patients who developed recurrent disease > 6 months after surgery and/or completion of adjuvant therapy were included. Patients must have had an adequate organ and bone marrow function, and have had acceptable serum bilirubin levels ($\leq 2.0 \times$ the upper limit of normal (ULN)), and any clinically significant biliary obstruction had to be resolved before randomisation.

The study excluded patients with ampullary carcinoma, with brain metastases, active or prior documented autoimmune or inflammatory disorders, HIV infection or active infections, including tuberculosis or hepatitis C or patients with current or prior use of immunosuppressive medication within 14 days before the first dose of IMFINZI. Patients with active HBV were allowed to participate if they were on antiviral therapy.

Randomisation was stratified by disease status (initially unresectable vs. recurrent) and primary tumour location (intrahepatic cholangiocarcinoma vs. extrahepatic cholangiocarcinoma vs. gallbladder carcinoma).

Patients were randomised 1:1 to receive:

- Arm 1: IMFINZI 1 500 mg administered on Day 1 + gemcitabine 1 000 mg/m² and cisplatin 25 mg/m² (each administered on Days 1 and 8) every 3 weeks (21 days) for up to 8 cycles, followed by IMFINZI 1 500 mg every 4 weeks until disease progression or unacceptable toxicity, or
- Arm 2: Placebo administered on Day 1 + gemcitabine 1 000 mg/m² and cisplatin 25 mg/m² (each administered on Days 1 and 8) every 3 weeks (21 days) for up to 8 cycles, followed by placebo every 4 weeks until disease progression or unacceptable toxicity.

Tumour assessments were conducted every 6 weeks for the first 24 weeks after the date of randomisation, and then every 8 weeks until confirmed objective disease progression.

The primary endpoint of the study was OS, the key secondary endpoint was PFS. Other secondary endpoints were ORR, DoR and PRO. PFS, ORR and DoR were investigator-assessed according to RECIST v1.1.

The demographics and baseline disease characteristics were well balanced between the two study arms (341 patients in Arm 1 and 344 patients in Arm 2). Baseline demographics of the overall study population were as follows: male (50.4%), age < 65 years (53.3%), white (37.2%), Asian (56.4%), Black or African American (2.0%), other (4.2%), non-Hispanic or Latino (93.1%), ECOG PS 0 (49.1%), vs. PS 1 (50.9%), primary tumour location (intrahepatic bile duct 55.9%, extrahepatic bile duct 19.1% and gallbladder 25.0%), disease status [recurrent (19.1%) vs. unresectable (80.7%), metastatic (86.0%) vs. locally advanced (13.9%)]. PD-L1 expression was evaluated on tumour and immune cells using the Ventana PD-L1 (SP263) assay and the TAP (tumour area positivity) algorithm, 58.7% patients had TAP $\geq$ 1% and 30.1% TAP < 1%.

OS and PFS were formally tested at a pre-planned interim analysis (data cut-off 11 Aug 2021) after a median follow-up of 9.8 months. Efficacy results are shown in Table 22 and Figure 14. The maturity for OS was 62% and the maturity for PFS was 84%. IMFINZI + chemotherapy (Arm 1) showed statistically significant improvement vs. placebo + chemotherapy (Arm 2) in OS and in PFS.

Table 22. Efficacy Results for the TOPAZ-1 Study^a

	IMFINZI + gemcitabine and cisplatin (N=341)	Placebo + gemcitabine and cisplatin (N=344)
OS		
Number of deaths (%)	198 (58.1)	226 (65.7)
Median OS (months) (95% CI)^b	12.8 (11.1, 14.0)	11.5 (10.1, 12.5)
HR (95% CI) ^c	0.80 (0.66, 0.97)	
p-value ^{c,d}	0.021	
Median follow-up in all patients (months)	10.2	9.5
PFS		
Number of events (%)	276 (80.9)	297 (86.3)
Median PFS (months) (95% CI)^b	7.2 (6.7, 7.4)	5.7 (5.6, 6.7)
HR (95% CI) ^c	0.75 (0.63, 0.89)	
p-value ^{c,e}	0.001	
Median follow-up in all patients (months)	7.2	5.6
ORR^f	91 (26.7)	64 (18.7)
Complete Response n (%)	7 (2.1)	2 (0.6)
Partial Response n (%)	84 (24.6)	62 (18.1)
DoR		
Median DoR (months) (95% CI)^b	6.4 (5.9, 8.1)	6.2 (4.4, 7.3)

^a Analysis at data cut-off 11 August 2021.

^b Calculated using the Kaplan-Meier technique. CI for median derived based on Brookmeyer-Crowley method.

^c The analysis for HR was performed using a stratified Cox proportional hazards model and 2-sided p-value is based on a stratified log-rank test, both are adjusted for disease status and primary tumour location.

^d At the interim analysis (data cut-off 11 August 2021) the OS p-value was 0.021, which met the boundary for declaring statistical significance of 0.03 for a 4.9% overall 2-sided alpha, based on a Lan-DeMets alpha spending function with O'Brien Fleming type boundary with the actual number of events observed.

^e At the interim analysis (data cut-off 11 August 2021) the PFS p-value was 0.001, which met the boundary for declaring statistical significance of 0.0481 for a 4.9% overall 2-sided alpha, based on a Lan-DeMets alpha spending function with Pocock-type boundary with the actual number of events observed.

^f Confirmed objective response.

An additional planned follow-up analysis of OS (data cut-off 25 Feb 2022) was performed 6.5 months after the interim analysis with an OS maturity of 77%. IMFINZI + chemotherapy continued to demonstrate improved OS vs. chemotherapy alone [HR=0.76, (95% CI: 0.64, 0.91)] and the median follow-up increased to 12 months.

Figure 14: Kaplan-Meier curve of OS, follow-up OS analysis at data cut-off 25 February 2022

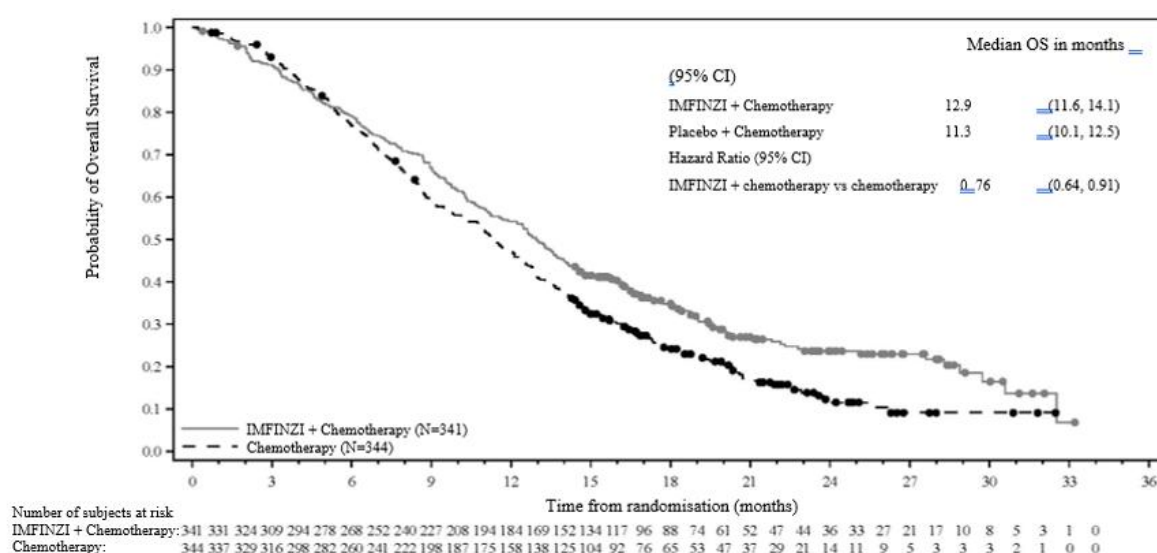
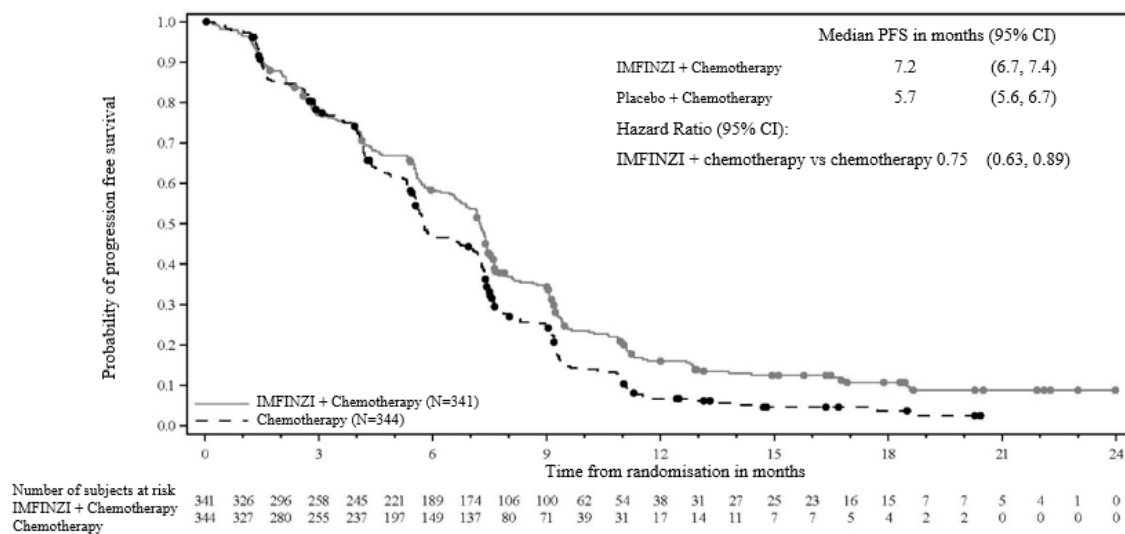


Figure 15: Kaplan-Meier curve of PFS, inferential (primary) analysis at data cut-off 11 August 2021



Hepatocellular Carcinoma (HCC)

Unresectable HCC – HIMALAYA Study

The efficacy of IMFINZI given in combination with a single dose of tremelimumab 300 mg was evaluated in the HIMALAYA Study, a randomization, open-label, multicentre study in patients with confirmed uHCC who did not receive prior systemic treatment for HCC. The study included patients with Barcelona Clinic Liver Cancer (BCLC) Stage C or B (not eligible for locoregional therapy) and Child-Pugh Score Class A.

The study excluded patients with brain metastases or a history of brain metastases, co-infection of viral hepatitis B and hepatitis C; active or prior documented gastrointestinal (GI) bleeding within 12 months; ascites requiring non-pharmacologic intervention within 6 months; hepatic encephalopathy within 12 months before the start of treatment; active or prior documented autoimmune or inflammatory disease.

Patients with oesophageal varices were included except those with active or prior documented GI bleeding within 12 months prior to study entry.

Randomisation was stratified by macrovascular invasion (MVI) (yes vs. no), aetiology of liver disease (confirmed hepatitis B virus vs. confirmed hepatitis C virus vs. others) and ECOG performance status (0 vs. 1). The HIMALAYA study randomized 1171 patients 1:1:1 to receive:

- IMFINZI: durvalumab 1500 mg every 4 weeks
- Tremelimumab 300 mg as a single dose + IMFINZI 1500 mg; followed by IMFINZI 1500 mg every 4 weeks.
- S: Sorafenib 400 mg twice daily

Tumour assessments were conducted every 8 weeks for the first 12 months and then every 12 weeks thereafter. Survival assessments were conducted every month for the first 3 months following treatment discontinuation and then every 2 months.

The primary endpoint was OS. Secondary endpoints included PFS, Investigator-assessed ORR and DoR according to RECIST v1.1.

The demographics and baseline disease characteristics were well balanced between study arms. The baseline demographics of the overall study population were as follows: male (83.7%), age < 65 years (50.4%), White (44.6%), Asian (50.7%), Black or African American (1.7%), Other race (2.3%), ECOG PS 0 (62.6%); Child-Pugh Class score A (99.5%), macrovascular invasion (25.2%), extrahepatic spread (53.4%), baseline AFP < 400 ng/mL (63.7%), baseline AFP ≥ 400 ng/mL (34.5%), viral aetiology; hepatitis B (30.6%), hepatitis C (27.2%), uninfected (42.2%), evaluable PD-L1 data (86.3%), PD-L1 Tumour area positivity (TAP) ≥ 1% (38.9%), PD-L1 TAP < 1% (48.3%) [Ventana PD-L1 (SP263) assay].

Results are presented in Table 23 and Figure 16.

Table 23. Efficacy Results for the HIMALAYA Study for IMFINZI given in combination with a single dose of tremelimumab 300 mg vs. S

	IMFINZI + tremelimumab 300 mg (n=393)	S (n=389)
Follow-up duration		
Median follow-up (months) ^a	33.2	32.2
OS		
Number of deaths (%)	262 (66.7)	293 (75.3)
Median OS (months) (95% CI)	16.4 (14.2, 19.6)	13.8 (12.3, 16.1)
HR (95% CI)	0.78 (0.66, 0.92)	
p-value ^b	0.0035	
PFS		
Number of events (%)	335 (85.2)	327 (84.1)
Median PFS (months) (95% CI)	3.78 (3.68, 5.32)	4.07 (3.75, 5.49)
HR (95% CI)	0.90 (0.77, 1.05)	
ORR		
ORR n (%) ^c	79 (20.1)	20 (5.1)
Complete Response n (%)	12 (3.1)	0
Partial Response n (%)	67 (17.0)	20 (5.1)
DoR		
Median DoR (months)	22.3	18.4

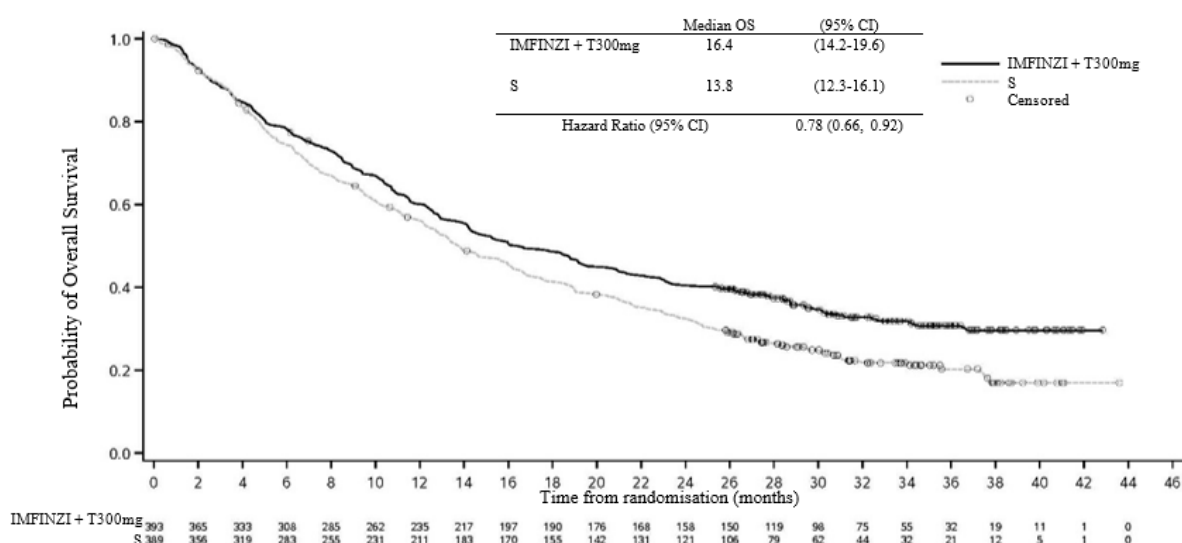
^a Calculated using reverse the Kaplan-Meier technique (with censor indicator reversed).

^b Based on a Lan-DeMets alpha spending function with O'Brien Fleming type boundary and the actual number of events observed, the boundary for declaring statistical significance for IMFINZI + tremelimumab 300 mg vs. S was 0.0398 (Lan and DeMets 1983).

^c Confirmed complete response.

NR=Not Reached, CI=Confidence Interval

Figure 16. Kaplan-Meier curve of OS



Endometrial Cancer

Advanced or Recurrent Endometrial Cancer – DUO-E Study

DUO-E was a randomised, multicentre, double-blind, placebo-controlled Phase III study of first-line platinum-based chemotherapy in combination with IMFINZI, followed by IMFINZI with or without olaparib in patients with advanced or recurrent endometrial cancer. Patients had to have endometrial cancer in one of the following categories: newly diagnosed Stage III disease (measurable disease per RECIST v1.1 following surgery or diagnostic biopsy), newly diagnosed Stage IV disease (with or without disease following surgery or diagnostic biopsy), or recurrence of disease (measurable or nonmeasurable disease per RECIST v1.1) where the potential for cure by surgery alone or in combination is poor. For patients with recurrent disease, prior chemotherapy was allowed only if it was administered in the adjuvant setting and there was at least 12 months from the date of last dose of chemotherapy administered to the date of subsequent relapse. The study included patients with epithelial endometrial carcinomas of all histologies, including carcinosarcomas. Patients with endometrial sarcoma were excluded.

Randomisation was stratified by tumour tissue's mismatch repair (MMR) status (proficient versus deficient), disease status (recurrent versus newly diagnosed), and geographic region (Asia versus rest of the world). Patients were randomised 1:1:1 to one of the following arms:

- **Arm 1 (Platinum-based chemotherapy):** Platinum-based chemotherapy (paclitaxel and carboplatin) every 3 weeks for a maximum of 6 cycles with durvalumab placebo every 3 weeks. Following completion of chemotherapy treatment, patients without objective disease progression received durvalumab placebo every 4 weeks and olaparib placebo tablets twice daily as maintenance treatment until disease progression.
- **Arm 2 (Platinum-based chemotherapy + IMFINZI):** Platinum-based chemotherapy (paclitaxel and carboplatin) every 3 weeks for a maximum of 6 cycles with 1 120 mg durvalumab every 3 weeks. Following completion of chemotherapy treatment, patients without objective disease progression received 1 500 mg durvalumab every 4 weeks with olaparib placebo tablets twice daily as maintenance treatment until disease progression.

- Arm 3 (Platinum-based chemotherapy + IMFINZI + olaparib): Platinum-based chemotherapy (paclitaxel and carboplatin) every 3 weeks for a maximum of 6 cycles with 1 120 mg durvalumab every 3 weeks. Following completion of chemotherapy treatment, patients without objective disease progression received 1 500 mg durvalumab every 4 weeks with 300 mg olaparib tablets twice daily as maintenance treatment until disease progression.

Patients who discontinued either product (IMFINZI/placebo or olaparib/placebo) for reasons other than disease progression could continue treatment with the other product if appropriate based on toxicity considerations and investigator discretion.

Treatment was continued until RECIST v1.1-defined progression of disease or unacceptable toxicity. Assessment of tumour status was performed every 9 weeks for the first 18 weeks relative to randomisation and every 12 weeks thereafter.

The primary endpoint was PFS, determined by investigator assessment using RECIST v1.1. Secondary efficacy endpoints included OS, ORR and DoR.

The study demonstrated a statistically significant improvement in PFS in the ITT population, for patients treated with platinum-based chemotherapy + IMFINZI + olaparib compared to platinum-based chemotherapy [HR=0.55 (95% CI: 0.43, 0.69), $p < 0.0001$], and for patients treated with platinum-based chemotherapy + IMFINZI compared to platinum-based chemotherapy [HR=0.71 (95% CI: 0.57, 0.89), $p = 0.003$]. At the time of PFS analysis, interim OS data were 28% mature with events in 199 of 718 patients.

Mismatch repair (MMR) status was determined centrally using an MMR immunohistochemistry panel assay. Of a total of 718 patients randomized in the study, 575 (80%) patients had MMR-proficient (pMMR) tumour status and 143 (20%) patients had MMR-deficient (dMMR) tumour status.

Patients with MMR-deficient (dMMR) endometrial cancer

Among patients with dMMR tumour status, demographic and baseline characteristics were generally well balanced between the treatment arms. Baseline demographics across all three arms were as follows: median age of 62 years (range: 34 to 85), 41% age 65 or older, 1.5% age 75 or older, 62% White, 29% Asian, and 2% Black or African American. Disease characteristics were as follows: ECOG PS of 0 (58%) or 1 (42%), 46% newly diagnosed and 54% recurrent disease. The histologic subtypes were endometrioid (83%), mixed epithelial (5%), serous (3%), carcinosarcoma (3%), undifferentiated (2%), and other (3%).

In patients with dMMR tumour status, the results are summarised in Table 24 and Figure 17. The median follow-up time for PFS in censored patients with dMMR tumour status was 15.5 months in the platinum-based chemotherapy + IMFINZI arm and 10.2 months in the platinum-based chemotherapy arm. At the time of PFS analysis, interim OS data were 26% mature with events in 25 of 95 patients treated with platinum-based chemotherapy + IMFINZI and platinum-based chemotherapy.

Table 24. Efficacy results for the DUO-E Study (Patients with dMMR tumour status)

	Platinum-based chemotherapy + IMFINZI N=46	Platinum-based chemotherapy N=49
PFS^{a,b}		
Number of events (%)	15 (32.6)	25 (51.0)
Median PFS (months) (95% CI)^c	NR (NR, NR)	7.0 (6.7, 14.8)
HR (95% CI)	0.42 (0.22, 0.80)	-
OS^b		
Number of events (%)	7 (15.2)	18 (36.7)
Median OS (months) (95% CI)^c	NR (NR, NR)	23.7 (16.9, NR)
HR (95% CI)	0.34 (0.13, 0.79)	-
ORR^b		
ORR ^d n (%)	30 (71.4)	17 (40.5)
DoR^b		
Median DoR (months) (95% CI)^c	NR (NR, NR)	10.5 (4.3, NR)

^a Investigator assessed.

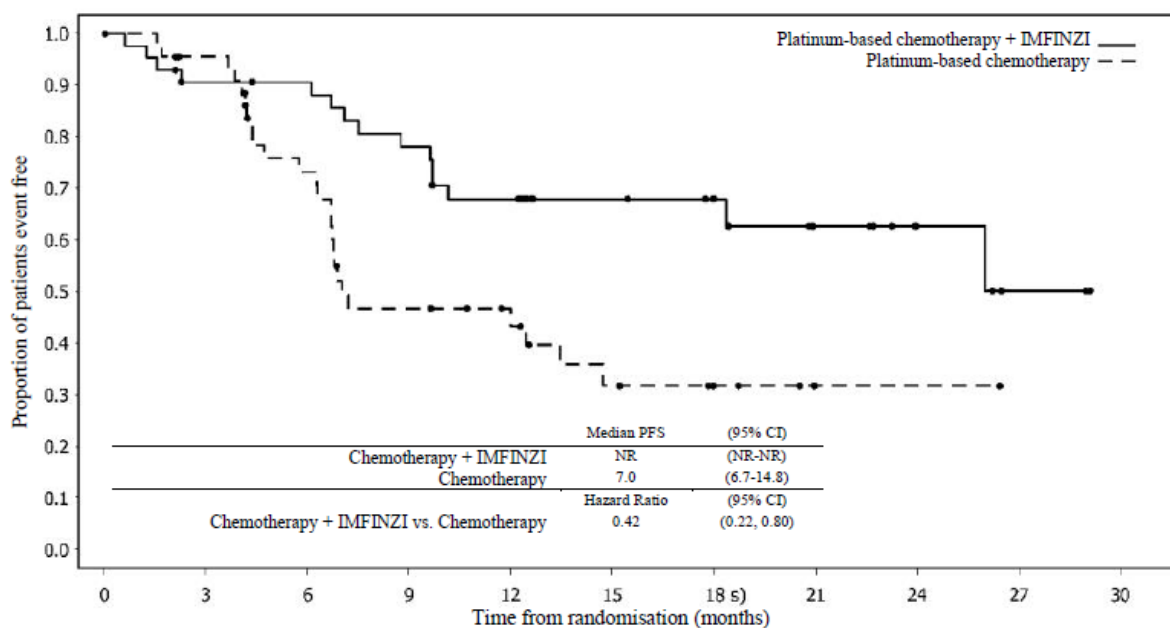
^b Results are based on the first interim analysis (DCO: 12 April 2023).

^c Calculated using the Kaplan-Meier technique.

^d Response: Best objective response as confirmed complete response or partial response. Based on number of patients in treatment group with measurable disease at baseline (N=42 in platinum-based chemotherapy + IMFINZI arm, N=42 in platinum-based chemotherapy arm).

CI=Confidence Interval, HR=Hazard Ratio, NR=Not Reached

Figure 17. Kaplan-Meier curve of PFS in DUO-E (Patients with dMMR tumour status)



Number of patients at risk:

Platinum-based chemotherapy + IMFINZI										
46	37	36	31	26	19	14	9	5	2	0
Platinum-based chemotherapy										
49	41	28	17	13	8	5	2	2	0	0

Patients with MMR-proficient (pMMR) endometrial cancer

Among patients with pMMR tumour status, demographic and baseline characteristics were generally well balanced between the treatment arms. Baseline demographics across all three arms were as follows: median age of 64 years (range: 22 to 86), 48% age 65 or older, 8.1% age 75 or older, 56% White, 30% Asian, and 6% Black or African American. Disease characteristics were as follows: ECOG PS of 0 (69%) or 1 (31%), 47% newly diagnosed and 53% recurrent disease. The histologic subtypes were endometrioid (54%), serous (26%), carcinosarcoma (8%), mixed epithelial (4%), clear cell (3%), undifferentiated (2%), mucinous (<1%), and other (3%).

Results in patients with pMMR tumour status are summarised in Table 25 and Figure 18. The median follow-up time in censored patients with pMMR tumour status was 15.2 months in the platinum-based chemotherapy + IMFINZI + olaparib arm, and 12.8 months in the platinum-based chemotherapy arm. At the time of PFS analysis, interim OS data were 29% mature with events in 110 of 383 patients treated with platinum-based chemotherapy + IMFINZI + olaparib and platinum-based chemotherapy.

Table 25. Efficacy results for the DUO-E Study (Patients with pMMR tumour status)

	Platinum-based chemotherapy + IMFINZI + olaparib N=191	Platinum-based chemotherapy N=192
PFS^{a,b}		
Number of events (%)	108 (56.5)	148 (77.1)
Median PFS (months) (95% CI)^c	15.0 (12.4, 18.0)	9.7 (9.2, 10.1)
HR (95% CI)	0.57 (0.44, 0.73)	-
OS^b		
Number of events (%)	46 (24.1)	64 (33.3)
Median OS (months) (95% CI)^c	NR (NR, NR)	25.9 (25.1, NR)
HR (95% CI)	0.69 (0.47, 1.00)	-
ORR^b		
ORR ^d n (%)	90 (61.2)	92 (59.0)
DoR^b		
Median DoR (months) (95% CI)^c	18.7 (10.5, NR)	7.6 (7.1, 10.2)

^a Investigator assessed.

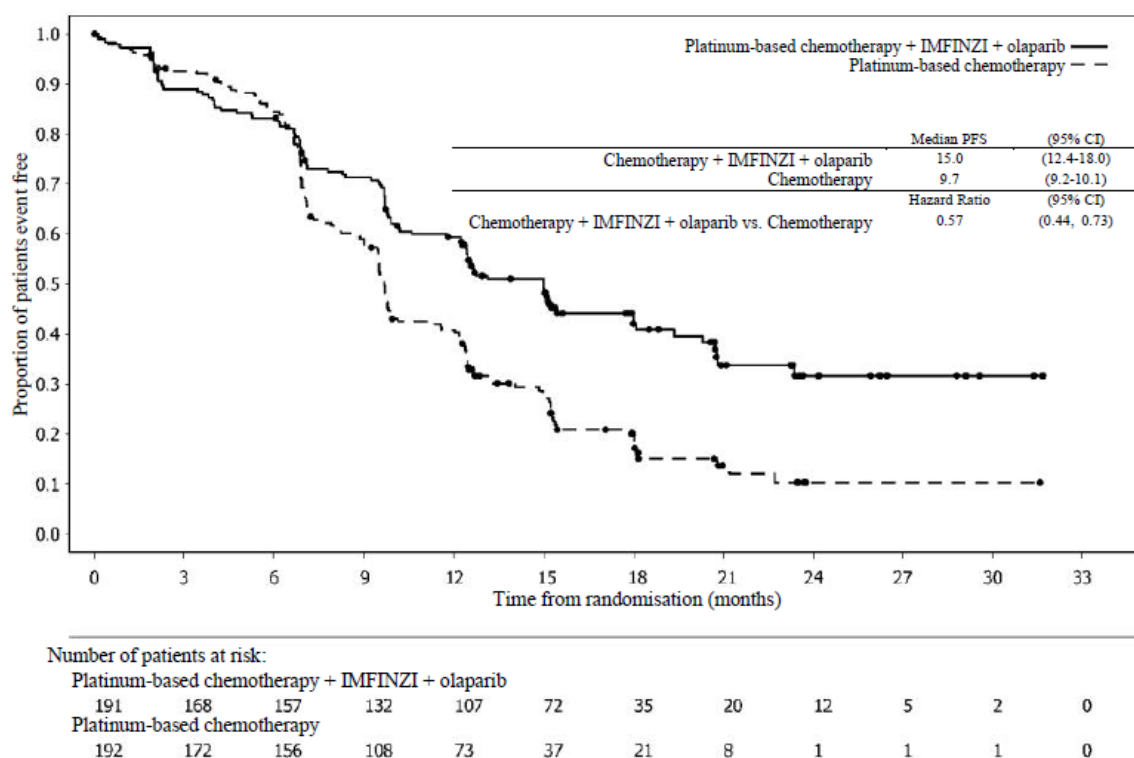
^b Results are based on the first interim analysis (DCO: 12 April 2023).

^c Calculated using the Kaplan-Meier technique.

^d Response: Best objective response as confirmed complete response or partial response. Based on number of patients in treatment group with measurable disease at baseline (N=147 in platinum-based chemotherapy + IMFINZI + olaparib arm, N=156 in platinum-based chemotherapy arm).

CI=Confidence Interval, HR=Hazard Ratio, NR=Not Reached

Figure 18. Kaplan-Meier curve of PFS in DUO-E (Patients with pMMR tumour status)



Among patients with pMMR tumour status, the PFS HRs were 0.44 (95% CI: 0.31, 0.61) in patients with PD-L1 expression positive status (236/383; 62%) and 0.87 (95% CI: 0.59, 1.28) in patients with PD-L1 expression negative status (140/383; 37%), for the platinum-based chemotherapy + IMFINZI + olaparib arm compared to the platinum-based chemotherapy arm. PD-L1 expression positive was defined as tumour area positive (TAP) $\geq 1\%$.

Muscle Invasive Bladder Cancer (MIBC)

MIBC – NIAGARA Study

The efficacy of neoadjuvant IMFINZI in combination with gemcitabine and cisplatin followed by adjuvant IMFINZI as a single agent in patients with MIBC was evaluated in NIAGARA (NCT03732677), a randomized, open-label, multicenter study. The study randomized (1:1) 1,063 patients who were candidates for radical cystectomy and who had not received prior systemic chemotherapy or immune-mediated therapy for the treatment of NMIBC or MIBC. The study excluded patients with pure non-urothelial histology, any small cell histology and primary non-bladder (i.e., ureter, urethral, or renal pelvis) cancer of the urothelium.

Randomization was stratified by clinical tumor stage T2N0 vs. > T2N0 (including T2N1, T3, and T4a), renal function (creatinine clearance [CrCl] ≥ 60 mL/min vs. CrCl ≥ 40 mL/min to < 60 mL/min), and PD-L1 expression (high vs. low/negative). Patients received:

- Neoadjuvant IMFINZI 1,500 mg plus gemcitabine 1,000 mg/m² and cisplatin 70 mg/m² every 3 weeks for 4 cycles prior to surgery, followed by IMFINZI 1,500 mg every 4 weeks as a single agent adjuvant treatment for up to 8 cycles after surgery, or
- Neoadjuvant gemcitabine 1,000 mg/m² and cisplatin 70 mg/m² every 3 weeks for 4 cycles prior to surgery, without adjuvant treatment.

Patients with CrCl ≥ 40 mL/min to < 60 mL/min in both arms received split-dose cisplatin of 35 mg/m² on days 1 and 8 of each cycle.

A RECIST 1.1 tumor assessment was performed at baseline and upon completion of neoadjuvant therapy (prior to surgery). After surgery, tumor assessments were performed every 12 weeks for the first 24 months, then every 24 weeks for 36 months, and then every 52 weeks thereafter until progression, the end of study, or death.

The major efficacy outcome measure was EFS by BICR assessment. OS was an additional efficacy outcome measure.

Baseline demographics were: male (82%), median age 65 years (range: 32 to 84), age ≥ 65 years (53%), White (67%), Asian (28%), Black or African American (0.9%), other (0.8%), Hispanic or Latino (8%), and ECOG PS 0 (78%) vs. PS 1 (22%). Disease characteristics were: tumor stage T2N0 (40%) and $>T2N0$ (60%), regional lymph nodes N0 (95%) and N1 (5%), adequate renal function (81%) and borderline renal function (19%). The histologic subtypes included urothelial carcinoma (85%) and urothelial carcinoma with variant histology (15%).

In the overall population, 469 (88%) patients in the IMFINZI plus gemcitabine and cisplatin arm and 441 (83%) patients in the gemcitabine and cisplatin arm underwent radical cystectomy. At a pre-specified interim analysis, the study demonstrated a statistically significant improvement in EFS and OS in the IMFINZI plus gemcitabine and cisplatin arm compared to the gemcitabine and cisplatin arm. The trial was not designed to isolate the effect of IMFINZI in each phase (neoadjuvant or adjuvant) of treatment. See Table 26 and Figures 19 and 20.

Table 26. Efficacy Results for the NIAGARA Study

	IMFINZI chemotherapy (N=533)	+ Chemotherapy (N = 530)
EFS^a		
Number of events (%)	187 (35.1)	246 (46.4)
Median EFS (months) (95% CI)^b	NR (NR, NR)	46.1 (32.2, NR)
HR (95% CI) ^c	0.68 (0.56, 0.82)	
2-sided p-value ^{d,e}	<0.0001	
pCR^f		
Number of patients with response	180	137
Response rate, % (95% CI) ^g	33.8 (29.7, 38.0)	25.8 (22.2, 29.8)
Odds ratio (95% CI) ^h	1.49 (1.14, 1.96)	
2-sided p-value ^h	0.0038	
OS^a		
Number of events (%)	136 (25.5)	169 (31.9)
Median OS (months) (95% CI)^b	NR (NR, NR)	NR (NR, NR)
HR (95% CI) ^c	0.75 (0.59, 0.93)	
2-sided p-value ^{d,e}	0.0106	

^a Results are based on a pre-planned interim analysis (DCO: 29 April 2024) which occurred 68 months after study initiation.

^b Calculated using the Kaplan-Meier technique.

^c Based on stratified Cox proportional hazard model with tumour stage [TN20 vs. > T2N0], renal function [adequate vs. borderline], and PD-L1 status [high vs. low/negative] as stratification factors.

^d Based on stratified log-rank test with tumour stage [T2N0 vs. > T2N0], renal function [adequate vs. borderline], and PD-L1 status [high vs. low/negative] as stratification factors.

^e The boundary for declaring statistical significance for the primary efficacy endpoints pCR rate, EFS and the key secondary endpoint OS were determined by a multiple test procedure with an alpha-exhaustive recycling strategy. Alpha allocated to EFS and OS at the interim analysis was based on a Lan DeMets alpha spending function with O'Brien Fleming approach (pCR = 0.001, EFS = 0.0412, OS = 0.0154, 2-sided).

^f Based on the final analysis of pCR (DCO: 14 Jan 2022).

^g CI was calculated using the Clopper Pearson method.

^h Obtained using logistic regression adjusted for the stratification factors (renal function [adequate vs. borderline], tumour stage [T2N0 vs. > T2N0] and PD-L1 status [high vs. low/negative] per IVRS).

CI=Confidence Interval, HR=Hazard Ratio, NR=Not Reached.

Figure 19. Kaplan-Meier Curve of EFS

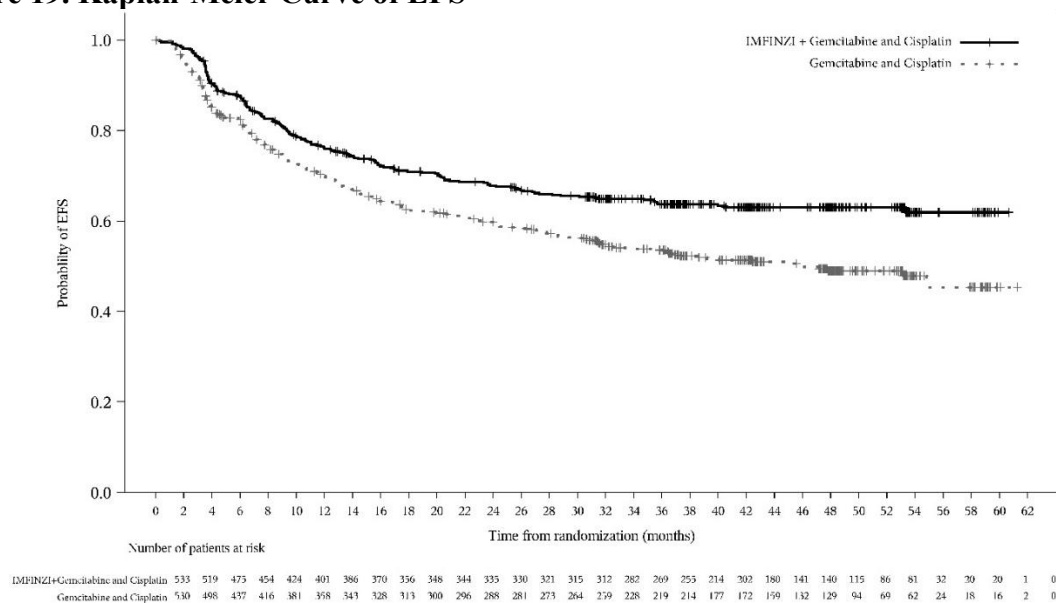
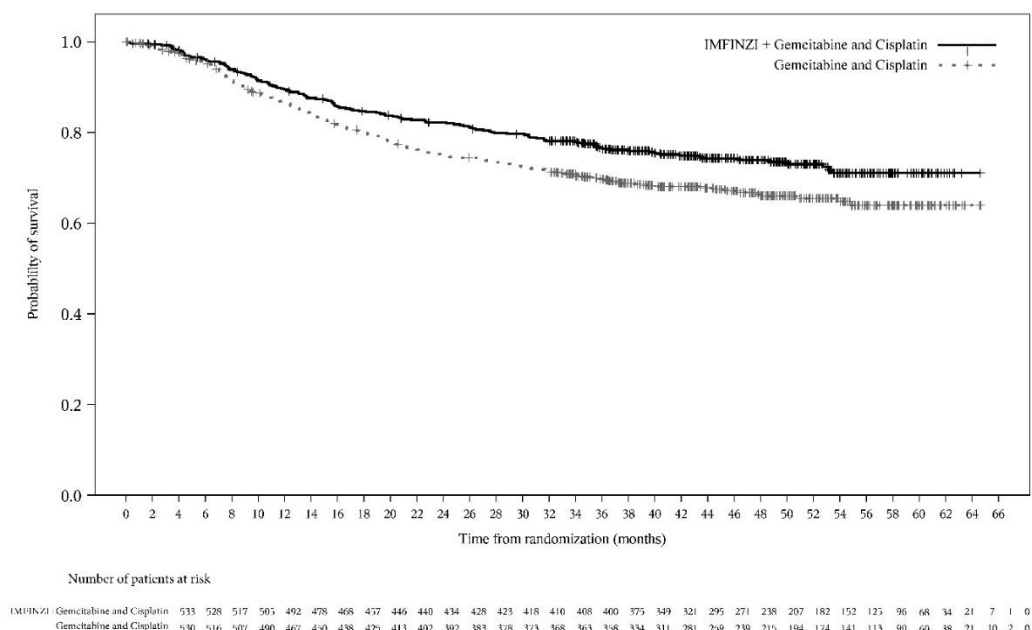


Figure 20. Kaplan-Meier Curve of OS



HOW SUPPLIED/STORAGE AND HANDLING

IMFINZI (durvalumab) Injection is a clear to opalescent, colorless to slightly yellow solution supplied in a carton containing one single-dose vial either as:

- 500 mg/10 mL
- 120 mg/2.4 mL

Store in a refrigerator at 2°C to 8°C (36°F to 46°F) in original carton to protect from light.

Do not freeze. Do not shake.

INCOMPATIBILITIES

Durvalumab

No incompatibilities between IMFINZI and 9 g/L (0.9%) sodium chloride or 50 g/L (5%) dextrose in polyvinylchloride or polyolefin intravenous IV bags have been observed.

This drug product must not be mixed with other drug products except those mentioned in *Preparation and Administration* section.

Do not co-administer other drugs through the same intravenous line.

MANUFACTURER

Manufactured by

Vetter Pharma-Fertigung GmbH & Co. KG, Mooswiesen 2, 88214 Ravensburg, Germany

AstraZeneca AB, Gartunavagen 152 57, Sodertalje Sweden

Released by

AstraZeneca AB, Gartunavagen 152 57, Sodertalje Sweden

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